ENGLISH FOR WORK / ACCESS / OPERATIONS / PRIVACY / PATIENT COMMUNICATION

Healthcare Administration English

Clarify the status. Coordinate the next step. Keep the patient informed.



Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.
No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

Eight practical lessons. Eleven extended conversations.
Vocabulary, phrase banks, guided speaking, and explained answers.

Revised 2026 edition / Upper-intermediate to advanced

Your route through the book

01 Patient Access, Scheduling, and Referrals, page 4

Request missing referral information and give a patient a reliable update without inventing clinical details.

02 Revenue Cycle, Coding, and Denials, page 16

Distinguish submission rejections from adjudicated denials and assign follow-up using the actual response record.

03 Patient Flow, Capacity, and Staffing, page 28

Report usable capacity with a time reference and distinguish physical space from staffed, ready capacity.

04 Quality, Safety, and Accreditation, page 39

Describe a near miss factually and distinguish a corrective action from evidence that recurrence risk has been reduced.

05 HIPAA, Privacy, and Information Governance, page 53

Clarify a data request and apply the appropriate privacy review without turning a limited task into unnecessary disclosure.

06 Patient Experience and Service Recovery, page 64

Acknowledge a communication failure, offer an accountable next step, and avoid making a clinical promise.

07 Population Health and Care Coordination, page 76

Hand over unresolved care-transition tasks with verified status, ownership, and a clear escalation condition.

08 Executive Dashboards and Board Updates, page 88

Explain a performance change with comparable definitions and turn a dashboard into a bounded decision request.

Professional communication notes, page 3

Three additional workplace cases, page 102

Answers and explanations, page 113

Phrase finder, page 171

Vocabulary index, page 173

References and publication notes, page 178

Suggested study range: B2-C1. This is an editorial study range, not a certified proficiency assessment.

A clear handoff includes an owner.

Healthcare administration connects people, records, schedules, and decisions. A referral received is not an appointment booked; a discharge planned is not a bed available; a message sent is not a handoff accepted. These fictional cases practice the language needed to preserve those distinctions while remaining considerate and useful.

Tell the patient what is known

Acknowledge the request, explain the missing administrative step in ordinary language, and name the next update. Avoid making patients decode internal queue names or promises that depend on an unconfirmed clinical decision.
"We received the referral. I am checking the missing information and will update you by three."

Separate current status from expected status

Forecasts help teams plan, but they must remain visibly conditional. Confirm what has actually happened before reporting capacity, payment, transport, or approval as available.
"Two discharges are planned; neither bed is confirmed available now."

Match access to the actual task

A colleague's job title alone does not settle the scope of a data request. Confirm purpose, role, applicable permissions, and the appropriate sharing route through organizational policy.
"Which fields are needed for this task, and has the requested access been confirmed?"

Close the communication loop

A named owner, a next action, an update time, and acknowledgment make a handoff usable. Administrative follow-up does not replace clinical judgment or the appropriate route for urgent concerns.
"I will confirm receipt and record who owns the unresolved transport request."

Professional boundaries

Original fictional language practice, not medical, legal, billing, coding, or compliance advice. US terminology appears where identified. Follow applicable law, payer rules, privacy safeguards, current organizational procedures, and qualified clinical judgment. No real patient information is used.

A Read the situation

LESSON 01 / PATIENT ACCESS, SCHEDULING, AND REFERRALS

Request missing referral information and give a patient a reliable update without inventing clinical details.

> **The referral arrived, but the booking is not ready**
>
> Scheduler Elena receives a referral with patient identifiers and contact details but no requested specialty or reason for the appointment. The patient expects a booking confirmation today. No appointment has been reserved and no clinical priority has been assigned. Referral-office colleague Marcus can seek clarification from the referring team. Elena can promise a status update by 3 p.m., not a confirmed appointment. Clinical urgency questions must follow the clinical review process rather than a scheduler's guess.

People: Elena, Scheduler; Marcus, Referral-office colleague.

Read the record

Choose one answer per question using the facts above.

1. What information is missing?

A. Every patient identifier and contact detail
B. A confirmed appointment that already exists
C. A documented clinical priority marked urgent
D. The requested specialty and reason for the appointment

2. What can Elena reliably promise on the supplied facts?

A. A particular specialist's clinical assessment
B. A same-day consultation regardless of availability
C. A status update by 3 p.m.
D. A confirmed appointment by 3 p.m.

3. Who should determine clinical urgency?

A. The appropriately qualified clinical team through its review process
B. The scheduler from an assumption about the referral
C. The patient-access dashboard from receipt time alone
D. The newest staff member to open the file

Answers and explanations, page 113

B Vocabulary in use

CORE TERMS

patient access
Administrative processes helping patients reach appropriate services.

coordinate patient access

referral
A request directing a patient to another service or professional.

process a referral

referring clinician
The clinician requesting another service or consultation.

contact the referring clinician

receiving service
The team or department expected to act on a referral.

identify the receiving service

specialty
A defined branch of clinical practice.

confirm the requested specialty

reason for referral
The stated clinical purpose of the requested consultation.

clarify the reason for referral

referral completeness
Whether the required referral information has been supplied.

check referral completeness

clinical triage
Qualified assessment of clinical priority and appropriate next care steps.

route for clinical triage

appointment slot
A defined time available for a scheduled visit.

confirm an appointment slot

scheduling template
The configured pattern of appointment types and times.

review the scheduling template

booking confirmation
A message establishing that an appointment has been arranged.

issue a booking confirmation

provisional hold
A temporary reservation subject to stated conditions.

distinguish a provisional hold from a booking

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

patient access PATIENT ACCESS

coordinate patient access

Keep the meaning precise: Administrative processes helping patients reach appropriate services.

reason for referral REASON FOR REFERRAL

clarify the reason for referral

Keep the meaning precise: The stated clinical purpose of the requested consultation.

B Precision and professional shorthand

EXTEND YOUR RANGE

waitlist

A managed list of patients awaiting an available appointment.

maintain the waitlist

cancellation list

A list used to offer newly available appointments.

check the cancellation list

intake queue

The worklist holding incoming requests for initial processing.

monitor the intake queue

demographic verification

Checking patient identity and contact details for accuracy.

complete demographic verification

eligibility verification

Checking current insurance or service eligibility information.

request eligibility verification

network status

Whether a provider participates in the relevant insurance network.

confirm network status

authorization requirement

A condition requiring the relevant approval before a specified service or coverage.

check authorization requirements

interpreter request

A request for appropriate language assistance.

arrange an interpreter request

accessibility need

A requirement affecting a person's ability to use a service.

record an accessibility need

callback commitment

An explicit promise to contact someone again by a stated time.

honor a callback commitment

closed-loop referral

A referral process with tracked receipt, action, and feedback to the relevant parties.

support closed-loop referrals

escalation pathway

The designated route for unresolved or urgent issues.

follow the escalation pathway

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

provisional hold PROVISIONAL HOLD

distinguish a provisional hold from a booking

Keep the meaning precise: A temporary reservation subject to stated conditions.

eligibility verification ELIGIBILITY VERIFICATION

request eligibility verification

Keep the meaning precise: Checking current insurance or service eligibility information.

C Phrases that move the work forward

Acknowledge receipt

We received your referral and are checking the information needed to book.

Request clarification

Please confirm the intended receiving service with the referring team.

Set an update

I will contact you by 3 p.m. with the current status.

Mark booking status

No appointment has been reserved yet.

Take ownership

I will track the clarification request rather than ask you to restart it.

Support access

Let us confirm any interpreter or accessibility arrangements through the usual process.

Check receipt

Please confirm that the referring team received the clarification request.

Avoid a guarantee

I cannot promise an available slot before the receiving service is confirmed.

Name the missing detail

The requested specialty and appointment reason are not shown.

Avoid an invented reason

I cannot supply a clinical reason from the contact details alone.

Separate the promise

That is an update time, not a confirmed appointment time.

Route urgency

Clinical priority needs review by the qualified clinical team.

Confirm the contact

What is the appropriate contact route recorded for this request?

Keep the patient informed

We are waiting for specific information, and I will update you even if it is still pending.

Preserve the distinction

A provisional hold would still need its conditions explained.

Close the loop

Record the response, next action, owner, and patient update.

Precision in real exchanges

Purpose: Avoid an invented reason
Useful wording: I cannot supply a clinical reason from the contact details alone.

Purpose: Set an update
Useful wording: I will contact you by 3 p.m. with the current status.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which message matches the referral's actual status?

A. We received the referral and need its specialty and reason; I will update you by 3 p.m.
B. Your referral is accepted, with the appointment awaiting only a convenient time.
C. Your booking is provisional because the patient identifiers have been checked.
D. The referral is routine, and three o'clock is the consultation window.

2. Which clarification should Marcus request?

A. A clinical reason reconstructed from the patient's address
B. Permission to call the referral complete without further information
C. The intended specialty and clinical reason from the referring team
D. A specialty selected to match today's first free slot

3. A colleague needs the term for this meaning: The team or department expected to act on a referral. Which term fits?

A. appointment slot
B. intake queue
C. accessibility need
D. receiving service

D Choose the response that fits / continued

4. Rebuild the exchange from "The referral arrived, but the booking is not ready". Elena: "Could you ask the referring office to confirm the specialty? I should not choose whichever service happens to have an opening this afternoon." Marcus: [missing response] Elena: "We also have no clinical priority recorded. I cannot assume that the missing reason means the request is routine or unimportant." Which response belongs in the gap?

A. Start with referral completeness. We have identifiers and contact details, but not enough information to identify the requested service. Receipt and readiness to book are different statuses.

B. I will contact the referring clinician through the appropriate office route. The request will identify both missing fields so we do not resolve one and overlook the other.

C. Yes. The scheduling template needs to match the confirmed request. We should not turn an available time into an unsuitable booking simply to make the status look complete.

D. Include an interpreter request where appropriate, and check other relevant access needs. The booking should be usable for the patient, not merely occupy a line on the calendar.

Answers and explanations, page 113

One useful distinction

Received means the referral arrived; complete means the required information is present; booked means an appointment is arranged. None of these statuses automatically establishes clinical urgency, insurance coverage, or authorization. State each confirmed fact separately.

E Conversation

The referral arrived, but the booking is not ready

The referral arrived, but the booking is not ready

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

referral completeness / callback commitment / escalation pathway / booking confirmation / closed-loop referral / intake queue / scheduling template / clinical triage / referring clinician / interpreter request

01 **Elena:** The referral arrived this morning, but the specialty and appointment reason are blank. The patient expects a booking confirmation today. I need to explain what happens next.

02 **Marcus:** Start with [1] _____. We have identifiers and contact details, but not enough information to identify the requested service. Receipt and readiness to book are different statuses.

03 **Elena:** The patient may think we lost the referral if I say we cannot confirm a visit yet. I want to acknowledge that it is here.

04 **Marcus:** Say it is in the [2] _____ and explain the missing details in ordinary language. Avoid giving the patient an internal queue name as though that answers the practical question.

05 **Elena:** Could you ask the referring office to confirm the specialty? I should not choose whichever service happens to have an opening this afternoon.

06 **Marcus:** I will contact the [3] _____ through the appropriate office route. The request will identify both missing fields so we do not resolve one and overlook the other.

E Conversation / continued

07 **Elena:** We also have no clinical priority recorded. I cannot assume that the missing reason means the request is routine or unimportant.

08 **Marcus:** Correct. Any urgency question belongs in [4] _____. Follow the established clinical review pathway instead of assigning a category from incomplete administrative information.

09 **Elena:** No appointment is reserved. I can update the patient by three, but I cannot promise that the missing information or a suitable slot will be available then.

10 **Marcus:** Make that a [5] _____. State the time and your responsibility clearly. The patient should hear a reliable next contact, not an implied promise of a consultation.

Conversation continues on the next page.

The referral arrived, but the booking is not ready

CONVERSATION / CONTINUED

11 **Elena:** I will say we received the referral and are checking two details before booking. I can avoid blaming the other office or making the patient repeat everything.

12 **Marcus:** That distinguishes the update from a [6] _____. If the information is still pending at three, contact the patient anyway and explain the next step rather than letting the promise expire silently.

13 **Elena:** Once the specialty is confirmed, we still need the receiving team to identify the appropriate appointment type. A general opening may not fit the requested service.

14 **Marcus:** Yes. The [7] _____ needs to match the confirmed request. We should not turn an available time into an unsuitable booking simply to make the status look complete.

15 **Elena:** If the patient needs language assistance or an access arrangement, that should be recorded through our usual process before the visit is finalized.

16 **Marcus:** Include an [8] _____ where appropriate, and check other relevant access needs. The booking should be usable for the patient, not merely occupy a line on the calendar.

17 **Elena:** I will update the patient by three. Please confirm when the referring office receives your request, and tell me if you need help reaching them.

18 **Marcus:** That gives us a [9] _____ process rather than two disconnected messages. We need a visible response and next action, not just evidence that someone pressed send.

The referral arrived, but the booking is not ready / continued

19 **Elena:** If the usual contact cannot resolve it, who takes the next step? I want a clear route while I keep the patient updated.

20 **Marcus:** Follow the [10] _____ and preserve the distinction between administrative follow-up and clinical judgment. The immediate commitment remains a clear update by three, with no invented appointment or priority.

CHECK THE GAPS

1. referral completeness / **2.** intake queue / **3.** referring clinician / **4.** clinical triage / **5.** callback commitment / **6.** booking confirmation / **7.** scheduling template / **8.** interpreter request / **9.** closed-loop referral / **10.** escalation pathway

Why these answers fit, page 114

F Check accuracy and scope

The referral arrived, but the booking is not ready

Return to the case

Scheduler Elena receives a referral with patient identifiers and contact details but no requested specialty or reason for the appointment. The patient expects a booking confirmation today. No appointment has been reserved and no clinical priority has been assigned. Referral-office colleague Marcus can seek clarification from the referring team. Elena can promise a status update by 3 p.m., not a confirmed appointment. Clinical urgency questions must follow the clinical review process rather than a scheduler's guess.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What does the absence of an assigned priority mean?

A. Clinical priority is unconfirmed.
B. The patient is definitely low priority.
C. The patient automatically requires the highest priority.
D. Receipt time alone establishes the clinical category.

2. At 3 p.m., clarification is still pending. What honors the commitment?

A. Wait silently until an appointment can be guaranteed.
B. Report a booking that has not been made.
C. Close the request because the deadline passed.
D. Contact the patient with the current status and a clear next update.

Answers and explanations, page 116

Listen for the distinction

Switch roles for the scheduling discussion. Stress update by three, no reserved slot, and clinical priority unconfirmed; do not choose a specialty for convenience.

Make the next step concrete

Patients may hear received as ready to book. Explain the difference without blaming the referring office or listing internal obstacles. Promise an update you can control, identify who is seeking the missing information, and keep clinical prioritization with the appropriately qualified team.

F Say it / transfer to a new case

A callback is not a consultation

A scheduler promises a status call by 2 p.m. The specialist has not reviewed the referral, and no appointment is reserved. At 2 p.m., review is still pending.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

reserved / call / pending / confirmation

01 **Scheduler:** I promised a status [1] _____.

02 **Colleague:** The specialist review remains [2] _____.

03 **Scheduler:** No appointment is [3] _____.

04 **Colleague:** Make the promised update without implying a booking [4] _____.

CHECK THE GAPS

1. call / **2.** pending / **3.** reserved / **4.** confirmation

Why these answers fit, page 116

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "The referral arrived, but the booking is not ready". Elena: "Once the specialty is confirmed, we still need the receiving team to identify the appropriate appointment type. A general opening may not fit the requested service." Marcus: [missing response] Elena: "If the patient needs language assistance or an access arrangement, that should be recorded through our usual process before the visit is finalized." Which response belongs in the gap?

A. Yes. The scheduling template needs to match the confirmed request. We should not turn an available time into an unsuitable booking simply to make the status look complete.

B. Start with referral completeness. We have identifiers and contact details, but not enough information to identify the requested service. Receipt and readiness to book are different statuses.

C. I will contact the referring clinician through the appropriate office route. The request will identify both missing fields so we do not resolve one and overlook the other.

D. Include an interpreter request where appropriate, and check other relevant access needs. The booking should be usable for the patient, not merely occupy a line on the calendar.

Answers and explanations, page 117

A Read the situation

LESSON 02 / REVENUE CYCLE, CODING, AND DENIALS

Distinguish submission rejections from adjudicated denials and assign follow-up using the actual response record.

> **One exception total hides two kinds of follow-up**
>
> A dashboard lists 20 exceptions among 100 submitted claims. The underlying records show 12 rejected before adjudication because required member identifiers were missing or invalid, and eight denied after payer review. The other 80 claims have not been analyzed in this meeting and must not be called paid. Revenue-cycle manager Leo and billing analyst Tessa need separate worklists, reasons, owners, and applicable deadlines. They must verify payer-specific correction or appeal routes, not change clinical codes merely to obtain payment.

People: Leo, Revenue-cycle manager; Tessa, Billing analyst.

Read the record

Choose one answer per question using the facts above.

1. How many exceptions were rejected before adjudication?

A. Twenty
B. Eighty
C. Twelve
D. Eight

2. What is known about the other 80 claims?

A. They were not analyzed in this meeting.
B. All were paid in full.
C. All were denied for missing identifiers.
D. They are all patient balances.

3. What must guide the correction or appeal route?

A. The actual response and applicable payer rules
B. The assumption that every exception uses the same appeal
C. A change to whichever diagnosis code pays more
D. The color of the dashboard category

Answers and explanations, page 118

B Vocabulary in use

CORE TERMS

revenue cycle
The sequence of administrative and financial processes from patient access through payment follow-up.

manage revenue-cycle performance

clearinghouse
An intermediary processing and transmitting electronic health-care transactions.

review clearinghouse responses

front-end rejection
A submission failure before the relevant adjudication stage in the stated workflow.

correct a front-end rejection

claim denial
A payer decision not to allow payment as requested after review in this case.

analyze claim denials

claim adjustment reason code
A standardized code explaining why a claim payment differs from the billed amount.

interpret the claim adjustment reason code

member identifier
The information identifying the person within the relevant health plan.

verify the member identifier

claim submission
Sending a request for payment to a payer.

track claim submissions

acknowledgment
A response indicating receipt or initial processing status.

inspect the claim acknowledgment

adjudication
Payer review determining how a claim is processed for benefits and payment.

confirm adjudication status

remittance advice
Information explaining payment and adjustments for processed claims.

review the remittance advice

remittance advice remark code
A code supplying additional explanation related to an adjustment or processing result.

review remittance advice remark codes

payer identifier
The identifier used to direct a transaction to the intended payer.

validate the payer identifier

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

revenue cycle REVENUE CYCLE

manage revenue-cycle performance

Keep the meaning precise: The sequence of administrative and financial processes from patient access through payment follow-up.

adjudication ADJUDICATION

confirm adjudication status

Keep the meaning precise: Payer review determining how a claim is processed for benefits and payment.

B Precision and professional shorthand

EXTEND YOUR RANGE

clean claim

A claim meeting the applicable completeness and processing requirements.

define the clean-claim measure

corrected claim

A submission changing information through the payer's applicable correction process.

submit a corrected claim

resubmission

Sending a claim again through the appropriate process.

track the resubmission

appeal

A request to reconsider an adverse determination under the applicable rules.

evaluate appeal options

timely-filing limit

The applicable deadline for submitting a claim.

verify the timely-filing limit

appeal deadline

The applicable time limit for requesting review of a decision.

track the appeal deadline

denial reason

The stated basis for a payer's denial.

categorize denial reasons

workqueue

A managed list of items awaiting assigned action.

assign the billing workqueue

accounts receivable

Amounts recorded as due to the organization.

review accounts receivable

aging bucket

A grouping of outstanding balances by elapsed time.

monitor aging buckets

patient responsibility

The amount properly assigned to the patient under applicable coverage and billing rules.

verify patient responsibility

coding integrity

Accurate coding supported by the clinical record and applicable rules.

preserve coding integrity

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

payer identifier PAYER IDENTIFIER

validate the payer identifier

Keep the meaning precise: The identifier used to direct a transaction to the intended payer.

timely-filing limit TIMELY-FILING LIMIT

verify the timely-filing limit

Keep the meaning precise: The applicable deadline for submitting a claim.

C Phrases that move the work forward

Separate the stages

Twelve submissions were rejected before adjudication; eight claims were denied after review.

Avoid an inflated conclusion

The other eighty claims have not been analyzed here.

Request the source

Show the acknowledgment or remittance record behind this category.

Name the rejection issue

The member identifier is missing or invalid on these submissions.

Verify before correcting

Check the identifier against the appropriate source record.

Route by reason

Assign follow-up according to the actual response and payer process.

Check the deadline

Confirm the applicable filing or appeal deadline for each item.

Preserve coding accuracy

Do not change a clinical code solely to obtain payment.

Distinguish next steps

A correction and an appeal are not interchangeable in every workflow.

Clarify the denominator

The twenty exceptions are out of one hundred submitted claims.

Avoid a payment promise

Resubmission does not establish that the payer will pay.

Track completion

Record the action, owner, date, and resulting status.

Keep responsibility open

Do not transfer an unresolved amount to the patient without the required review.

Explain the adjustment

Read the reason code together with any relevant remark.

Prevent double-counting

Count a claim consistently as it moves through the workqueues.

Close with visibility

Show separate rejection and denial totals alongside unresolved follow-up.

Precision in real exchanges

Purpose: Name the rejection issue
Useful wording: The member identifier is missing or invalid on these submissions.

Purpose: Verify before correcting
Useful wording: Check the identifier against the appropriate source record.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. What percentage of the 20 exceptions are pre-adjudication rejections?

A. 60%
B. 12%
C. 40%
D. 80%

2. Which dashboard breakdown preserves both stage and denominator?

A. Twelve rejected claims, eight appeals won, and eighty claims awaiting payment
B. Twenty rejected submissions out of eighty successfully paid claims
C. Twelve pre-adjudication rejections and eight reviewed denials out of 100 submissions
D. Twenty reviewed denials and eighty paid claims out of 100 submissions

3. A colleague needs the term for this meaning: A response indicating receipt or initial processing status. Which term fits?

A. claim adjustment reason code
B. resubmission
C. accounts receivable
D. acknowledgment

D Choose the response that fits / continued

4. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "For the twelve identifier errors, could we copy the number from an older paid claim? That would clear them quickly." Tessa: [missing response] Leo: "And for the eight denials, I need more than denied on a list. Can we see why payment was not allowed?" Which response belongs in the gap?

A. Check for a remittance advice remark code alongside the adjustment reason and group code. Read them together before assigning correction, reconsideration, or appeal work.

B. Their payment status is unresolved in this meeting. We cannot label them paid or assign patient responsibility merely because this view shows no exception.

C. I will structure the workqueue that way and retain claim identity across resubmissions. Several messages about one claim must not become several claims.

D. Verify the current member identifier against the appropriate source first. An old claim can be a lead, but it is not permission to guess or reuse unchecked information.

Answers and explanations, page 118

> **One useful distinction**
>
> For this case, rejected means not accepted into adjudication; denied means reviewed and not allowed as requested. Real systems may use labels differently. Verify the acknowledgment and remittance details rather than relying on one word in a dashboard.

E Conversation

One exception total hides two kinds of follow-up

One exception total hides two kinds of follow-up

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

coding integrity / front-end rejections / member identifier / timely-filing limit / remittance advice remark code / remittance advice / patient responsibility / workqueue / acknowledgment / denial analysis

01 **Leo:** Twenty exceptions out of one hundred submissions. Can I call that a twenty-percent denial rate in the management update?

02 **Tessa:** Not from this breakdown. Twelve are [1] _____ for missing or invalid member identifiers. Only eight reached payer review and were denied.

03 **Leo:** Then my appeals-backlog label is wrong too. Where do we check which stage each submission actually reached?

04 **Tessa:** Start with its [2] _____ and payer response. The dashboard groups work for convenience; it does not establish the correct next action.

05 **Leo:** For the twelve identifier errors, could we copy the number from an older paid claim? That would clear them quickly.

06 **Tessa:** Verify the current [3] _____ against the appropriate source first. An old claim can be a lead, but it is not permission to guess or reuse unchecked information.

07 **Leo:** And for the eight denials, I need more than denied on a list. Can we see why payment was not allowed?

08 **Tessa:** Yes. Review the [4] _____ and supporting details. A documentation issue and a coverage determination may require different payer-specific follow-up.

09 **Leo:** The summary has a broad adjustment reason. Is there another code that explains the missing detail?

10 **Tessa:** Check for a [5] _____ alongside the adjustment reason and group code. Read them together before assigning correction, reconsideration, or appeal work.

Conversation continues on the next page.

One exception total hides two kinds of follow-up

CONVERSATION / CONTINUED

11 **Leo:** Prioritize by deadline as well as category. I do not want older items expiring while we fix the easier ones.

12 **Tessa:** We will check each [6] _____ and any separate appeal deadline. Sending something again does not necessarily restart the applicable clock.

13 **Leo:** A colleague suggested a different diagnosis code on one denial because it might pay. I want that handled properly.

14 **Tessa:** That needs qualified review for [7] _____, based on the clinical record. Payment preference cannot supply evidence for changing a diagnosis.

15 **Leo:** What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here.

16 **Tessa:** Their payment status is unresolved in this meeting. We cannot label them paid or assign [8] _____ merely because this view shows no exception.

17 **Leo:** Create two lists, then: twelve rejections and eight denials. I need an owner, reason, deadline, and latest status against each claim.

18 **Tessa:** I will structure the [9] _____ that way and retain claim identity across resubmissions. Several messages about one claim must not become several claims.

19 **Leo:** The twelve are sixty percent of exceptions, but twelve percent of all submissions. I will put the denominator next to each percentage.

20 **Tessa:** Good. Keep the [10] _____ focused on the eight adjudicated denials. We can describe the follow-up clearly without promising its payment outcome.

CHECK THE GAPS

1. front-end rejections / **2.** acknowledgment / **3.** member identifier / **4.** remittance advice / **5.** remittance advice remark code / **6.** timely-filing limit / **7.** coding integrity / **8.** patient responsibility / **9.** workqueue / **10.** denial analysis

Why these answers fit, page 119

F Check accuracy and scope

One exception total hides two kinds of follow-up

Return to the case

A dashboard lists 20 exceptions among 100 submitted claims. The underlying records show 12 rejected before adjudication because required member identifiers were missing or invalid, and eight denied after payer review. The other 80 claims have not been analyzed in this meeting and must not be called paid. Revenue-cycle manager Leo and billing analyst Tessa need separate worklists, reasons, owners, and applicable deadlines. They must verify payer-specific correction or appeal routes, not change clinical codes merely to obtain payment.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. A rejection cites an invalid member identifier. What should happen first?

A. Charge the patient immediately because the submission failed.

B. Verify the identifier and applicable correction process against reliable records.

C. Replace the diagnosis with a higher-paying code.

D. Send a generic appeal without checking the stage.

2. Which summary preserves the financial uncertainty?

A. Correction or appeal may resolve the issue, but payment and patient responsibility remain subject to review.

B. Every corrected claim must be paid at its full billed amount.

C. Every denial automatically creates a patient debt.

D. A submitted appeal is equivalent to an overturned denial.

Answers and explanations, page 121

Listen for the distinction

Switch roles for turns 11-20. Say sixty percent of exceptions and twelve percent of submissions distinctly; do not label the other eighty paid.

Name the stage before naming the solution

A rejected submission and a denied claim may require different follow-up. Start with the actual acknowledgment or remittance record, not a dashboard label. Explain the stage, reason, and next responsible team without promising payment or treating every unpaid item as a patient balance.

F Say it / transfer to a new case

The denominator changes the percentage

A separate batch contains 200 submitted claims and 30 exceptions. Of the exceptions, 18 were rejected before adjudication and 12 denied after review. No payment status is supplied for the remainder.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

9% / twelve / unspecified / 60%

01 **Analyst:** Rejections are [1] _____ of all submissions.

02 **Manager:** Rejections are [2] _____ of the exceptions.

03 **Analyst:** Reviewed denials total [3] _____.

04 **Manager:** Payment status for the remainder is [4] _____.

CHECK THE GAPS

1. 9% / **2.** 60% / **3.** twelve / **4.** unspecified

Why these answers fit, page 121

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "A colleague suggested a different diagnosis code on one denial because it might pay. I want that handled properly." Tessa: [missing response] Leo: "What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here." Which response belongs in the gap?

A. That needs qualified review for coding integrity, based on the clinical record. Payment preference cannot supply evidence for changing a diagnosis.

B. Start with its acknowledgment and payer response. The dashboard groups work for convenience; it does not establish the correct next action.

C. Not from this breakdown. Twelve are front-end rejections for missing or invalid member identifiers. Only eight reached payer review and were denied.

D. We will check each timely-filing limit and any separate appeal deadline. Sending something again does not necessarily restart the applicable clock.

Answers and explanations, page 122

Review 1 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Patient Access, Scheduling, and Referrals" and "Revenue Cycle, Coding, and Denials". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: The clinician requesting another service or consultation. Which term fits?

A. interpreter request
B. cancellation list
C. clinical triage
D. referring clinician

2. A colleague needs the term for this meaning: Information explaining payment and adjustments for processed claims. Which term fits?

A. clean claim
B. revenue cycle
C. denial reason
D. remittance advice

3. Rebuild the exchange from "The referral arrived, but the booking is not ready". Elena: "No appointment is reserved. I can update the patient by three, but I cannot promise that the missing information or a suitable slot will be available then." Marcus: [missing response] Elena: "I will say we received the referral and are checking two details before booking. I can avoid blaming the other office or making the patient repeat everything." Which response belongs in the gap?

A. I will contact the referring clinician through the appropriate office route. The request will identify both missing fields so we do not resolve one and overlook the other.
B. Start with referral completeness. We have identifiers and contact details, but not enough information to identify the requested service. Receipt and readiness to book are different statuses.
C. That gives us a closed-loop referral process rather than two disconnected messages. We need a visible response and next action, not just evidence that someone pressed send.
D. Make that a callback commitment. State the time and your responsibility clearly. The patient should hear a reliable next contact, not an implied promise of a consultation.

Review 1 / Meaning and context / continued

4. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "Create two lists, then: twelve rejections and eight denials. I need an owner, reason, deadline, and latest status against each claim." Tessa: [missing response] Leo: "The twelve are sixty percent of exceptions, but twelve percent of all submissions. I will put the denominator next to each percentage." Which response belongs in the gap?

A. I will structure the workqueue that way and retain claim identity across resubmissions. Several messages about one claim must not become several claims.

B. Their payment status is unresolved in this meeting. We cannot label them paid or assign patient responsibility merely because this view shows no exception.

C. Not from this breakdown. Twelve are front-end rejections for missing or invalid member identifiers. Only eight reached payer review and were denied.

D. Check for a remittance advice remark code alongside the adjustment reason and group code. Read them together before assigning correction, reconsideration, or appeal work.

Answers and explanations, page 153

A Read the situation

LESSON 03 / PATIENT FLOW, CAPACITY, AND STAFFING

Report usable capacity with a time reference and distinguish physical space from staffed, ready capacity.

Planned discharges are not available beds

A unit has 28 physical beds, of which 24 are staffed and operational today. All 24 operational beds are occupied at the 10 a.m. census. Two discharges are planned, but neither timing nor completion is confirmed. The other four beds are not staffed for use. Flow coordinator Hana and unit manager Paul must correct a message saying two beds are available now. Any new placement depends on confirmed departure, room readiness, appropriate staffing, and the relevant clinical allocation process.

People: Hana, Flow coordinator; Paul, Unit manager.

Read the record

Choose one answer per question using the facts above.

1. How many staffed operational beds are unoccupied at 10 a.m.?

A. Zero
B. Two
C. Four
D. Six

2. What is the occupancy rate using staffed operational beds?

A. 100%
B. About 86%
C. About 92%
D. 50%

3. What remains unconfirmed?

A. The timing and completion of the two planned discharges
B. Whether twenty-four beds are occupied at the census
C. Whether four physical beds are unstaffed
D. Whether the census has a stated time

Answers and explanations, page 123

B Vocabulary in use

CORE TERMS

patient flow
Movement of patients through stages and locations of care.

coordinate patient flow

bed census
A count of occupied beds at a defined time.

report the bed census

physical bed capacity
The number of bed spaces physically present.

distinguish physical bed capacity

staffed capacity
Capacity supported by the required available staffing.

confirm staffed capacity

operational bed
A bed available for use when applicable operational conditions are met.

verify operational-bed status

occupancy rate
Occupied capacity divided by a specified capacity denominator.

state the occupancy-rate denominator

anticipated discharge
An expected departure that has not necessarily occurred.

track anticipated discharges

confirmed discharge
A departure or discharge status verified under the relevant process.

distinguish confirmed discharges

discharge barrier
An unresolved condition delaying a planned departure.

identify discharge barriers

bed turnaround
The process and time needed to prepare a vacated bed for reuse.

monitor bed turnaround

environmental services
The function providing cleaning and related environmental support.

coordinate with environmental services

room readiness
Confirmation that a room meets the conditions for its next use.

verify room readiness

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

patient flow PATIENT FLOW

coordinate patient flow

Keep the meaning precise: Movement of patients through stages and locations of care.

occupancy rate OCCUPANCY RATE

state the occupancy-rate denominator

Keep the meaning precise: Occupied capacity divided by a specified capacity denominator.

B Precision and professional shorthand

EXTEND YOUR RANGE

bed allocation

Assignment of a suitable bed through the appropriate process.

confirm bed allocation

acuity

The level of a patient's clinical complexity or care needs.

account for patient acuity

skill mix

The combination of staff capabilities available for a workload.

review the skill mix

staffing coverage

The personnel available to support defined work over a period.

verify staffing coverage

surge capacity

Additional capacity that can be mobilized under defined conditions.

assess surge capacity

contingency staffing

Alternative staffing arrangements used under an approved plan.

confirm contingency staffing

admission demand

The number and nature of patients requiring admission.

review admission demand

transfer request

A request to move a patient between care locations.

track transfer requests

boarding

Waiting in a location for an appropriate inpatient placement or next care setting.

monitor boarding

throughput

The rate at which patients move through a defined process.

evaluate patient throughput

flow huddle

A brief coordination meeting about current flow and constraints.

conduct a flow huddle

capacity forecast

An estimate of future usable capacity with stated assumptions.

qualify the capacity forecast

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

room readiness ROOM READINESS

verify room readiness

Keep the meaning precise: Confirmation that a room meets the conditions for its next use.

surge capacity SURGE CAPACITY

assess surge capacity

Keep the meaning precise: Additional capacity that can be mobilized under defined conditions.

C Phrases that move the work forward

Time-stamp the count

At 10 a.m., all twenty-four staffed operational beds are occupied.

State the denominator

Occupancy is 100% of staffed operational capacity.

Keep the forecast conditional

Two beds may become available after the required conditions are met.

Avoid an implied placement

This forecast does not authorize a patient transfer.

Identify the owner

Who will update the bed status after readiness is verified?

Protect the distinction

Vacated does not automatically mean ready for the next patient.

Avoid a false promise

I cannot report capacity that is not yet confirmed.

Label the estimate

This is a capacity forecast, not an available-bed count.

Correct the availability

The two discharges are planned, not completed.

Separate physical space

Four additional physical beds are not staffed for use today.

Name the dependency

Departure, room readiness, staffing, and allocation still need confirmation.

Request a fresh status

Please confirm the position before the next flow huddle.

Expose the barrier

Which unresolved issue affects the expected discharge time?

Check suitability

Clinical placement must follow the appropriate assessment process.

Coordinate the handoff

Notify the receiving team when the actual status changes.

Close the correction

Replace available now with anticipated, timing unconfirmed.

Precision in real exchanges

Purpose: Separate physical space
Useful wording: Four additional physical beds are not staffed for use today.

Purpose: Keep the forecast conditional
Useful wording: Two beds may become available after the required conditions are met.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which message accurately corrects the original update?

A. Two beds are available because two discharges appear in the plan.

B. Four beds are available because physical capacity exceeds occupied beds.

C. Occupancy is low enough to accept any transfer immediately.

D. At 10 a.m., no staffed operational beds are free; two discharges are anticipated with timing unconfirmed.

2. What does 24 divided by 28 measure here, and what does it omit?

A. Physical-bed occupancy; it includes four beds not staffed for use.

B. Staffed-bed occupancy; it includes two completed discharges.

C. Available operational capacity; it excludes the four unstaffed spaces.

D. Post-discharge occupancy; it assumes the two rooms are cleaned.

3. A colleague needs the term for this meaning: Capacity supported by the required available staffing. Which term fits?

A. discharge barrier

B. staffed capacity

C. boarding

D. skill mix

D Choose the response that fits / continued

4. Rebuild the exchange from "Planned discharges are not available beds". Hana: "The building has twenty-eight beds. Someone may divide twenty-four by twenty-eight and conclude that four spaces are free for admissions." Paul: [missing response] Hana: "For today's operational position, twenty-four occupied out of twenty-four staffed beds means full occupancy. We should name the denominator in the update." Which response belongs in the gap?

A. That is a capacity forecast. Include the dependencies and the next update time. A forecast without those limits can quickly become a promise as it moves between teams.

B. That matches the facts. We still need room readiness and the other applicable conditions before reporting availability. Keep the current count and the future possibility on separate lines.

C. That confuses physical space with staffed capacity. The other four beds are not staffed for use today. They are not an immediately usable reserve merely because they appear on the floor plan.

D. I will own the unit update and confirm it at the next flow huddle. If the position changes sooner, I will notify the desk through the usual channel rather than wait for the meeting.

Answers and explanations, page 123

> **One useful distinction**
>
> Occupancy is 100% on the staffed operational basis: 24 divided by 24. Dividing by 28 gives about 86% physical-bed occupancy, a different measure. Neither calculation makes the four unstaffed beds or two anticipated discharges available now.

E Conversation

Planned discharges are not available beds

Planned discharges are not available beds

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

bed census / Bed turnaround / room readiness / occupancy rate / anticipated discharges / flow huddle / capacity forecast / discharge-barrier / bed allocation / staffed capacity

01 **Hana:** The transfer desk has been told that two beds are available. Your ten o'clock count shows twenty-four occupied beds. What changed after the count?

02 **Paul:** Nothing has been confirmed. The [1] _____ still shows all twenty-four staffed operational beds occupied. Two departures are planned, but neither has a confirmed completion time.

03 **Hana:** Then the message has converted an expectation into present capacity. I will correct it before the receiving team treats it as permission to send someone.

04 **Paul:** Please call them [2] _____, not available beds. That keeps the information useful for planning while making clear that the patients have not yet left.

05 **Hana:** The building has twenty-eight beds. Someone may divide twenty-four by twenty-eight and conclude that four spaces are free for admissions.

06 **Paul:** That confuses physical space with [3] _____. The other four beds are not staffed for use today. They are not an immediately usable reserve merely because they appear on the floor plan.

E Conversation / continued

07 **Hana:** For today's operational position, twenty-four occupied out of twenty-four staffed beds means full occupancy. We should name the denominator in the update.

08 **Paul:** Yes. The [4] _____ is one hundred percent on that basis. A physical-bed percentage answers a different question and should not be substituted without explanation.

09 **Hana:** Do we know what is holding up the two expected departures? A forecast would be more useful if we understood which conditions remain unresolved.

10 **Paul:** We need a current [5] _____ check through the responsible team. I will request the status without inventing a departure time or making the clinical decision myself.

Conversation continues on the next page.

Planned discharges are not available beds

CONVERSATION / CONTINUED

11 **Hana:** Even after a patient leaves, the room may not be ready for immediate reuse. The next team needs more than a departure notification.

12 **Paul:** Correct. [6] _____ must be tracked through the relevant process. Environmental services and the responsible staff need to confirm their parts before the status becomes ready.

13 **Hana:** Once a room is ready, who confirms it is suitable for the incoming patient? I do not want a room-status change treated as placement approval.

14 **Paul:** The [7] _____ process handles that. Capacity language should support the decision, not bypass the appropriate assessment of patient needs, staffing, and the setting.

15 **Hana:** I will label the two possible openings as conditional. The transfer desk can plan around the possibility without treating it as a completed availability check.

16 **Paul:** That is a [8] _____. Include the dependencies and the next update time. A forecast without those limits can quickly become a promise as it moves between teams.

17 **Hana:** Who will update the desk when the status changes? I do not want the corrected message to sit unchanged after the room actually becomes ready.

18 **Paul:** I will own the unit update and confirm it at the next [9] _____. If the position changes sooner, I will notify the desk through the usual channel rather than wait for the meeting.

19 **Hana:** I will correct the desk now: no operational bed is free at ten, and the two possible openings depend on discharge and readiness confirmation.

20 **Paul:** That matches the facts. We still need [10] _____ and the other applicable conditions before reporting availability. Keep the current count and the future possibility on separate lines.

CHECK THE GAPS

1. bed census / **2.** anticipated discharges / **3.** staffed capacity / **4.** occupancy rate / **5.** discharge-barrier / **6.** Bed turnaround / **7.** bed allocation / **8.** capacity forecast / **9.** flow huddle / **10.** room readiness

Why these answers fit, page 124

F Check accuracy and scope

Planned discharges are not available beds

Return to the case

A unit has 28 physical beds, of which 24 are staffed and operational today. All 24 operational beds are occupied at the 10 a.m. census. Two discharges are planned, but neither timing nor completion is confirmed. The other four beds are not staffed for use. Flow coordinator Hana and unit manager Paul must correct a message saying two beds are available now. Any new placement depends on confirmed departure, room readiness, appropriate staffing, and the relevant clinical allocation process.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What must follow a confirmed departure before availability is reported?

A. Automatically mark every adjacent bed ready.

B. Assume the forecast has supplied clinical approval.

C. Treat the discharge plan as proof that cleaning already occurred.

D. Verify the relevant readiness, staffing, and allocation conditions.

2. Which update best supports planning without promising capacity?

A. No update should be given until every future admission is known.

B. Two discharges are anticipated; the unit will confirm timing and readiness at the next agreed check.

C. Two beds are definitely available now, subject to later confirmation.

D. Physical space alone is sufficient for every placement.

Answers and explanations, page 126

Listen for the distinction

Switch roles for the forecast. Keep two anticipated discharges separate from available beds, and retain the four unstaffed physical spaces.

Attach time and conditions to capacity

Capacity figures become misleading when yesterday's position, a forecast, and the current state share the same label. Say available now only when the relevant readiness conditions are confirmed. Explain the next check and owner so a conditional forecast remains useful without becoming an instruction to place a patient.

F Say it / transfer to a new case

An empty room still has conditions

One patient has left. The room is awaiting cleaning, and staffing for the next placement has not been confirmed. The status system distinguishes vacated from ready.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

ready / confirmation / vacated / pending

01 **Coordinator:** The room is [1] _____.

02 **Colleague:** Cleaning remains [2] _____.

03 **Coordinator:** Staffing still needs [3] _____.

04 **Colleague:** Do not mark the room [4] _____ yet.

CHECK THE GAPS

1. vacated / **2.** pending / **3.** confirmation / **4.** ready

Why these answers fit, page 126

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Planned discharges are not available beds". Hana: "Once a room is ready, who confirms it is suitable for the incoming patient? I do not want a room-status change treated as placement approval." Paul: [missing response] Hana: "I will label the two possible openings as conditional. The transfer desk can plan around the possibility without treating it as a completed availability check." Which response belongs in the gap?

A. The bed allocation process handles that. Capacity language should support the decision, not bypass the appropriate assessment of patient needs, staffing, and the setting.

B. Nothing has been confirmed. The bed census still shows all twenty-four staffed operational beds occupied. Two departures are planned, but neither has a confirmed completion time.

C. Please call them anticipated discharges, not available beds. That keeps the information useful for planning while making clear that the patients have not yet left.

D. Yes. The occupancy rate is one hundred percent on that basis. A physical-bed percentage answers a different question and should not be substituted without explanation.

Answers and explanations, page 127

A Read the situation

LESSON 04 / QUALITY, SAFETY, AND ACCREDITATION

Describe a near miss factually and distinguish a corrective action from evidence that recurrence risk has been reduced.

> **Caught in time, but not yet explained**
>
> A patient was assigned to the wrong specialty clinic. A second check caught the error before the appointment, and no clinical harm is reported. The referral record and booking history are available, but the cause has not been established. Quality coordinator Asha and operations lead Ben are preparing a learning review and evidence for an upcoming accreditation discussion. The event must follow the organization's reporting process. A proposed refresher session is not yet supported by a finding that lack of training caused the error.

People: Asha, Quality coordinator; Ben, Operations lead.

Read the record

Choose one answer per question using the facts above.

1. What is established about the event?

A. A training failure has been proved.
B. The system is guaranteed to prevent recurrence.
C. The accreditation review has already approved every process.
D. The wrong-specialty booking was caught before the appointment, with no clinical harm reported.

2. Why is a refresher session not yet a complete response?

A. Near misses must be excluded from learning reviews.
B. A proposed meeting automatically demonstrates effectiveness.
C. The review has not established a training gap or how the action would address the cause.
D. Training can never be part of a quality response.

3. What should the review use?

A. The referral record, booking history, and other relevant verified evidence
B. Only the first explanation offered in conversation
C. Only the absence of reported clinical harm
D. A rewritten record removing the error

Answers and explanations, page 128

B Vocabulary in use

CORE TERMS

near miss
A potentially harmful event intercepted or otherwise not resulting in harm.

report a near miss

safety event
An occurrence relevant to patient safety under the applicable reporting system.

document a safety event

incident report
A structured record of an event requiring review or follow-up.

submit an incident report

event chronology
The verified sequence of events and actions.

reconstruct the event chronology

contributing factor
A condition that helped an event occur or affected its outcome.

identify contributing factors

root cause analysis
A structured investigation of underlying factors behind an event.

scope the root cause analysis

systems approach
Examination of how processes and conditions shape performance.

use a systems approach

human factors
Study of how people interact with tools, tasks, and environments.

assess human-factors issues

just culture
An approach combining learning, fair accountability, and attention to system conditions.

support a just culture

psychological safety
A climate in which people can raise concerns without interpersonal fear.

strengthen psychological safety

detection control
A process intended to identify an error before further consequences.

evaluate the detection control

preventive control
A process intended to reduce the chance that an error occurs.

strengthen preventive controls

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

near miss NEAR MISS

report a near miss

Keep the meaning precise: A potentially harmful event intercepted or otherwise not resulting in harm.

root cause analysis ROOT CAUSE ANALYSIS

scope the root cause analysis

Keep the meaning precise: A structured investigation of underlying factors behind an event.

B Precision and professional shorthand

EXTEND YOUR RANGE

process map

A representation of the steps and handoffs in a workflow.

build a process map

failure mode

A way in which a process or component can fail.

identify a failure mode

risk register

A record of identified risks, responses, and ownership.

update the risk register

corrective action plan

A documented set of actions addressing an identified problem.

agree the corrective action plan

action owner

The person accountable for completing a specified action.

name the action owner

implementation evidence

Records showing that a planned action was carried out.

collect implementation evidence

effectiveness measure

A defined measure used to judge whether an action worked.

specify the effectiveness measure

process measure

A measure of whether a defined activity or step occurs as intended.

track a process measure

outcome measure

A measure of the result the process is intended to influence.

review the outcome measure

balancing measure

A measure monitoring unintended effects of a change.

include a balancing measure

accreditation evidence

Material supporting assessment against relevant accreditation requirements.

organize accreditation evidence

tracer review

Examination following a process or experience across relevant steps and records.

prepare for a tracer review

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

preventive control PREVENTIVE CONTROL

strengthen preventive controls

Keep the meaning precise: A process intended to reduce the chance that an error occurs.

action owner ACTION OWNER

name the action owner

Keep the meaning precise: The person accountable for completing a specified action.

C Phrases that move the work forward

State the event

The patient was booked into the wrong specialty clinic.

Report the interception

A second check caught the error before the appointment.

Limit the impact claim

No clinical harm has been reported in the available information.

Keep cause open

The cause has not yet been established.

Request the sequence

Let us reconstruct the booking steps from the records.

Avoid premature blame

We should not label this a training failure without evidence.

Value the detection

The second check worked in this event; we still need to examine the earlier steps.

Connect the action

Which verified contributing factor does this proposed action address?

Assign accountability

Each action needs an owner and completion date.

Define effectiveness

What measure would show that the relevant error risk has decreased?

Separate completion

A completed session does not itself demonstrate improved performance.

Preserve the record

Keep the original booking history and the documented correction.

Prepare the evidence

Show the event, response, implementation, and follow-up findings.

Monitor side effects

Check whether the change introduces delays or new workarounds.

Avoid a guarantee

One intercepted event does not prove that recurrence is impossible.

Close the review

Record supported findings and unresolved questions separately.

Precision in real exchanges

Purpose: Keep cause open
Useful wording: The cause has not yet been established.

Purpose: Request the sequence
Useful wording: Let us reconstruct the booking steps from the records.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which review opening is most precise?

A. The second check guarantees that this process is safe in every case.

B. The wrong-specialty booking was intercepted; we will reconstruct the sequence and assess contributing factors.

C. The scheduler needs training because all booking errors have that cause.

D. No review is needed because no harm was reported.

2. What does refresher attendance demonstrate?

A. That the original event was definitely caused by poor training

B. That every accreditation requirement has been satisfied

C. That the recorded participants attended, not necessarily that the process improved

D. That recurrence risk is now zero

3. A colleague needs the term for this meaning: The verified sequence of events and actions. Which term fits?

A. just culture

B. risk register

C. event chronology

D. outcome measure

D Choose the response that fits / continued

4. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "The immediate suggestion is a refresher session for schedulers. That may help, but we have not established that missing knowledge caused this event." Asha: [missing response] Ben: "We can thank the person who noticed the mismatch. At the same time, the review should not stop with the fact that the second check worked." Which response belongs in the gap?

A. Name an action owner and a date for each specific task. Connect each action to the relevant finding so the plan is more than a list of familiar training activities.

B. Identify the contributing factors first. The interface, referral wording, workflow, or other conditions may need examination. We should not select a cause merely because the proposed action is easy to organize.

C. We should report the near miss through our process. No clinical harm is reported, but that does not explain the error or establish that the same process will always catch it.

D. Exactly. It was a detection control in this event. We need to examine the earlier steps and whether the check operates reliably, rather than assuming one successful catch proves universal protection.

Answers and explanations, page 128

One useful distinction

No clinical harm reported is a statement about known impact, not proof that the process is reliable. A successful second check is evidence of detection in this event. The review still needs to understand the earlier failure and whether the control works consistently.

E Conversation

Caught in time, but not yet explained

Caught in time, but not yet explained

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

detection control / implementation evidence / action owner / contributing factors / accreditation evidence / just culture / event chronology / effectiveness measure / process map / near miss

01 **Ben:** The patient was booked into the wrong specialty, but the second check caught it before the appointment. Can we close the issue as a harmless administrative mistake?

02 **Asha:** We should report the [1] _____ through our process. No clinical harm is reported, but that does not explain the error or establish that the same process will always catch it.

03 **Ben:** The referral record and booking history are available. We can identify the sequence without relying only on what people remember after the correction.

04 **Asha:** Build the [2] _____ from those records and relevant accounts. Keep confirmed facts distinct from assumptions, especially where the timing or handoff is unclear.

05 **Ben:** The immediate suggestion is a refresher session for schedulers. That may help, but we have not established that missing knowledge caused this event.

06 **Asha:** Identify the [3] _____ first. The interface, referral wording, workflow, or other conditions may need examination. We should not select a cause merely because the proposed action is easy to organize.

E Conversation / continued

07 **Ben:** We can thank the person who noticed the mismatch. At the same time, the review should not stop with the fact that the second check worked.

08 **Asha:** Exactly. It was a [4] _____ in this event. We need to examine the earlier steps and whether the check operates reliably, rather than assuming one successful catch proves universal protection.

09 **Ben:** A simple diagram of the referral-to-booking route may help the teams see where information changed or responsibility became unclear.

10 **Asha:** Use a [5] _____ grounded in actual practice. Include the handoffs and workarounds people really use, not just the ideal sequence written in the procedure.

Conversation continues on the next page.

Caught in time, but not yet explained

CONVERSATION / CONTINUED

11 **Ben:** Staff are worried this meeting will start by naming someone to blame. How do we get an honest account of the workarounds without losing accountability?

12 **Asha:** A [6] _____ supports both learning and fair accountability. Ask what happened and why the process allowed it, while preserving the responsibilities appropriate to each role.

13 **Ben:** Once the findings are supported, the action plan should show who will do what. A general instruction to improve accuracy will not be easy to follow up.

14 **Asha:** Name an [7] _____ and a date for each specific task. Connect each action to the relevant finding so the plan is more than a list of familiar training activities.

15 **Ben:** For the accreditation discussion, we can show that an action was completed. But completion alone will not tell us whether it reduced the error risk.

16 **Asha:** Separate [8] _____ from effectiveness evidence. A revised form or attendance record proves an activity occurred; it does not automatically prove that bookings became more accurate.

17 **Ben:** We could review a consistently defined sample of bookings afterward. The measure and review period should be agreed before we decide whether the change worked.

18 **Asha:** That provides an [9] _____. Also monitor unintended consequences, such as delays or new workarounds, so an apparent improvement in one measure does not hide another problem.

19 **Ben:** I will prepare the timeline and mark the open questions. We can show what caught the error without claiming that we have already established its cause.

20 **Asha:** Good. The [10] _____ should reflect the actual state of the work. A clear account of an open investigation is more accurate than a completed-looking story unsupported by the records.

CHECK THE GAPS

1. near miss / **2.** event chronology / **3.** contributing factors / **4.** detection control / **5.** process map / **6.** just culture / **7.** action owner / **8.** implementation evidence / **9.** effectiveness measure / **10.** accreditation evidence

Why these answers fit, page 130

F Check accuracy and scope

Caught in time, but not yet explained

Return to the case

A patient was assigned to the wrong specialty clinic. A second check caught the error before the appointment, and no clinical harm is reported. The referral record and booking history are available, but the cause has not been established. Quality coordinator Asha and operations lead Ben are preparing a learning review and evidence for an upcoming accreditation discussion. The event must follow the organization's reporting process. A proposed refresher session is not yet supported by a finding that lack of training caused the error.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which measure most directly tests whether wrong-specialty booking accuracy improved?

A. Verified wrong-specialty bookings divided by comparable reviewed bookings over the planned period

B. Staff attending the refresher divided by staff scheduled for it

C. Completed action-plan tasks divided by tasks assigned

D. Refresher slides approved divided by slides submitted for review

2. Why include a balancing measure?

A. To ensure every result can be described as successful

B. To avoid recording the original error

C. To detect unintended effects such as added delays or workarounds

D. To replace the main effectiveness question

Answers and explanations, page 131

Listen for the distinction

Switch roles for the action discussion. Contrast refresher attendance with a defined measure of booking accuracy; preserve the original event record.

Investigate the process without inventing blame

A factual account separates what happened, how it was detected, the known impact, and what remains uncertain. Thank the person who detected the problem while examining why the earlier steps allowed it. An action plan should address supported contributing factors rather than assume that another reminder will solve everything.

F Say it / transfer to a new case

Attendance is not effectiveness

A refresher session has been delivered to all ten scheduled staff. No post-session booking review has yet occurred. The action plan calls for a four-week review of booking accuracy.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

untested / evidence / review / delivered

01 **Lead:** The session has been [1] _____.

02 **Reviewer:** Attendance supplies implementation [2] _____.

03 **Lead:** Effectiveness remains [3] _____.

04 **Reviewer:** Complete the planned four-week [4] _____.

CHECK THE GAPS

1. delivered / **2.** evidence / **3.** untested / **4.** review

Why these answers fit, page 131

F Say it / transfer to a new case / continued

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "Once the findings are supported, the action plan should show who will do what. A general instruction to improve accuracy will not be easy to follow up." Asha: [missing response] Ben: "For the accreditation discussion, we can show that an action was completed. But completion alone will not tell us whether it reduced the error risk." Which response belongs in the gap?

A. We should report the near miss through our process. No clinical harm is reported, but that does not explain the error or establish that the same process will always catch it.

B. Good. The accreditation evidence should reflect the actual state of the work. A clear account of an open investigation is more accurate than a completed-looking story unsupported by the records.

C. Identify the contributing factors first. The interface, referral wording, workflow, or other conditions may need examination. We should not select a cause merely because the proposed action is easy to organize.

D. Name an action owner and a date for each specific task. Connect each action to the relevant finding so the plan is more than a list of familiar training activities.

Answers and explanations, page 132

Review 2 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Patient Flow, Capacity, and Staffing" and "Quality, Safety, and Accreditation". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: The number of bed spaces physically present. Which term fits?

A. confirmed discharge

B. physical bed capacity

C. acuity

D. transfer request

2. A colleague needs the term for this meaning: Study of how people interact with tools, tasks, and environments. Which term fits?

A. near miss

B. human factors

C. process map

D. effectiveness measure

3. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "The summary has a broad adjustment reason. Is there another code that explains the missing detail?" Tessa: [missing response] Leo: "Prioritize by deadline as well as category. I do not want older items expiring while we fix the easier ones." Which response belongs in the gap?

A. Their payment status is unresolved in this meeting. We cannot label them paid or assign patient responsibility merely because this view shows no exception.

B. Check for a remittance advice remark code alongside the adjustment reason and group code. Read them together before assigning correction, reconsideration, or appeal work.

C. Not from this breakdown. Twelve are front-end rejections for missing or invalid member identifiers. Only eight reached payer review and were denied.

D. I will structure the workqueue that way and retain claim identity across resubmissions. Several messages about one claim must not become several claims.

Review 2 / Meaning and context / continued

4. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "We could review a consistently defined sample of bookings afterward. The measure and review period should be agreed before we decide whether the change worked." Asha: [missing response] Ben: "I will prepare the timeline and mark the open questions. We can show what caught the error without claiming that we have already established its cause." Which response belongs in the gap?

A. Separate implementation evidence from effectiveness evidence. A revised form or attendance record proves an activity occurred; it does not automatically prove that bookings became more accurate.

B. That provides an effectiveness measure. Also monitor unintended consequences, such as delays or new workarounds, so an apparent improvement in one measure does not hide another problem.

C. Use a process map grounded in actual practice. Include the handoffs and workarounds people really use, not just the ideal sequence written in the procedure.

D. A just culture supports both learning and fair accountability. Ask what happened and why the process allowed it, while preserving the responsibilities appropriate to each role.

Answers and explanations, page 156

A Read the situation

LESSON 05 / HIPAA, PRIVACY, AND INFORMATION GOVERNANCE

Clarify a data request and apply the appropriate privacy review without turning a limited task into unnecessary disclosure.

A full patient list for a counting task

An operations colleague wants to count last month's missed appointments by clinic. The request asks for every patient's name, date of birth, diagnosis, and full visit notes. A clinic-level count may meet the stated purpose, but neither the appropriate data scope nor the requester's export permission has been confirmed. This is an administrative reporting task, not a provider-to-provider treatment request. Analyst Omar and privacy colleague Nina must confirm the purpose, access, output, and approved sharing route before any extract is sent.

People: Omar, Reporting analyst; Nina, Privacy colleague.

Read the record

Choose one answer per question using the facts above.

1. What is the stated purpose?

A. Count missed appointments by clinic for last month
B. Review individual treatment decisions between providers
C. Publish full clinical notes for public use
D. Change every patient's clinical priority

2. What remains unconfirmed?

A. Appropriate data scope and export permission
B. Whether a full patient list has been requested
C. Whether the reporting period is last month
D. Whether the task concerns missed appointments

3. Which output may meet the stated purpose?

A. A suitably reviewed clinic-level count
B. Full visit notes for every patient by default
C. A diagnosis list with names removed and no further review
D. Any extract the analyst can technically download

Answers and explanations, page 133

B Vocabulary in use

CORE TERMS

HIPAA
US Health Insurance Portability and Accountability Act; includes relevant privacy and security provisions.

follow applicable HIPAA requirements

protected health information
Identifiable health information covered by the applicable HIPAA definitions.

safeguard protected health information

electronic PHI
Protected health information maintained or transmitted electronically.

protect electronic PHI

covered entity
A health plan, clearinghouse, or qualifying provider covered by HIPAA.

identify the covered entity

business associate
A party handling PHI for specified functions on a covered entity's behalf.

assess business-associate responsibilities

business associate agreement
A contract setting relevant duties for a business associate handling PHI.

verify the business associate agreement

minimum necessary
Reasonable limitation of information to the purpose where the applicable standard applies.

apply the minimum-necessary standard

permitted use
A use allowed under the applicable legal and organizational basis.

verify the permitted use

disclosure
Release or access to information outside the entity holding it in the relevant privacy context.

assess the proposed disclosure

authorization
A specific permission meeting applicable requirements for a defined use or disclosure.

verify the authorization

role-based access
Access assigned according to work responsibilities.

enforce role-based access

access entitlement
The permission assigned to a user for defined information or functions.

review access entitlements

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

HIPAA HIPAA

follow applicable HIPAA requirements

Keep the meaning precise: US Health Insurance Portability and Accountability Act; includes relevant privacy and security provisions.

business associate agreement BUSINESS ASSOCIATE AGREEMENT

verify the business associate agreement

Keep the meaning precise: A contract setting relevant duties for a business associate handling PHI.

B Precision and professional shorthand

EXTEND YOUR RANGE

export permission

Authority to extract information from a system under the relevant controls.

confirm export permission

data minimization

Limiting collected or shared data to what is appropriate for the task.

practice data minimization

de-identification

A process meeting applicable criteria for removing or reducing identification risk.

verify de-identification

small-cell disclosure

Identification risk arising from very small groups in reported data.

assess small-cell disclosure risk

recipient verification

Confirmation of the intended recipient's identity and authority.

complete recipient verification

audit log

A record of system access or activity used for review.

review the audit log

purpose limitation

Restricting information handling to a specified appropriate purpose.

document the purpose limitation

aggregate report

A report combining individual records into group-level results.

review an aggregate report

re-identification risk

The possibility that information can be linked back to a person.

assess re-identification risk

secure transfer

Sharing through an approved protected method.

use an approved secure transfer

retention schedule

Rules specifying how long records are kept and when they are disposed of.

follow the retention schedule

privacy incident

An event involving possible inappropriate handling of protected information.

report a suspected privacy incident

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

access entitlement ACCESS ENTITLEMENT

review access entitlements

Keep the meaning precise: The permission assigned to a user for defined information or functions.

de-identification DE-IDENTIFICATION

verify de-identification

Keep the meaning precise: A process meeting applicable criteria for removing or reducing identification risk.

C Phrases that move the work forward

Clarify the purpose

Which decision requires this information, and what output is needed?

Challenge excess fields

Why would this counting task require full clinical notes?

Separate capability

Being able to download a file does not establish permission to share it.

Apply the right rule

Check the applicable privacy basis and any relevant exceptions.

Review group risk

Small counts may still reveal information about individuals.

Use the approved route

Share only through the authorized protected channel.

Consult the owner

Route unresolved privacy questions to the designated reviewer.

Do not send yet

Scope and permission are unconfirmed, so the extract remains unsent.

Match the scope

A clinic-level count may answer the stated question.

Confirm the role

Please verify the requester's relevant access and export permissions.

Identify the context

This is administrative reporting, not a treatment disclosure request.

Avoid a false shortcut

Removing names alone does not make the extract de-identified.

Verify the recipient

Confirm identity, authority, and the intended destination.

Keep a record

Document the purpose, approved scope, and handling decision.

Limit onward use

The output should not be repurposed beyond its authorized context.

Close the request

Confirm the approved output and record that the recipient received it appropriately.

Precision in real exchanges

Purpose: Confirm the role
Useful wording: Please verify the requester's relevant access and export permissions.

Purpose: Separate capability
Useful wording: Being able to download a file does not establish permission to share it.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which proposed output best matches the stated reporting purpose, subject to privacy review?

A. Every field visible to the analyst because the requester is an employee
B. Clinic-level monthly missed-appointment counts with appropriate small-group checks
C. Patient names and diagnoses with visit notes omitted but no purpose review
D. Full notes with names removed and the result labeled anonymous

2. What does technical download access establish?

A. That the requester needs all available fields
B. That privacy review is unnecessary for all exports
C. That the system permits the action technically, not that the proposed sharing is authorized
D. That every external destination is approved

3. A colleague needs the term for this meaning: A health plan, clearinghouse, or qualifying provider covered by HIPAA. Which term fits?

A. recipient verification
B. covered entity
C. disclosure
D. data minimization

D Choose the response that fits / continued

4. Rebuild the exchange from "A full patient list for a counting task". Omar: "The colleague works in operations and can view some records in the system. That does not tell me whether they can receive a broad export." Nina: [missing response] Omar: "My reporting account can technically download the full table. I should not treat the availability of an export button as approval for this request." Which response belongs in the gap?

A. Propose an aggregate report for review. It may meet the purpose without individual details, but we should still assess the output and its disclosure risks.

B. Clarify the permitted use and the intended output first. We should understand the decision they need to make before choosing fields from the patient record.

C. Good. Complete recipient verification and scope review before sharing. We can support the operational question without assuming that more personal information produces a better report.

D. Check role-based access and the specific permissions. A legitimate role can require some information without authorizing every field or every way of transferring it.

Answers and explanations, page 133

One useful distinction

The US minimum-necessary standard has exceptions, including disclosures to or requests by health-care providers for treatment. This case concerns administrative reporting, not that treatment exception. Confirm the applicable basis and organizational policy rather than applying one slogan to every information request.

E Conversation

A full patient list for a counting task

A full patient list for a counting task

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

permitted use / minimum necessary / audit log / role-based access / export permission / aggregate report / de-identification / re-identification risk / recipient verification / secure transfer

01 **Omar:** Operations wants last month's missed appointments by clinic. The request asks for names, dates of birth, diagnoses, and full visit notes. That seems broader than the question.

02 **Nina:** Clarify the [1] _____ and the intended output first. We should understand the decision they need to make before choosing fields from the patient record.

03 **Omar:** A count for each clinic may answer the question. I do not see why the report would need individual clinical notes.

04 **Nina:** Propose an [2] _____ for review. It may meet the purpose without individual details, but we should still assess the output and its disclosure risks.

05 **Omar:** The colleague works in operations and can view some records in the system. That does not tell me whether they can receive a broad export.

06 **Nina:** Check [3] _____ and the specific permissions. A legitimate role can require some information without authorizing every field or every way of transferring it.

E Conversation / continued

07 **Omar:** My reporting account can technically download the full table. I should not treat the availability of an export button as approval for this request.

08 **Nina:** Exactly. Confirm [4] _____ and the proposed recipient. Technical capability does not settle the purpose, scope, or appropriate destination of the information.

09 **Omar:** This is an administrative reporting task. It is not a request between providers to support treatment of an individual patient.

10 **Nina:** That distinction matters for [5] _____. The US standard has exceptions, including provider treatment disclosures and requests. We should apply the actual rule and policy to this context, not an oversimplified slogan.

Conversation continues on the next page.

A full patient list for a counting task

CONVERSATION / CONTINUED

11 **Omar:** Would removing the name column be enough? The export would still contain dates of birth, diagnoses, and free-text visit notes.

12 **Nina:** That is not established [6] _____. Removing one identifier does not prove the extract meets the relevant criteria or eliminates the possibility of linking information back to an individual.

13 **Omar:** Even clinic counts could be revealing if a group is very small or the recipient can combine the report with other information.

14 **Nina:** Review [7] _____ before release. Appropriate aggregation or other controls may be needed, depending on the purpose, context, and applicable review process.

15 **Omar:** Once the output is approved, I will verify the recipient and use our designated channel. I will not attach it to an unapproved personal message.

16 **Nina:** Use an approved [8] _____ method and confirm the destination. Security of the channel matters, but it does not make an excessive or unauthorized disclosure acceptable.

17 **Omar:** I will document the purpose, approved fields or counts, and the decision. That will help prevent a later request from silently expanding the same report.

18 **Nina:** Preserve the relevant [9] _____ and request records under the retention process. The output should remain connected to its authorized purpose rather than becoming an unrestricted reusable patient list.

19 **Omar:** I will hold the extract and ask whether clinic-level counts answer the question. Please review the scope and recipient before we send anything.

20 **Nina:** Good. Complete [10] _____ and scope review before sharing. We can support the operational question without assuming that more personal information produces a better report.

CHECK THE GAPS

1. permitted use / **2.** aggregate report / **3.** role-based access / **4.** export permission / **5.** minimum necessary / **6.** de-identification / **7.** re-identification risk / **8.** secure transfer / **9.** audit log / **10.** recipient verification

Why these answers fit, page 135

F Check accuracy and scope

A full patient list for a counting task

Return to the case

An operations colleague wants to count last month's missed appointments by clinic. The request asks for every patient's name, date of birth, diagnosis, and full visit notes. A clinic-level count may meet the stated purpose, but neither the appropriate data scope nor the requester's export permission has been confirmed. This is an administrative reporting task, not a provider-to-provider treatment request. Analyst Omar and privacy colleague Nina must confirm the purpose, access, output, and approved sharing route before any extract is sent.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which statement about the treatment exception is accurate?

A. The standard has no exceptions under HIPAA.

B. Provider treatment disclosures and requests have an exception to minimum necessary; this case is administrative reporting.

C. Every task performed in a hospital is automatically a treatment disclosure.

D. The exception means all workforce members may export every record.

2. Why review an aggregate report?

A. All aggregate reports are automatically prohibited.

B. Counts always contain full medical notes.

C. Removing names guarantees that no identification is possible.

D. Small groups or combined information may still allow identification.

Answers and explanations, page 136

Listen for the distinction

Switch roles for the proposed output. Distinguish a reviewed aggregate count, export permission, and a protected sharing route; do not call name removal de-identification.

Clarify the purpose before choosing the fields

A privacy question need not sound obstructive. Explain that the requested output should match the task and ask which decision the data must support. Avoid treating internal status, removal of names, or technical access as sufficient evidence that an extract is appropriate. Use the designated privacy process for uncertainty.

F Say it / transfer to a new case

An export button is not permission

An analyst can download a patient table. The requester needs monthly clinic counts, but the requested export includes diagnoses. Recipient authority and output scope are not yet verified.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

capability / purpose / unconfirmed / sharing

01 **Analyst:** The button shows technical [1] _____.

02 **Reviewer:** The report's approved scope is [2] _____.

03 **Analyst:** Clinic-level counts may meet the stated [3] _____.

04 **Reviewer:** Complete recipient and scope checks before [4] _____.

CHECK THE GAPS

1. capability / **2.** unconfirmed / **3.** purpose / **4.** sharing

Why these answers fit, page 136

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "A full patient list for a counting task". Omar: "Even clinic counts could be revealing if a group is very small or the recipient can combine the report with other information." Nina: [missing response] Omar: "Once the output is approved, I will verify the recipient and use our designated channel. I will not attach it to an unapproved personal message." Which response belongs in the gap?

A. Review re-identification risk before release. Appropriate aggregation or other controls may be needed, depending on the purpose, context, and applicable review process.

B. Check role-based access and the specific permissions. A legitimate role can require some information without authorizing every field or every way of transferring it.

C. Clarify the permitted use and the intended output first. We should understand the decision they need to make before choosing fields from the patient record.

D. Exactly. Confirm export permission and the proposed recipient. Technical capability does not settle the purpose, scope, or appropriate destination of the information.

Answers and explanations, page 137

A Read the situation

LESSON 06 / PATIENT EXPERIENCE AND SERVICE RECOVERY

Acknowledge a communication failure, offer an accountable next step, and avoid making a clinical promise.

> **Two calls, still no clear answer**
>
> Patient Leah has called twice to ask when a clinician will discuss a test result. The required identity checks for this call are complete. The call record confirms both messages, but no clinician callback time is confirmed. Leah says this call concerns the missing response, not a current urgent symptom. Administrator Arun can trace the messages, contact the clinical team, and promise his own status update by 3 p.m. He cannot interpret the result, assure Leah that it is normal, or promise a clinical callback at a time the team has not agreed.

People: Leah, Patient; Arun, Administrator.

Read the record

Choose one answer per question using the facts above.

1. What can Arun verify?

A. Both earlier messages appear in the call record.
B. The result is normal.
C. A clinician will definitely call at 3 p.m.
D. Leah has received a clinical explanation.

2. What can Arun promise?

A. A clinician callback at an unagreed time
B. That no further clinical follow-up is needed
C. His own status update by 3 p.m.
D. A diagnosis during this administrative call

3. What does Leah say about the present call?

A. She wants the administrator to prescribe treatment.
B. She has already spoken with the clinician about the result.
C. It concerns the missing response, not a current urgent symptom.
D. She is reporting a confirmed medical emergency.

Answers and explanations, page 138

B Vocabulary in use

CORE TERMS

patient experience
The interactions a patient has with the health-care system.

improve patient experience

acknowledgment
Recognition of a person's concern or the facts of a situation.

offer a specific acknowledgment

communication delay
A gap between needed or expected communication and its occurrence.

investigate a communication delay

contact attempt
An effort to reach a person through a communication channel.

document each contact attempt

status update
Information about the current state of an unresolved matter.

provide a status update

clinical interpretation
Qualified explanation of medical findings and their meaning.

route for clinical interpretation

service recovery
Actions addressing a service failure and its effects on the person affected.

initiate service recovery

empathy statement
Language recognizing another person's experience or difficulty.

use an appropriate empathy statement

call history
The recorded sequence of telephone contacts and messages.

review the call history

callback window
An agreed period during which a return call is expected.

confirm the callback window

resolution commitment
A promise to deliver a specified outcome.

distinguish an update from a resolution commitment

scope of role
The responsibilities and limits of a person's position.

stay within the scope of role

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

patient experience PATIENT EXPERIENCE

improve patient experience

Keep the meaning precise: The interactions a patient has with the health-care system.

call history CALL HISTORY

review the call history

Keep the meaning precise: The recorded sequence of telephone contacts and messages.

B Precision and professional shorthand

EXTEND YOUR RANGE

warm transfer

A handoff connecting the person with the next team while conveying relevant context.

arrange a warm transfer

handoff acceptance

Confirmation that the receiving person or team accepts responsibility for the specified task.

verify handoff acceptance

identity verification

Checking that the person is who they claim to be under the relevant process.

complete identity verification

complaint

An expression of dissatisfaction requiring the applicable response.

document a complaint

de-escalation

Communication intended to reduce tension and support a constructive exchange.

use respectful de-escalation

follow-through

Completion of the action or update that was promised.

demonstrate follow-through

named owner

An identified person accountable for a follow-up action.

assign a named owner

contact preference

A person's preferred appropriate method or timing of communication.

confirm the contact preference

plain language

Wording intended to be readily understood by the audience.

use plain language

grievance pathway

The designated process for formally reviewing specified patient concerns.

explain the grievance pathway

expectation setting

Clarifying what will happen, what is uncertain, and who is responsible.

improve expectation setting

closure confirmation

Checking that the relevant follow-up or resolution has actually occurred.

seek closure confirmation

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

scope of role SCOPE OF ROLE

stay within the scope of role

Keep the meaning precise: The responsibilities and limits of a person's position.

identity verification IDENTITY VERIFICATION

complete identity verification

Keep the meaning precise: Checking that the person is who they claim to be under the relevant process.

C Phrases that move the work forward

Acknowledge the history

I can see both earlier messages in the record.

Apologize specifically

I am sorry you have not received a clear update.

Set a controlled promise

I will update you myself by 3 p.m.

Respect the role

I cannot interpret the result, but I can help get your question to the clinical team.

Keep ownership

You should not have to restart the same explanation at each transfer.

Explain a handoff

I will pass along the earlier call history and confirm that the team received it.

Offer the process

I can explain the patient-concern process if you want the delay formally reviewed.

Read back the plan

I will contact the team now and update you by three.

Recognize the impact

I understand why having to call again is frustrating.

State your action

I will trace the messages and contact the clinical team.

Separate the timing

That is my update time, not yet a confirmed clinician callback time.

Avoid unsupported reassurance

I do not have a basis to tell you the result is normal.

Confirm the channel

Let us check the appropriate contact method on your record.

Use plain language

I am checking who will respond and when, rather than simply moving your message.

Update even when pending

I will contact you at the agreed time even if the clinical response is still unconfirmed.

Confirm closure

We will check that the promised follow-up actually occurred.

Precision in real exchanges

Purpose: State your action
Useful wording: I will trace the messages and contact the clinical team.

Purpose: Set a controlled promise
Useful wording: I will update you myself by 3 p.m.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which opening best addresses Leah's experience?

A. No call probably means the result is normal.

B. You should assume the clinician will call today.

C. I can see both messages, and I am sorry you have had to call again without a clear update.

D. The system has a queue, so there is nothing to discuss.

2. Which promise can Arun make without committing another team?

A. I will close the complaint by three as soon as the clinical team acknowledges receipt.

B. The result will be ready for explanation by three if no further messages arrive.

C. I will update you by three, even if the clinician's callback time is still unconfirmed.

D. The clinical team will explain the result by three because I have forwarded the request.

3. A colleague needs the term for this meaning: Language recognizing another person's experience or difficulty. Which term fits?

A. empathy statement

B. status update

C. handoff acceptance

D. de-escalation

D Choose the response that fits / continued

4. Rebuild the exchange from "Two calls, still no clear answer". Leah: "Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything." Arun: [missing response] Leah: "What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this." Which response belongs in the gap?

A. I will remain the named owner for this administrative follow-up. The clinical team explains the result; I will keep the communication commitment we have agreed.

B. I will still call. That follow-through is my responsibility. You will hear the actual status and next action, even if the clinical timing remains unresolved.

C. I cannot give that clinical interpretation. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.

D. I will give you a status update myself by three today. I will tell you what the team has confirmed and what is still outstanding.

Answers and explanations, page 138

> **One useful distinction**
>
> An apology for a communication delay does not require interpreting the test result. A status update promises information about progress, not a clinical conclusion. Explain the difference in ordinary language, particularly when the patient has already waited through earlier unsuccessful contacts.

E Conversation

Two calls, still no clear answer

Two calls, still no clear answer

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

clinical interpretation / named owner / status update / contact preference / follow-through / grievance pathway / callback window / communication delay / handoff acceptance / call history

01 **Leah:** This is my third call about the result. I have answered the identity questions again, but I still do not know who will call me.

02 **Arun:** I can see both earlier messages in the [1] _____. I am sorry you have had to call again. I will trace where those messages went.

03 **Leah:** This is about the missing response, not a new urgent symptom. Last time I was told the message had been passed on.

04 **Arun:** That [2] _____ needs follow-up, not another unexplained transfer. I will contact the clinical team and check who is taking responsibility for your question.

05 **Leah:** Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything.

06 **Arun:** I cannot give that [3] _____. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.

07 **Leah:** What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this.

08 **Arun:** I will give you a [4] _____ myself by three today. I will tell you what the team has confirmed and what is still outstanding.

09 **Leah:** So should I keep three o'clock free for the clinician? I do not want to miss the explanation if that is the time.

10 **Arun:** We do not have a clinician's [5] _____ yet. Three is my update time; I will not describe it as a medical appointment that has not been agreed.

Conversation continues on the next page.

Two calls, still no clear answer

CONVERSATION / CONTINUED

11 **Leah:** Please do not make me explain the first two calls to another person. They are already in the record.

12 **Arun:** I will include that history and seek [6] _____, so a responsible team member accepts the follow-up. Forwarding your message alone is not enough.

13 **Leah:** Use the contact method already on my record. I do not want details sent to a different number during another handover.

14 **Arun:** I will confirm that [7] _____ through our usual checks. We will use the agreed route rather than assume a new destination is acceptable.

15 **Leah:** I also want the delay reviewed. Will raising a complaint distract from getting the result explained?

16 **Arun:** Those can proceed separately. I can explain the [8] _____ for the communication concern while continuing to seek the clinical response.

17 **Leah:** And if you have not got a clinician's time by three? That is usually when the promised update disappears.

18 **Arun:** I will still call. That [9] _____ is my responsibility. You will hear the actual status and next action, even if the clinical timing remains unresolved.

19 **Leah:** All right. Include the earlier messages, find out who is handling it, and contact me by three whether or not the time is confirmed.

20 **Arun:** I will remain the [10] _____ for this administrative follow-up. The clinical team explains the result; I will keep the communication commitment we have agreed.

CHECK THE GAPS

1. call history / **2.** communication delay / **3.** clinical interpretation / **4.** status update / **5.** callback window / **6.** handoff acceptance / **7.** contact preference / **8.** grievance pathway / **9.** follow-through / **10.** named owner

Why these answers fit, page 139

F Check accuracy and scope

Two calls, still no clear answer

Return to the case

Patient Leah has called twice to ask when a clinician will discuss a test result. The required identity checks for this call are complete. The call record confirms both messages, but no clinician callback time is confirmed. Leah says this call concerns the missing response, not a current urgent symptom. Administrator Arun can trace the messages, contact the clinical team, and promise his own status update by 3 p.m. He cannot interpret the result, assure Leah that it is normal, or promise a clinical callback at a time the team has not agreed.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Leah asks whether silence means the result is normal. What should Arun say?

A. I cannot infer that or interpret the result; I will route the question to the clinical team.

B. Yes, silence reliably means normal.

C. The length of the delay determines the diagnosis.

D. A booking administrator can confirm normality from the call log.

2. What closes the administrative handoff?

A. Confirm that a clinical team member accepts the follow-up and keep the promised patient update.

B. Mark the task complete immediately after forwarding a message.

C. Remove the earlier contacts so only the latest call remains.

D. Promise a new clinician time without checking it.

Answers and explanations, page 141

Listen for the distinction

Switch roles for turns 9-20. Distinguish Arun's three-o'clock update from an unconfirmed clinician callback and seek explicit acceptance of clinical follow-up.

Acknowledge the experience before explaining the process

A patient who has already called twice needs recognition and a dependable next step, not another unexplained transfer. State what you can personally do and when you will update them. Keep empathy separate from unsupported reassurance about a result or a clinical decision outside your role.

F Say it / transfer to a new case

Keep the promised update

At 3 p.m., the clinical team has acknowledged the request but has not confirmed a callback time. The administrator promised a status update at 3 p.m.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

acknowledged / update / interpretation / unconfirmed

01 **Administrator:** The clinical team has [1] _____ the request.

02 **Patient:** Is the callback time confirmed?" Administrator: "It is still [2] _____.

03 **Administrator:** This call honors my promised [3] _____.

04 **Patient:** Does receipt establish a result?" Administrator: "No clinical [4] _____ follows from receipt alone.

CHECK THE GAPS

1. acknowledged / **2.** unconfirmed / **3.** update / **4.** interpretation

Why these answers fit, page 141

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Two calls, still no clear answer". Leah: "Use the contact method already on my record. I do not want details sent to a different number during another handover." Arun: [missing response] Leah: "I also want the delay reviewed. Will raising a complaint distract from getting the result explained?" Which response belongs in the gap?

A. I will include that history and seek handoff acceptance, so a responsible team member accepts the follow-up. Forwarding your message alone is not enough.

B. I cannot give that clinical interpretation. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.

C. That communication delay needs follow-up, not another unexplained transfer. I will contact the clinical team and check who is taking responsibility for your question.

D. I will confirm that contact preference through our usual checks. We will use the agreed route rather than assume a new destination is acceptable.

Answers and explanations, page 142

Review 3 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "HIPAA, Privacy, and Information Governance" and "Patient Experience and Service Recovery". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Protected health information maintained or transmitted electronically. Which term fits?

A. electronic PHI
B. secure transfer
C. purpose limitation
D. permitted use

2. A colleague needs the term for this meaning: An agreed period during which a return call is expected. Which term fits?

A. complaint
B. callback window
C. patient experience
D. warm transfer

3. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "A simple diagram of the referral-to-booking route may help the teams see where information changed or responsibility became unclear." Asha: [missing response] Ben: "Staff are worried this meeting will start by naming someone to blame. How do we get an honest account of the workarounds without losing accountability?" Which response belongs in the gap?

A. A just culture supports both learning and fair accountability. Ask what happened and why the process allowed it, while preserving the responsibilities appropriate to each role.
B. Use a process map grounded in actual practice. Include the handoffs and workarounds people really use, not just the ideal sequence written in the procedure.
C. Separate implementation evidence from effectiveness evidence. A revised form or attendance record proves an activity occurred; it does not automatically prove that bookings became more accurate.
D. Build the event chronology from those records and relevant accounts. Keep confirmed facts distinct from assumptions, especially where the timing or handoff is unclear.

Review 3 / Meaning and context / continued

4. Rebuild the exchange from "Two calls, still no clear answer". Leah: "And if you have not got a clinician's time by three? That is usually when the promised update disappears." Arun: [missing response] Leah: "All right. Include the earlier messages, find out who is handling it, and contact me by three whether or not the time is confirmed." Which response belongs in the gap?

A. I will remain the named owner for this administrative follow-up. The clinical team explains the result; I will keep the communication commitment we have agreed.

B. I cannot give that clinical interpretation. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.

C. I will still call. That follow-through is my responsibility. You will hear the actual status and next action, even if the clinical timing remains unresolved.

D. I will give you a status update myself by three today. I will tell you what the team has confirmed and what is still outstanding.

Answers and explanations, page 159

A Read the situation

LESSON 07 / POPULATION HEALTH AND CARE COORDINATION

Hand over unresolved care-transition tasks with verified status, ownership, and a clear escalation condition.

> **Transport pending, but whose next action?**
>
> Outgoing coordinator Rachel hands over a discharge coordination case to Imran. The note says transport pending. The transport service has acknowledged the request but has not assigned a vehicle or pickup time. A home-health referral has been received, but its first visit is unconfirmed. Rachel will finish her shift at noon. Imran is to confirm responsibility for both follow-ups and give the care team a status update at 2 p.m. Clinical discharge readiness remains for the qualified team to decide; the administrative handoff does not authorize departure.

People: Rachel, Outgoing coordinator; Imran, Incoming coordinator.

Read the record

Choose one answer per question using the facts above.

1. What has the transport service confirmed?

A. Clinical discharge readiness
B. Receipt of the request only
C. A vehicle and pickup time
D. Completion of the journey

2. What is the home-health status?

A. The transport provider has accepted clinical responsibility.
B. The referral was received; the first visit is unconfirmed.
C. A first visit is booked at noon.
D. The service has already visited the patient.

3. What does the 2 p.m. commitment mean?

A. The patient is authorized to depart at two.
B. A vehicle is guaranteed to arrive at two.
C. The home-health visit is confirmed for two.
D. Imran will give the care team a status update.

Answers and explanations, page 143

B Vocabulary in use

CORE TERMS

population health
Work addressing health outcomes and needs across a defined group.

define the population-health cohort

care coordination
Organization of activities and information across people involved in a person's care.

strengthen care coordination

transition of care
Movement between care settings, providers, or stages.

support a transition of care

discharge coordination
Organization of practical arrangements related to leaving a care setting.

document discharge coordination

care plan
The agreed account of needs, goals, and responsibilities for care.

communicate the care plan

care gap
A difference between needed or planned care and care received.

identify a care gap

patient panel
A defined group of patients associated with a team or service.

review the patient panel

risk stratification
Grouping by assessed risk to support appropriate planning.

review risk-stratification criteria

social needs
Practical social circumstances affecting a person's ability to access or manage care.

assess reported social needs

transport barrier
A transport-related obstacle to reaching or leaving care.

resolve a transport barrier

non-emergency medical transport
Transport for medical access when emergency transport is not required, under applicable arrangements.

coordinate non-emergency medical transport

dispatch confirmation
Verification that the transport provider has made specified dispatch arrangements.

request dispatch confirmation

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

population health POPULATION HEALTH

define the population-health cohort

Keep the meaning precise: Work addressing health outcomes and needs across a defined group.

care gap CARE GAP

identify a care gap

Keep the meaning precise: A difference between needed or planned care and care received.

B Precision and professional shorthand

EXTEND YOUR RANGE

pickup window

The period during which a pickup is expected.

verify the pickup window

receiving provider

The professional or organization expected to take the next care role.

contact the receiving provider

home-health referral

A request for an appropriate service delivered in the home setting.

track a home-health referral

service acceptance

Confirmation that a receiving service will take the specified referral or task.

distinguish receipt from service acceptance

first-visit confirmation

Verification of the initial service visit's agreed arrangements.

obtain first-visit confirmation

medication reconciliation

Qualified comparison and clarification of medication information across care transitions.

route medication-reconciliation questions

follow-up appointment

A subsequent visit arranged for continued review or care.

verify the follow-up appointment

community resource

A local service supporting practical or health-related needs.

verify community-resource availability

handoff checklist

A structured aid for communicating required transition information.

complete the handoff checklist

closed-loop communication

An exchange including acknowledgment and confirmation of understanding.

use closed-loop communication

contingency plan

An agreed response if the expected arrangement cannot occur.

confirm the contingency plan

care-team update

A status communication to the relevant people coordinating care.

provide a care-team update

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

dispatch confirmation DISPATCH CONFIRMATION

request dispatch confirmation

Keep the meaning precise: Verification that the transport provider has made specified dispatch arrangements.

first-visit confirmation FIRST-VISIT CONFIRMATION

obtain first-visit confirmation

Keep the meaning precise: Verification of the initial service visit's agreed arrangements.

C Phrases that move the work forward

Clarify pending

Is transport awaiting acceptance, vehicle assignment, or a pickup time?

Name the next owner

I will take responsibility for the transport follow-up after noon.

Set the check

I will update the care team at 2 p.m.

Verify suitability

The relevant qualified team must confirm the required transport arrangements.

Preserve the distinction

Received does not necessarily mean accepted for service.

Record the response

Document the provider's exact status and the time of contact.

Read back ownership

You are handing over both follow-ups, and I am accepting them.

Avoid duplicate work

Check the existing request before creating another transport booking.

State the confirmed step

The provider acknowledged the request but has not assigned a vehicle.

Separate the service

Home health received the referral; the first visit is not yet confirmed.

Avoid a departure promise

That update time is not authorization for the patient to leave.

Check the next visit

Please confirm the receiving service's first-visit plan.

Escalate a barrier

If arrangements remain unresolved, use the agreed escalation route.

Confirm the fallback

What is the approved contingency if transport cannot be arranged?

Keep the patient informed

Share the confirmed practical arrangements through the appropriate care-team process.

Close the handoff

Record acknowledgment, next actions, and the next update time.

Precision in real exchanges

Purpose: Separate the service
Useful wording: Home health received the referral; the first visit is not yet confirmed.

Purpose: Set the check
Useful wording: I will update the care team at 2 p.m.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which handoff statement is most actionable?

A. Transport is booked because the provider replied.

B. The patient can leave at two because an update is due.

C. Transport acknowledged the request; no vehicle or time is assigned; Imran owns follow-up and a 2 p.m. update.

D. Transport pending; someone should check later.

2. Which next check goes beyond a home-health referral receipt?

A. Whether the sender saved a copy of the original referral email

B. Whether the referral title contains the expected service name

C. Whether transport has acknowledged its separate request

D. Whether the service accepts responsibility and what first-visit arrangements are confirmed

3. A colleague needs the term for this meaning: Organization of practical arrangements related to leaving a care setting. Which term fits?

A. discharge coordination

B. home-health referral

C. handoff checklist

D. social needs

D Choose the response that fits / continued

4. Rebuild the exchange from "Transport pending, but whose next action?". Rachel: "The home-health referral has also been received, but we do not have a confirmed first visit. It belongs on the handoff as a separate open item." Imran: [missing response] Rachel: "Good. Those arrangements support discharge planning, but neither of us should turn the administrative status into a decision about clinical readiness." Which response belongs in the gap?

A. I will confirm the contingency plan with the relevant team. Suitability and authorization must be established through the proper route before an alternative is reported as available.

B. Agreed. The transition of care remains coordinated with the qualified team. We will communicate the practical facts without substituting our judgment for the clinical discharge process.

C. Let us identify the transport barrier. Has the provider received the request, accepted it, assigned a vehicle, or confirmed a pickup time? Those are different stages.

D. I will verify service acceptance and the first-visit plan. The fact that a referral arrived does not tell the care team when the next service will begin.

Answers and explanations, page 143

One useful distinction

Request received, service accepted, vehicle assigned, and pickup confirmed describe different steps. Use the provider's actual response. A transport arrangement also does not establish clinical readiness for discharge; that decision remains with the qualified team under the relevant process.

E Conversation

Transport pending, but whose next action?

Transport pending, but whose next action?

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

care-team update / pickup window / transition of care / handoff checklist / closed-loop communication / service acceptance / first-visit confirmation / contingency plan / transport barrier / dispatch confirmation

01 **Rachel:** Before I finish at noon, I need to hand over this discharge coordination case. The note says transport pending, but that wording leaves too much unclear.

02 **Imran:** Let us identify the [1] _____. Has the provider received the request, accepted it, assigned a vehicle, or confirmed a pickup time? Those are different stages.

03 **Rachel:** The provider acknowledged receipt. No vehicle or pickup time has been assigned, so we should not describe the journey as booked.

04 **Imran:** I will request [2] _____ through the existing request. I will check what remains unresolved instead of creating another booking and risking two competing requests.

05 **Rachel:** The home-health referral has also been received, but we do not have a confirmed first visit. It belongs on the handoff as a separate open item.

06 **Imran:** I will verify [3] _____ and the first-visit plan. The fact that a referral arrived does not tell the care team when the next service will begin.

E Conversation / continued

07 **Rachel:** Good. Those arrangements support discharge planning, but neither of us should turn the administrative status into a decision about clinical readiness.

08 **Imran:** Agreed. The [4] _____ remains coordinated with the qualified team. We will communicate the practical facts without substituting our judgment for the clinical discharge process.

09 **Rachel:** I will give you the request references and the last contact times. You should not have to reconstruct everything from a note that simply says pending.

10 **Imran:** I will use the [5] _____ to capture the open actions. Let us record the provider responses exactly and separate verified information from expected next steps.

Conversation continues on the next page.

Transport pending, but whose next action?

CONVERSATION / CONTINUED

11 **Rachel:** You will own both follow-ups after noon. I had planned a care-team update at two, even if the providers had not confirmed the arrangements by then.

12 **Imran:** I accept that responsibility and will give the [6] _____ at two. I will state what is confirmed, what remains open, and any escalation underway rather than wait silently for a complete answer.

13 **Rachel:** If transport is still unresolved, the team needs to know early enough to use the appropriate escalation route. We should not imply that two is a pickup time.

14 **Imran:** Correct. Any [7] _____ must come from the provider's actual arrangements. The update deadline is our communication commitment, not a transport guarantee.

15 **Rachel:** If this transport cannot be arranged, check the approved alternative with the team. Please do not book a different type before suitability and authority are confirmed.

16 **Imran:** I will confirm the [8] _____ with the relevant team. Suitability and authorization must be established through the proper route before an alternative is reported as available.

17 **Rachel:** For home health, please distinguish a general acknowledgment from an actual start date. The receiving team and patient need an accurate expectation.

18 **Imran:** I will ask for [9] _____ and document the answer. If it is still unknown, I will retain that status and identify who is following up.

Transport pending, but whose next action? / continued

19 **Rachel:** Before I leave, read back the two follow-ups and the update time. I want to make sure neither task remains assigned to someone whose shift has ended.

20 **Imran:** I own transport and home-health follow-up after noon, with a two o'clock status update. That [10] _____ confirms the tasks, not clinical readiness or departure. I will record the agreed actions and keep the care team informed.

CHECK THE GAPS

1. transport barrier / **2.** dispatch confirmation / **3.** service acceptance / **4.** transition of care / **5.** handoff checklist / **6.** care-team update / **7.** pickup window / **8.** contingency plan / **9.** first-visit confirmation / **10.** closed-loop communication

Why these answers fit, page 144

F Check accuracy and scope

Transport pending, but whose next action?

Return to the case

Outgoing coordinator Rachel hands over a discharge coordination case to Imran. The note says transport pending. The transport service has acknowledged the request but has not assigned a vehicle or pickup time. A home-health referral has been received, but its first visit is unconfirmed. Rachel will finish her shift at noon. Imran is to confirm responsibility for both follow-ups and give the care team a status update at 2 p.m. Clinical discharge readiness remains for the qualified team to decide; the administrative handoff does not authorize departure.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which statement preserves the clinical boundary?

A. The qualified team determines discharge readiness; administrative coordination supports that decision.

B. A completed handoff authorizes departure in every case.

C. A vehicle assignment replaces the clinical discharge process.

D. An administrator can resolve all medication questions from a transport note.

2. What closes the ownership transfer?

A. Both coordinators assume the other will call.

B. Imran acknowledges the tasks and the agreed next actions are documented.

C. Rachel copies Imran on an email without confirming receipt.

D. The note is shortened to pending.

Answers and explanations, page 146

Listen for the distinction

Switch roles for turns 11-20. Make Imran's acceptance of both follow-ups explicit, with the two-o'clock care-team update unchanged.

Pending needs a noun, an owner, and a next check

A short note is useful only when the next person can act on it. Explain what is awaiting confirmation, who holds the next action, and when a response is needed. Repeat the agreed responsibilities so that a shift change does not turn an active request into an unowned task.

F Say it / transfer to a new case

Acknowledged, not assigned

A transport provider confirms receipt of a request but says no vehicle is assigned. The coordinator will check again at 1 p.m.; this is not a pickup promise.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

pickup / check / acknowledged / unassigned

01 **Coordinator:** The request was [1] _____.

02 **Colleague:** A vehicle remains [2] _____.

03 **Coordinator:** One o'clock is the next [3] _____.

04 **Colleague:** Do not present it as a confirmed [4] _____.

CHECK THE GAPS

1. acknowledged / **2.** unassigned / **3.** check / **4.** pickup

Why these answers fit, page 146

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Transport pending, but whose next action?". Rachel: "If transport is still unresolved, the team needs to know early enough to use the appropriate escalation route. We should not imply that two is a pickup time." Imran: [missing response] Rachel: "If this transport cannot be arranged, check the approved alternative with the team. Please do not book a different type before suitability and authority are confirmed." Which response belongs in the gap?

A. Correct. Any pickup window must come from the provider's actual arrangements. The update deadline is our communication commitment, not a transport guarantee.

B. I will request dispatch confirmation through the existing request. I will check what remains unresolved instead of creating another booking and risking two competing requests.

C. Agreed. The transition of care remains coordinated with the qualified team. We will communicate the practical facts without substituting our judgment for the clinical discharge process.

D. Let us identify the transport barrier. Has the provider received the request, accepted it, assigned a vehicle, or confirmed a pickup time? Those are different stages.

Answers and explanations, page 147

A Read the situation

LESSON 08 / EXECUTIVE DASHBOARDS AND BOARD UPDATES

Explain a performance change with comparable definitions and turn a dashboard into a bounded decision request.

> **A shorter wait, or a different reporting window?**
>
> A board chart compares last month's median waiting time of 40 minutes across 200 visits with this week's median of 30 minutes across 50 visits. The clinic mix and measurement consistency have not been checked. The headline claims a proven 25% improvement caused by a staffing change. Analyst Jo and executive sponsor Malik must distinguish arithmetic from evidence of sustained improvement or causation. The proposed board request is approval of a four-week validation review, not a claim that staffing changes are already proven effective.

People: Jo, Performance analyst; Malik, Executive sponsor.

Read the record

Choose one answer per question using the facts above.

1. What changed between the compared data sets?

A. Only the font used in the chart
B. Nothing; both use the same monthly population
C. The outcome from waiting time to revenue
D. The reporting period and visit count, with clinic mix still unchecked

2. What does 40 to 30 minutes establish arithmetically?

A. Proof that every patient's wait fell by ten minutes
B. Proof that the staffing change caused the reduction
C. A confirmed sustained improvement across comparable months
D. A ten-minute, 25% reduction between the reported medians

3. What decision is proposed?

A. Approve a four-week validation review
B. Declare the staffing change conclusively effective
C. Approve unlimited additional staffing
D. Eliminate measurement checks because the chart improved

Answers and explanations, page 148

B Vocabulary in use

CORE TERMS

executive dashboard
A concise display of measures supporting leadership oversight and decisions.

review the executive dashboard

metric definition
The specified calculation, population, and interpretation of a measure.

document the metric definition

measurement window
The interval over which observations are collected for a measure.

keep the measurement window consistent

denominator
The reference quantity used below the line in a defined ratio.

state the denominator

mean
The total of observed values divided by their number.

distinguish the mean from the median

case mix
The composition of patients or cases with characteristics relevant to comparison.

assess case-mix differences

key performance indicator
A selected measure of important performance.

define the key performance indicator

reporting period
The time interval covered by a reported result.

align reporting periods

numerator
The quantity counted above the line in a defined ratio.

verify the numerator

median
The middle ordered observation, or mean of the two middle observations for an even count.

report median waiting time

percentile
A value indicating a specified position in an ordered distribution.

examine the ninetieth percentile

stratification
Separating data into defined groups for analysis.

use clinic-level stratification

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

executive dashboard EXECUTIVE DASHBOARD

review the executive dashboard

Keep the meaning precise: A concise display of measures supporting leadership oversight and decisions.

numerator NUMERATOR

verify the numerator

Keep the meaning precise: The quantity counted above the line in a defined ratio.

B Precision and professional shorthand

EXTEND YOUR RANGE

data lineage

The traceable path from source data to a reported result.

verify data lineage

completeness check

Assessment of whether expected records or fields are present.

perform a completeness check

run chart

A time-ordered display of a measure used to examine patterns.

review the run chart

special-cause variation

Variation associated with an identifiable change outside usual process behavior in context.

investigate potential special-cause variation

relative change

A difference expressed as a proportion of the reference value.

calculate relative change

data-quality caveat

A stated limitation affecting confidence in a reported result.

retain a data-quality caveat

refresh date

The time a data display was last updated.

show the refresh date

like-for-like comparison

A comparison using sufficiently consistent populations, definitions, and periods.

build a like-for-like comparison

common-cause variation

Variation inherent in the current process under the relevant statistical model.

assess common-cause variation

causal attribution

A conclusion that a specified factor produced an observed effect.

qualify causal attribution

absolute change

The numerical difference in the original measurement units.

report absolute change

decision request

The specific action a decision-making body is asked to authorize.

state the decision request

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

stratification STRATIFICATION

use clinic-level stratification

Keep the meaning precise: Separating data into defined groups for analysis.

run chart RUN CHART

review the run chart

Keep the meaning precise: A time-ordered display of a measure used to examine patterns.

C Phrases that move the work forward

Describe the figures

The reported medians are forty minutes last month and thirty this week.

State the arithmetic

That is ten minutes lower, or 25% relative to forty.

Limit the conclusion

The comparison does not yet establish sustained improvement.

Name the changed basis

The periods and visit counts differ, and clinic mix is unchecked.

Check the definition

Are arrival and service-start times measured consistently?

Preserve the statistic

Both figures are medians; do not describe them as every patient's wait.

Request comparable data

Rebuild the comparison on aligned periods and definitions.

Separate association

The timing of the staffing change does not prove it caused the result.

Show the limitation

Place the comparability caveat beside the headline.

Verify the source

Trace the measure to its underlying records and calculation.

Look beyond one point

Review the time pattern rather than declaring a trend from two unmatched values.

State the ask

Approve a four-week review to validate the comparison.

Bound the authority

This request does not authorize unlimited staffing changes.

Name the owner

The performance team will own the validation and report-back.

Keep the finding useful

Report the observed values while marking the conclusion provisional.

Close with evidence

Return with comparable results, limitations, and the next decision needed.

Precision in real exchanges

Purpose: Name the changed basis
Useful wording: The periods and visit counts differ, and clinic mix is unchecked.

Purpose: Check the definition
Useful wording: Are arrival and service-start times measured consistently?

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which headline accurately describes the observed 40-to-30-minute comparison?

A. Reported median is ten minutes lower; period, mix, and causal comparability remain under review.

B. Waiting times fell ten percentage points after the staffing change.

C. The typical patient is proven to wait 25% less because the newer sample is smaller.

D. Staffing delivered a sustained 25% improvement, with data validation to follow.

2. What is the relative reduction between the reported values?

A. 75%

B. 25%

C. 10%

D. 33.3%

3. A colleague needs the term for this meaning: The time interval covered by a reported result. Which term fits?

A. completeness check

B. mean

C. reporting period

D. relative change

D Choose the response that fits / continued

4. Rebuild the exchange from "A shorter wait, or a different reporting window?". Malik: "We should also be careful with the word average. Both values are medians, and people may hear that every patient's wait fell by ten minutes." Jo: [missing response] Malik: "One clinic may have contributed a larger share of visits this week. If clinics have different waiting patterns, that could affect the combined result." Which response belongs in the gap?

A. Keep the data-quality caveat beside the finding. We can report the observed values honestly and still offer a practical next step that will make the next decision better informed.

B. The reporting periods differ. Forty minutes is last month's median across two hundred visits; thirty is this week's median across fifty. We have not checked the clinic mix.

C. Retain median in the label. It is a group summary, not a statement about each person's experience. A different summary statistic could tell a different part of the story.

D. We will verify data lineage, completeness, and the refresh date. That should show what is included, what is missing, and whether the displayed figures reflect the intended periods.

Answers and explanations, page 148

One useful distinction

The reported median fell by ten minutes, or 25% relative to forty. That arithmetic does not show that each patient waited ten minutes less. Nor does it establish a like-for-like trend or prove that a staffing change caused the difference.

E Conversation

A shorter wait, or a different reporting window?

A shorter wait, or a different reporting window?

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

data-quality caveat / Causal attribution / metric definition / decision request / reporting periods / median / relative change / run chart / data lineage / case mix

01 **Malik:** The board chart says waiting time improved by twenty-five percent because of the staffing change. Before I use that headline, are the two figures comparable?

02 **Jo:** The [1] _____ differ. Forty minutes is last month's median across two hundred visits; thirty is this week's median across fifty. We have not checked the clinic mix.

03 **Malik:** The arithmetic is still a ten-minute reduction, relative to forty. I want to preserve that fact without suggesting it answers every other question.

04 **Jo:** Yes. The [2] _____ is twenty-five percent between the reported values. That does not establish a sustained operational improvement or explain why the difference occurred.

05 **Malik:** We should also be careful with the word average. Both values are medians, and people may hear that every patient's wait fell by ten minutes.

06 **Jo:** Retain [3] _____ in the label. It is a group summary, not a statement about each person's experience. A different summary statistic could tell a different part of the story.

E Conversation / continued

07 **Malik:** One clinic may have contributed a larger share of visits this week. If clinics have different waiting patterns, that could affect the combined result.

08 **Jo:** Check the [4] _____ and use appropriate clinic-level comparisons. We should not assume the smaller weekly sample has the same composition as the previous month's visits.

09 **Malik:** Do both extracts start the clock at the same event? Arrival and completed registration would give us different waiting intervals.

10 **Jo:** Exactly. Confirm the [5] _____ and trace the calculation. A familiar dashboard label does not prove that the underlying start and end events are identical.

Conversation continues on the next page.

A shorter wait, or a different reporting window?

CONVERSATION / CONTINUED

11 **Malik:** Can the team reconstruct the data from source records? I want a repeatable calculation rather than a number copied between presentations.

12 **Jo:** We will verify [6] _____, completeness, and the refresh date. That should show what is included, what is missing, and whether the displayed figures reflect the intended periods.

13 **Malik:** The staffing change happened before this week's result, but other factors may have changed too. We should not equate timing with proof.

14 **Jo:** Correct. [7] _____ needs more than these two values. We can describe the timing without presenting the staffing explanation as established.

15 **Malik:** A series over comparable periods would be more informative than selecting one month and one week. We should look at the broader pattern.

16 **Jo:** A [8] _____ on a consistent basis can help. It should support investigation of the pattern, not serve as a visual shortcut around checking definitions and context.

17 **Malik:** For the board, the immediate request is a four-week validation review. It is not a proposal for unlimited staffing changes or a declaration that the intervention succeeded.

18 **Jo:** Put that [9] _____ up front, with an owner and report-back. The directors should know what they are being asked to authorize before interpreting the chart.

19 **Malik:** I will remove the proven-improvement claim and ask for the four-week review. Put the comparability caveat beside the figures, where the board will see it.

20 **Jo:** Keep the [10] _____ beside the finding. We can report the observed values honestly and still offer a practical next step that will make the next decision better informed.

CHECK THE GAPS

1. reporting periods / **2.** relative change / **3.** median / **4.** case mix / **5.** metric definition / **6.** data lineage / **7.** Causal attribution / **8.** run chart / **9.** decision request / **10.** data-quality caveat

Why these answers fit, page 149

F Check accuracy and scope

A shorter wait, or a different reporting window?

Return to the case

A board chart compares last month's median waiting time of 40 minutes across 200 visits with this week's median of 30 minutes across 50 visits. The clinic mix and measurement consistency have not been checked. The headline claims a proven 25% improvement caused by a staffing change. Analyst Jo and executive sponsor Malik must distinguish arithmetic from evidence of sustained improvement or causation. The proposed board request is approval of a four-week validation review, not a claim that staffing changes are already proven effective.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which validation task is most relevant?

A. Remove the earlier month because it looks worse.
B. Assume fifty visits always represent the same mix as two hundred.
C. Align time periods, definitions, and clinic composition, then review the underlying data.
D. Replace the median with the smallest wait to strengthen the headline.

2. Which board request matches the brief?

A. Approve the four-week validation review and require a report-back on comparable results.
B. End the review because the arithmetic is correct.
C. Approve any future staffing plan without further evidence.
D. Treat the chart as a completed causal evaluation.

Answers and explanations, page 151

Listen for the distinction

Switch roles for the conclusion. State ten minutes or 25% lower, with clinic mix, measurement consistency, and causation still unresolved.

Explain the comparison before celebrating the result

A numerical reduction can be real in the reported values while the operational conclusion remains uncertain. Name the periods, populations, and definitions. Then give decision-makers a specific next step, rather than using caveats either to conceal a weak claim or to avoid making any useful request.

F Say it / transfer to a new case

Minutes and percentages are different

On an otherwise comparable basis in a separate exercise, median waiting time changes from 50 to 40 minutes. The figures alone do not identify the cause or every patient's experience.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

medians / 20% / ten / unestablished

01 **Analyst:** The absolute reduction is [1] _____ minutes.

02 **Reviewer:** Relative to fifty, the reduction is [2] _____.

03 **Analyst:** These are group [3] _____.

04 **Reviewer:** The cause remains [4] _____ from these figures alone.

CHECK THE GAPS

1. ten / **2.** 20% / **3.** medians / **4.** unestablished

Why these answers fit, page 151

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "A shorter wait, or a different reporting window?". Malik: "The staffing change happened before this week's result, but other factors may have changed too. We should not equate timing with proof." Jo: [missing response] Malik: "A series over comparable periods would be more informative than selecting one month and one week. We should look at the broader pattern." Which response belongs in the gap?

A. Check the case mix and use appropriate clinic-level comparisons. We should not assume the smaller weekly sample has the same composition as the previous month's visits.

B. Yes. The relative change is twenty-five percent between the reported values. That does not establish a sustained operational improvement or explain why the difference occurred.

C. Exactly. Confirm the metric definition and trace the calculation. A familiar dashboard label does not prove that the underlying start and end events are identical.

D. Correct. Causal attribution needs more than these two values. We can describe the timing without presenting the staffing explanation as established.

Answers and explanations, page 152

Review 4 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Population Health and Care Coordination" and "Executive Dashboards and Board Updates". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Movement between care settings, providers, or stages. Which term fits?

A. transition of care
B. receiving provider
C. community resource
D. risk stratification

2. A colleague needs the term for this meaning: The middle ordered observation, or mean of the two middle observations for an even count. Which term fits?

A. median
B. data lineage
C. executive dashboard
D. special-cause variation

3. Rebuild the exchange from "Two calls, still no clear answer". Leah: "So should I keep three o'clock free for the clinician? I do not want to miss the explanation if that is the time." Arun: [missing response] Leah: "Please do not make me explain the first two calls to another person. They are already in the record." Which response belongs in the gap?

A. I will give you a status update myself by three today. I will tell you what the team has confirmed and what is still outstanding.
B. I cannot give that clinical interpretation. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.
C. I can see both earlier messages in the call history. I am sorry you have had to call again. I will trace where those messages went.
D. We do not have a clinician's callback window yet. Three is my update time; I will not describe it as a medical appointment that has not been agreed.

Review 4 / Meaning and context / continued

4. Rebuild the exchange from "A shorter wait, or a different reporting window?". Malik: "For the board, the immediate request is a four-week validation review. It is not a proposal for unlimited staffing changes or a declaration that the intervention succeeded." Jo: [missing response] Malik: "I will remove the proven-improvement claim and ask for the four-week review. Put the comparability caveat beside the figures, where the board will see it." Which response belongs in the gap?

A. Exactly. Confirm the metric definition and trace the calculation. A familiar dashboard label does not prove that the underlying start and end events are identical.

B. Put that decision request up front, with an owner and report-back. The directors should know what they are being asked to authorize before interpreting the chart.

C. Yes. The relative change is twenty-five percent between the reported values. That does not establish a sustained operational improvement or explain why the difference occurred.

D. Check the case mix and use appropriate clinic-level comparisons. We should not assume the smaller weekly sample has the same composition as the previous month's visits.

Answers and explanations, page 162

Language for the next case

Language assistance belongs in the appointment plan

preferred language

Preferred language identifies the language the patient wants to use for communication; basic English conversation does not establish comfort with medical discussions.

Amira: Thank you for telling me. What is your preferred language for the visit, including any dialect we should specify?

qualified interpreter

A qualified interpreter provides accurate, impartial spoken communication with appropriate language and specialist skills.

Amira: We can arrange a qualified interpreter through our service. You will not be charged for that support at this clinic.

remote interpreting

Remote interpreting connects an interpreter by an approved telephone or video service rather than requiring physical attendance.

Amira: We can request remote interpreting. I will confirm availability and the clinic's setup before describing it as booked.

confidentiality

Confidentiality concerns protecting the patient's information; interpretation should not require discussing private matters in an exposed setting.

Amira: I will note that concern and arrange the appropriate setting. Your confidentiality matters during the connection as well as during the consultation.

communication need

Communication need records the support required for effective understanding, rather than treating a nod as proof of comprehension.

Amira: Then please say when you need something repeated. I will note the communication need so the team can allow for questions.

confirmation

Confirmation means the requested support has actually been arranged; a request alone is not that completed status.

Amira: It will distinguish the visit booking from the interpreter confirmation. If support is still pending, I will explain the next step.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Language assistance belongs in the appointment plan

A patient booking a nonurgent visit says medical discussions in English are difficult. This fictional clinic offers qualified interpreters without charging the patient. The registrar can arrange support but cannot explain clinical consent or substitute for the clinician.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

preferred language / confidentiality / remote interpreting / communication need / confirmation / qualified interpreter

01 **Huy:** I can arrange the appointment in English, but I find medical explanations difficult. My daughter usually helps me understand.

02 **Amira:** Thank you for telling me. What is your [1] _____ for the visit, including any dialect we should specify?

03 **Huy:** Vietnamese. I understand some English, but I worry I will agree to something because I cannot find the right question.

04 **Amira:** We can arrange a [2] _____ through our service. You will not be charged for that support at this clinic.

05 **Huy:** Would my daughter still be allowed to come? I would like her there for support, not to make every decision for me.

06 **Amira:** I will record your preference and confirm the visit arrangements. Her supporting role is separate from providing the interpretation.

07 **Huy:** Can the interpreter join remotely? It may be hard to find someone who can come at the same time.

08 **Amira:** We can request [3] _____. I will confirm availability and the clinic's setup before describing it as booked.

09 **Huy:** I would prefer not to explain private matters where everyone in the waiting room can hear.

10 **Amira:** I will note that concern and arrange the appropriate setting. Your [4] _____ matters during the connection as well as during the consultation.

Conversation continues on the next page.

Language assistance belongs in the appointment plan

CONVERSATION / CONTINUED

11 **Huy:** There is a form in the appointment message. Can you explain what the treatment permission section means?

12 **Amira:** I can help you obtain language support for the form. The clinical team needs to explain the treatment and answer your questions; I cannot give that explanation for them.

13 **Huy:** I sometimes nod when I have heard a sentence, even if I have not understood everything.

14 **Amira:** Then please say when you need something repeated. I will note the [5] _____ so the team can allow for questions.

15 **Huy:** Do I need to call the interpreting company myself?

16 **Amira:** No, I will send the request through our process and track the response. You should not have to coordinate the service between several offices.

17 **Huy:** Will the appointment confirmation say whether the interpreter is arranged, or only show the doctor's time?

18 **Amira:** It will distinguish the visit booking from the interpreter [6] _____. If support is still pending, I will explain the next step.

19 **Huy:** Thank you. Please use the contact method already recorded for me, and include the language arrangement in the update.

20 **Amira:** I will check that contact detail through our usual process and update you with both the appointment and support status.

CHECK THE GAPS

1. preferred language / **2.** qualified interpreter / **3.** remote interpreting / **4.** confidentiality / **5.** communication need / **6.** confirmation

Why these answers fit, page 165

A requested interpreter is reported as booked

A clinic has sent an interpreter request but received no acceptance. The appointment itself is booked.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

pending / status / service / update

01 **Registrar:** The appointment is booked, but interpreting remains [1] _____.

02 **Colleague:** Keep that separate from the visit booking [2] _____.

03 **Registrar:** I will follow up with the interpreting [3] _____.

04 **Colleague:** Tell the patient the actual position in the next [4] _____.

CHECK THE GAPS

1. pending / **2.** status / **3.** service / **4.** update

Why these answers fit, page 165

Keep the distinction audible

Offer language support respectfully and confirm the practical arrangements.

Language for the next case

Two similar names, two different patient records

patient identifiers

Patient identifiers are person-specific details used together to match the individual to the correct record, not merely a similar name.

Felix: Good. Verify the required patient identifiers through our process, without reading another person's details aloud.

duplicate record

A duplicate record is more than one record for the same person; these records have not been shown to belong to one person.

Felix: Keep that boundary. A similar name does not establish a duplicate record, and a mismatch does not tell us which record is correct.

referral provenance

Referral provenance identifies where the referral came from and how it entered the record, supporting investigation of the attachment error.

Felix: Exactly. Also preserve the referral provenance so we can trace how the document was attached.

impact assessment

Impact assessment examines what actions or disclosures may already have been affected by the mismatch, beyond correcting one attachment.

Felix: Yes. That is part of the impact assessment. Do not assume there was no effect because you caught it at check-in.

privacy

Privacy requires avoiding unnecessary disclosure of another patient's information while explaining the delay.

Felix: That protects privacy. Offer a clear update and route any urgent clinical concern promptly to the clinical team.

record merge

A record merge combines records and must follow authorized identity review; it is not a routine fix for similar names.

Felix: I will confirm the review owner and the next contact. Any record merge requires the separate authorized process, not our assumption during this call.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Two similar names, two different patient records

A registrar sees two similar names during check-in. Their dates of birth differ, and an incoming referral may be attached to the wrong record. No clinical information has been disclosed to the waiting patient. The patient-identification team must resolve the mismatch through the approved process.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

duplicate record / privacy / impact assessment / referral provenance / record merge / patient identifiers

01 **Mara:** I have two similar names on screen, and the referral may be attached to the wrong person. I have paused the check-in step.

02 **Felix:** Good. Verify the required [1] _____ through our process, without reading another person's details aloud.

03 **Mara:** The dates of birth differ. I have not shown either record to the patient or asked them to choose between the two.

04 **Felix:** Keep that boundary. A similar name does not establish a [2] _____, and a mismatch does not tell us which record is correct.

05 **Mara:** Could I create a third record to get the patient into the queue while we investigate?

06 **Felix:** Do not improvise a workaround. Use the identification escalation process so the care team can manage the delay safely without compounding the record problem.

07 **Mara:** I will notify the receiving team that check-in is delayed for identification review, not say the patient failed to provide information.

08 **Felix:** Exactly. Also preserve the [3] _____ so we can trace how the document was attached.

09 **Mara:** I have the source reference and arrival time. Should I move the document to the record with the closer spelling?

10 **Felix:** No. Spelling similarity is not enough. The authorized team must establish the right match and apply the correction with traceability.

Conversation continues on the next page.

Two similar names, two different patient records

CONVERSATION / CONTINUED

11 **Mara:** We should check whether anyone has already used the referral information for scheduling or clinical work.

12 **Felix:** Yes. That is part of the [4] _____. Do not assume there was no effect because you caught it at check-in.

13 **Mara:** The patient is asking why this is taking longer. I can explain that we are checking the record without discussing the other person.

14 **Felix:** That protects [5] _____. Offer a clear update and route any urgent clinical concern promptly to the clinical team.

15 **Mara:** If the team confirms that the referral was misfiled, will the original attachment history stay visible?

16 **Felix:** The correction must preserve the required history under the authorized process. Do not delete evidence just to make the screen look tidy.

17 **Mara:** I will send the discrepancy through the designated channel and keep the check-in status accurate.

18 **Felix:** I will confirm the review owner and the next contact. Any [6] _____ requires the separate authorized process, not our assumption during this call.

19 **Mara:** Please confirm when the match and any affected workflow are resolved. I do not want to restart simply because my message was received.

20 **Felix:** Agreed. We will distinguish acknowledgment, identity confirmation, correction, and permission to resume the relevant administrative step.

CHECK THE GAPS

1. patient identifiers / **2.** duplicate record / **3.** referral provenance / **4.** impact assessment / **5.** privacy / **6.** record merge

Why these answers fit, page 167

Similar names are mistaken for a duplicate

Two records have similar names but different verified dates of birth. A colleague proposes merging them without identity review.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

merge / resolution / person / delay

01 **Registrar:** Similar names do not prove these records belong to the same [1] _____ .

02 **Reviewer:** Do not perform a record [2] _____ on that basis.

03 **Registrar:** I will preserve the discrepancy and route it for authorized [3] _____.

04 **Reviewer:** Do not disclose the other record while explaining the [4] _____.

CHECK THE GAPS

1. person / **2.** merge / **3.** resolution / **4.** delay

Why these answers fit, page 167

> **Keep the distinction audible**
>
> Escalate a possible identity mismatch without guessing, merging, or exposing records.

Language for the next case

A clinic closure needs more than a cancellation message

service-continuity plan

The service-continuity plan sets out how essential services and communication are maintained during a disruption.

Priya: We can begin through the approved routes, but first I need the service-continuity plan and the clinical contact for cases that cannot simply be deferred.

contact worklist

The contact worklist tracks each affected patient's communication status and next action, not just the number of calls attempted.

Priya: Understood. I will prepare the contact worklist with the booked visit, approved contact route, and assigned caller.

alternative capacity

Alternative capacity means suitable usable service capacity elsewhere; a possible room alone is not confirmation.

Priya: Then we must not advertise that as alternative capacity. I will keep it pending until the receiving team confirms what they can accept.

successful notification

Successful notification means the message has been communicated through the defined process; an unanswered call alone does not establish it.

Priya: An attempted contact, not a successful notification. We will follow the approved message and privacy rules.

status update

A status update is a communication commitment, distinct from guaranteeing a replacement appointment.

Priya: Not without confirmed slots and clinical review. We can promise a specific status update from our team and keep it even if rebooking is unfinished.

handover owner

A handover owner is the named person responsible for unfinished actions after responsibility transfers.

Priya: Reached, not reached, rebooked, awaiting clinical review, and pending alternatives, with a handover owner for every unfinished action.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

A clinic closure needs more than a cancellation message

A facilities fault closes one outpatient clinic for the afternoon. Twenty patients are scheduled; none has yet been contacted. Alternative capacity is unconfirmed. Clinical staff must assess any time-sensitive care needs. Administrators are coordinating communication, not independently deciding which care can wait.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

status update / contact worklist / successful notification / service-continuity plan / alternative capacity / handover owner

01 **Soren:** Facilities has closed the clinic for this afternoon. We have twenty scheduled patients. How quickly can your team contact them?

02 **Priya:** We can begin through the approved routes, but first I need the [1] _____ and the clinical contact for cases that cannot simply be deferred.

03 **Soren:** The clinical lead is reviewing that now. Do not assign urgency from appointment labels or move everyone to next week automatically.

04 **Priya:** Understood. I will prepare the [2] _____ with the booked visit, approved contact route, and assigned caller.

05 **Soren:** Another site may have space, but they have not confirmed staff or suitable appointment types.

06 **Priya:** Then we must not advertise that as [3] _____. I will keep it pending until the receiving team confirms what they can accept.

07 **Soren:** Some patients may already be traveling. The message needs to say clearly that this site is closed, without implying their care need has disappeared.

08 **Priya:** Yes. We will give the verified closure information and the appropriate contact for next steps, with clinical questions passed to the clinical team.

09 **Soren:** What will you record when a call goes to voicemail?

10 **Priya:** An attempted contact, not a [4] _____. We will follow the approved message and privacy rules.

Conversation continues on the next page.

A clinic closure needs more than a cancellation message

CONVERSATION / CONTINUED

11 **Soren:** We also need language assistance for some calls. Use the recorded needs rather than assume an English voicemail is enough.

12 **Priya:** I will include those arrangements and track which messages still need an accessible follow-up.

13 **Soren:** Can you promise everyone a replacement visit tomorrow? That would make the first call easier.

14 **Priya:** Not without confirmed slots and clinical review. We can promise a specific [5] _____ from our team and keep it even if rebooking is unfinished.

15 **Soren:** Agreed. I will give you a confirmed update time and the route for unresolved cases before calls begin.

16 **Priya:** Please also confirm who handles people arriving at the closed site so the phone team and onsite staff give the same information.

17 **Soren:** The duty coordinator will handle that under the closure plan. What will your end-of-shift report show?

18 **Priya:** Reached, not reached, rebooked, awaiting clinical review, and pending alternatives, with a [6] _____ for every unfinished action.

19 **Soren:** That lets us see the remaining work without treating twenty attempted calls as twenty resolved appointments.

20 **Priya:** Exactly. We will close each item only when its communication and next-step status meet the actual plan, not when someone has dialed the number.

CHECK THE GAPS

1. service-continuity plan / **2.** contact worklist / **3.** alternative capacity / **4.** successful notification / **5.** status update / **6.** handover owner

Why these answers fit, page 169

A voicemail is counted as a completed rebooking

A scheduler leaves a permitted voicemail about a closure. No replacement slot has been offered or accepted.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

action / handover / rebooked / team

01 **Scheduler:** The call was attempted; the appointment is not yet [1] _____.

02 **Lead:** Record the voicemail and the required next contact [2] _____.

03 **Scheduler:** Any question about delaying care goes to the clinical [3] _____.

04 **Lead:** Carry the unfinished item into the shift [4] _____.

CHECK THE GAPS

1. rebooked / **2.** action / **3.** team / **4.** handover

Why these answers fit, page 169

Keep the distinction audible

Coordinate administrative disruption while keeping clinical prioritization with the clinical team.

01 / Answers and explanations

A / Read the situation

A1. What information is missing?

D. The requested specialty and reason for the appointment

The referral contains identifiers and contact details but lacks the specialty and appointment reason.

Return to the exercise, page 4

A2. What can Elena reliably promise on the supplied facts?

C. A status update by 3 p.m.

Elena controls the update, while booking depends on missing information and unconfirmed availability.

Return to the exercise, page 4

A3. Who should determine clinical urgency?

A. The appropriately qualified clinical team through its review process

The brief reserves clinical prioritization for the clinical review process, not administrative guesswork.

Return to the exercise, page 4

D / Response choices

D1. Which message matches the referral's actual status?

A. We received the referral and need its specialty and reason; I will update you by 3 p.m.

Receipt and identifiers do not establish acceptance, a held slot, or clinical priority. The supported commitment is a status update while the two missing details are clarified.

Return to the exercise, page 8

D2. Which clarification should Marcus request?

C. The intended specialty and clinical reason from the referring team

The missing fields require reliable clarification from the referring team rather than administrative invention.

Return to the exercise, page 8

01 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: The team or department expected to act on a referral. Which term fits?

D. receiving service

A Not here. appointment slot: A defined time available for a scheduled visit. The usual combination is "confirm an appointment slot".

B Not here. intake queue: The worklist holding incoming requests for initial processing. The usual combination is "monitor the intake queue".

C Not here. accessibility need: A requirement affecting a person's ability to use a service. The usual combination is "record an accessibility need".

D Correct. receiving service: The team or department expected to act on a referral. The usual combination is "identify the receiving service".

Return to the exercise, page 8

D4. Rebuild the exchange from "The referral arrived, but the booking is not ready". Elena: "Could you ask the referring office to confirm the specialty? I should not choose whichever service happens to have an opening this afternoon." Marcus: [missing response] Elena: "We also have no clinical priority recorded. I cannot assume that the missing reason means the request is routine or unimportant." Which response belongs in the gap?

B. I will contact the referring clinician through the appropriate office route. The request will identify both missing fields so we do not resolve one and overlook the other.

A Not here. This line belongs to turn 2, after Elena says: "The referral arrived this morning, but the specialty and appointment reason are blank. The patient expects a booking confirmation today. I need to explain what happens next." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Marcus's response in turn 6. It connects Elena's preceding point with Elena's follow-up. The referring clinician or team can clarify the intended clinical purpose and destination rather than leaving the scheduler to invent them.

C Not here. This line belongs to turn 14, after Elena says: "Once the specialty is confirmed, we still need the receiving team to identify the appropriate appointment type. A general opening may not fit the requested service." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Elena says: "If the patient needs language assistance or an access arrangement, that should be recorded through our usual process before the visit is finalized." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 8

E / Conversation

1. referral completeness

Turn 2. Referral completeness concerns the information required to process the request, and two essential details are missing here.

01 / Answers and explanations / continued

2. intake queue

Turn 4. The intake queue holds the received request for processing; its presence there does not mean an appointment is arranged.

3. referring clinician

Turn 6. The referring clinician or team can clarify the intended clinical purpose and destination rather than leaving the scheduler to invent them.

4. clinical triage

Turn 8. Clinical triage is the qualified assessment of priority; missing administrative information does not establish a clinical category.

5. callback commitment

Turn 10. The callback commitment promises a status update by a stated time, not a completed appointment booking.

6. booking confirmation

Turn 12. A booking confirmation establishes an arranged appointment; none has been made in this case.

7. scheduling template

Turn 14. The scheduling template defines appointment types and times; an available opening is not automatically suitable for every referral.

8. interpreter request

Turn 16. An interpreter request identifies a language-access need that should be arranged through the appropriate service process.

9. closed-loop referral

Turn 18. A closed-loop referral tracks receipt, action, and feedback so the request does not disappear between teams.

10. escalation pathway

Turn 20. The escalation pathway identifies how unresolved issues reach the appropriate owner without inventing clinical decisions or promises.

Return to the conversation, page 10

01 / Answers and explanations / continued

F / Accuracy and scope

F1. What does the absence of an assigned priority mean?

A. Clinical priority is unconfirmed.

Missing priority information is not evidence of either low or high urgency.

Return to the exercise, page 14

F2. At 3 p.m., clarification is still pending. What honors the commitment?

D. Contact the patient with the current status and a clear next update.

The promised action is a status update, so it should occur even when the underlying question remains unresolved.

Return to the exercise, page 14

F / Guided speaking

1. call

Scheduler: I promised a status call.

Turn 1. The commitment concerns communication at a stated time, not a clinical consultation.

2. pending

Colleague: The specialist review remains pending.

Turn 2. The supplied facts say review has not been completed.

3. reserved

Scheduler: No appointment is reserved.

Turn 3. The briefing expressly states that no appointment has been held or booked.

4. confirmation

Colleague: Make the promised update without implying a booking confirmation.

Turn 4. A status update must not be presented as confirmation of an appointment that does not exist.

Return to the conversation, page 15

01 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "The referral arrived, but the booking is not ready". Elena: "Once the specialty is confirmed, we still need the receiving team to identify the appropriate appointment type. A general opening may not fit the requested service." Marcus: [missing response] Elena: "If the patient needs language assistance or an access arrangement, that should be recorded through our usual process before the visit is finalized." Which response belongs in the gap?

A. Yes. The scheduling template needs to match the confirmed request. We should not turn an available time into an unsuitable booking simply to make the status look complete.

A Correct. This is Marcus's response in turn 14. It connects Elena's preceding point with Elena's follow-up. The scheduling template defines appointment types and times; an available opening is not automatically suitable for every referral.

B Not here. This line belongs to turn 2, after Elena says: "The referral arrived this morning, but the specialty and appointment reason are blank. The patient expects a booking confirmation today. I need to explain what happens next." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 6, after Elena says: "Could you ask the referring office to confirm the specialty? I should not choose whichever service happens to have an opening this afternoon." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Elena says: "If the patient needs language assistance or an access arrangement, that should be recorded through our usual process before the visit is finalized." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 15

02 / Answers and explanations

A / Read the situation

A1. How many exceptions were rejected before adjudication?

C. Twelve

The underlying records identify twelve submission rejections and eight post-review denials.

Return to the exercise, page 16

A2. What is known about the other 80 claims?

A. They were not analyzed in this meeting.

The briefing expressly leaves their status unanalyzed, so payment and responsibility cannot be inferred.

Return to the exercise, page 16

A3. What must guide the correction or appeal route?

A. The actual response and applicable payer rules

The stage, reason, and payer-specific process determine follow-up; an exception total alone does not.

Return to the exercise, page 16

D / Response choices

D1. What percentage of the 20 exceptions are pre-adjudication rejections?

A. 60%

Twelve divided by twenty equals 60%; 12% would use all one hundred submissions as the denominator.

Return to the exercise, page 20

D2. Which dashboard breakdown preserves both stage and denominator?

C. Twelve pre-adjudication rejections and eight reviewed denials out of 100 submissions

The records identify twelve rejections before adjudication and eight denials after review. They establish neither appeal outcomes nor the payment status of the other eighty.

Return to the exercise, page 20

02 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: A response indicating receipt or initial processing status. Which term fits?

D. acknowledgment

A Not here. claim adjustment reason code: A standardized code explaining why a claim payment differs from the billed amount. The usual combination is "interpret the claim adjustment reason code".

B Not here. resubmission: Sending a claim again through the appropriate process. The usual combination is "track the resubmission".

C Not here. accounts receivable: Amounts recorded as due to the organization. The usual combination is "review accounts receivable".

D Correct. acknowledgment: A response indicating receipt or initial processing status. The usual combination is "inspect the claim acknowledgment".

Return to the exercise, page 20

D4. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "For the twelve identifier errors, could we copy the number from an older paid claim? That would clear them quickly." Tessa: [missing response] Leo: "And for the eight denials, I need more than denied on a list. Can we see why payment was not allowed?" Which response belongs in the gap?

D. Verify the current member identifier against the appropriate source first. An old claim can be a lead, but it is not permission to guess or reuse unchecked information.

A Not here. This line belongs to turn 10, after Leo says: "The summary has a broad adjustment reason. Is there another code that explains the missing detail?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 16, after Leo says: "What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 18, after Leo says: "Create two lists, then: twelve rejections and eight denials. I need an owner, reason, deadline, and latest status against each claim." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Tessa's response in turn 6. It connects Leo's preceding point with Leo's follow-up. The member identifier identifies the person within the plan; missing or invalid values are the stated rejection cause.

Return to the exercise, page 20

E / Conversation

1. front-end rejections

Turn 2. Front-end rejections occurred before adjudication in this case, unlike the eight claims denied after review.

02 / Answers and explanations / continued

2. acknowledgment

Turn 4. The acknowledgment records the submission's receipt or processing status and helps establish where it stopped.

3. member identifier

Turn 6. The member identifier identifies the person within the plan; missing or invalid values are the stated rejection cause.

4. remittance advice

Turn 8. The remittance advice explains processed-claim payment and adjustments, helping identify the denial reason and relevant follow-up.

5. remittance advice remark code

Turn 10. A remittance advice remark code adds detail that may be needed to interpret the adjustment or processing result.

6. timely-filing limit

Turn 12. The timely-filing limit governs submission timing and must be verified for the relevant payer and claim context.

7. coding integrity

Turn 14. Coding integrity requires support from the clinical record and applicable rules, not changes selected solely to obtain payment.

8. patient responsibility

Turn 16. Patient responsibility must be established under applicable rules; unresolved or unanalyzed claims do not automatically become patient balances.

9. workqueue

Turn 18. The workqueue organizes follow-up with owners and status, rather than leaving a combined exception total without actions.

10. denial analysis

Turn 20. Denial analysis should use the reviewed-denial group and its actual reasons, not combine every submission exception into one category.

Return to the conversation, page 22

02 / Answers and explanations / continued

F / Accuracy and scope

F1. A rejection cites an invalid member identifier. What should happen first?

B. Verify the identifier and applicable correction process against reliable records.

The stated rejection concerns identifier data, so the first action is accurate verification and route selection.

Return to the exercise, page 24

F2. Which summary preserves the financial uncertainty?

A. Correction or appeal may resolve the issue, but payment and patient responsibility remain subject to review.

Taking a follow-up action does not establish its outcome or the correct assignment of financial responsibility.

Return to the exercise, page 24

F / Guided speaking

1. 9%

Analyst: Rejections are 9% of all submissions.

Turn 1. Eighteen divided by two hundred equals 9%, using all submissions as the denominator.

2. 60%

Manager: Rejections are 60% of the exceptions.

Turn 2. Eighteen divided by thirty equals 60%, using only the exception group.

3. twelve

Analyst: Reviewed denials total twelve.

Turn 3. The briefing identifies twelve claims denied after review, distinct from eighteen rejections.

4. unspecified

Manager: Payment status for the remainder is unspecified.

Turn 4. The exercise gives no payment information for the remaining claims, so paid status cannot be inferred.

Return to the conversation, page 25

02 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "A colleague suggested a different diagnosis code on one denial because it might pay. I want that handled properly." Tessa: [missing response] Leo: "What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here." Which response belongs in the gap?

A. That needs qualified review for coding integrity, based on the clinical record. Payment preference cannot supply evidence for changing a diagnosis.

A Correct. This is Tessa's response in turn 14. It connects Leo's preceding point with Leo's follow-up. Coding integrity requires support from the clinical record and applicable rules, not changes selected solely to obtain payment.

B Not here. This line belongs to turn 4, after Leo says: "Then my appeals-backlog label is wrong too. Where do we check which stage each submission actually reached?" It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Leo says: "Twenty exceptions out of one hundred submissions. Can I call that a twenty-percent denial rate in the management update?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 12, after Leo says: "Prioritize by deadline as well as category. I do not want older items expiring while we fix the easier ones." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 25

03 / Answers and explanations

A / Read the situation

A1. How many staffed operational beds are unoccupied at 10 a.m.?

A. Zero

All twenty-four staffed operational beds are occupied; planned departures and unstaffed spaces do not create current availability.

Return to the exercise, page 28

A2. What is the occupancy rate using staffed operational beds?

A. 100%

Twenty-four occupied beds divided by twenty-four staffed operational beds equals 100%; using all physical beds changes the denominator.

Return to the exercise, page 28

A3. What remains unconfirmed?

A. The timing and completion of the two planned discharges

The case supplies the current counts but expressly leaves discharge timing and completion unconfirmed.

Return to the exercise, page 28

D / Response choices

D1. Which message accurately corrects the original update?

D. At 10 a.m., no staffed operational beds are free; two discharges are anticipated with timing unconfirmed.

The corrected message preserves the current census and keeps planned departures conditional.

Return to the exercise, page 32

D2. What does 24 divided by 28 measure here, and what does it omit?

A. Physical-bed occupancy; it includes four beds not staffed for use.

The denominator 28 counts physical beds, whereas today's staffed operational denominator is 24. The ratio is about 86%, but all 24 operational beds are occupied and neither discharge is confirmed.

Return to the exercise, page 32

03 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Capacity supported by the required available staffing. Which term fits?

B. staffed capacity

A Not here. discharge barrier: An unresolved condition delaying a planned departure. The usual combination is "identify discharge barriers".

B Correct. staffed capacity: Capacity supported by the required available staffing. The usual combination is "confirm staffed capacity".

C Not here. boarding: Waiting in a location for an appropriate inpatient placement or next care setting. The usual combination is "monitor boarding".

D Not here. skill mix: The combination of staff capabilities available for a workload. The usual combination is "review the skill mix".

Return to the exercise, page 32

D4. Rebuild the exchange from "Planned discharges are not available beds". Hana: "The building has twenty-eight beds. Someone may divide twenty-four by twenty-eight and conclude that four spaces are free for admissions." Paul: [missing response] Hana: "For today's operational position, twenty-four occupied out of twenty-four staffed beds means full occupancy. We should name the denominator in the update." Which response belongs in the gap?

C. That confuses physical space with staffed capacity. The other four beds are not staffed for use today. They are not an immediately usable reserve merely because they appear on the floor plan.

A Not here. This line belongs to turn 16, after Hana says: "I will label the two possible openings as conditional. The transfer desk can plan around the possibility without treating it as a completed availability check." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 20, after Hana says: "I will correct the desk now: no operational bed is free at ten, and the two possible openings depend on discharge and readiness confirmation." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Paul's response in turn 6. It connects Hana's preceding point with Hana's follow-up. Staffed capacity excludes the four physical beds not staffed for use today.

D Not here. This line belongs to turn 18, after Hana says: "Who will update the desk when the status changes? I do not want the corrected message to sit unchanged after the room actually becomes ready." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 32

E / Conversation

1. bed census

Turn 2. The bed census is the count at a stated time; at 10 a.m. all operational beds were occupied.

03 / Answers and explanations / continued

2. anticipated discharges

Turn 4. Anticipated discharges are expected departures, not confirmation that beds are empty or ready.

3. staffed capacity

Turn 6. Staffed capacity excludes the four physical beds not staffed for use today.

4. occupancy rate

Turn 8. The occupancy rate is 100% on the stated staffed operational basis: 24 occupied out of 24.

5. discharge-barrier

Turn 10. A discharge-barrier check identifies unresolved conditions affecting departure without assuming that the discharge has occurred.

6. Bed turnaround

Turn 12. Bed turnaround includes preparing a vacated bed for reuse; departure does not automatically establish readiness.

7. bed allocation

Turn 14. Bed allocation assigns a suitable bed through the appropriate process, including relevant clinical and operational requirements.

8. capacity forecast

Turn 16. A capacity forecast estimates future availability under conditions and is distinct from the current available-bed count.

9. flow huddle

Turn 18. The flow huddle is the coordination point for sharing current status, constraints, and responsible follow-up.

10. room readiness

Turn 20. Room readiness confirms the relevant conditions for reuse; it has not yet been established for the anticipated discharges.

Return to the conversation, page 34

03 / Answers and explanations / continued

F / Accuracy and scope

F1. What must follow a confirmed departure before availability is reported?

D. Verify the relevant readiness, staffing, and allocation conditions.

A departed patient does not by itself establish all conditions needed for the next placement.

Return to the exercise, page 37

F2. Which update best supports planning without promising capacity?

B. Two discharges are anticipated; the unit will confirm timing and readiness at the next agreed check.

The useful forecast states its uncertainty and next confirmation step without contradicting the current count.

Return to the exercise, page 37

F / Guided speaking

1. vacated

Coordinator: The room is vacated.

Turn 1. Departure establishes that the patient has left, not that the room meets all readiness conditions.

2. pending

Colleague: Cleaning remains pending.

Turn 2. The briefing says the room is awaiting cleaning.

3. confirmation

Coordinator: Staffing still needs confirmation.

Turn 3. Staffing for the next placement has not been verified.

4. ready

Colleague: Do not mark the room ready yet.

Turn 4. Cleaning and staffing are unresolved, so the supplied readiness conditions have not been met.

Return to the conversation, page 38

03 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Planned discharges are not available beds". Hana: "Once a room is ready, who confirms it is suitable for the incoming patient? I do not want a room-status change treated as placement approval." Paul: [missing response] Hana: "I will label the two possible openings as conditional. The transfer desk can plan around the possibility without treating it as a completed availability check." Which response belongs in the gap?

A. The bed allocation process handles that. Capacity language should support the decision, not bypass the appropriate assessment of patient needs, staffing, and the setting.

A Correct. This is Paul's response in turn 14. It connects Hana's preceding point with Hana's follow-up. Bed allocation assigns a suitable bed through the appropriate process, including relevant clinical and operational requirements.

B Not here. This line belongs to turn 2, after Hana says: "The transfer desk has been told that two beds are available. Your ten o'clock count shows twenty-four occupied beds. What changed after the count?" It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 4, after Hana says: "Then the message has converted an expectation into present capacity. I will correct it before the receiving team treats it as permission to send someone." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Hana says: "For today's operational position, twenty-four occupied out of twenty-four staffed beds means full occupancy. We should name the denominator in the update." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 38

04 / Answers and explanations

A / Read the situation

A1. What is established about the event?

D. The wrong-specialty booking was caught before the appointment, with no clinical harm reported.

The brief establishes the booking error and interception, while cause and recurrence protection remain unresolved.

Return to the exercise, page 39

A2. Why is a refresher session not yet a complete response?

C. The review has not established a training gap or how the action would address the cause.

The response should follow evidence about contributing factors rather than assume a cause before investigation.

Return to the exercise, page 39

A3. What should the review use?

A. The referral record, booking history, and other relevant verified evidence

The available records can help reconstruct the sequence; absence of harm alone does not explain the event.

Return to the exercise, page 39

D / Response choices

D1. Which review opening is most precise?

B. The wrong-specialty booking was intercepted; we will reconstruct the sequence and assess contributing factors.

The opening states known facts and a defensible investigation purpose without assuming cause or perfect protection.

Return to the exercise, page 43

D2. What does refresher attendance demonstrate?

C. That the recorded participants attended, not necessarily that the process improved

Attendance is implementation evidence; effectiveness requires a relevant measure of the intended improvement.

Return to the exercise, page 43

04 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: The verified sequence of events and actions. Which term fits?

C. event chronology

A Not here. just culture: An approach combining learning, fair accountability, and attention to system conditions. The usual combination is "support a just culture".

B Not here. risk register: A record of identified risks, responses, and ownership. The usual combination is "update the risk register".

C Correct. event chronology: The verified sequence of events and actions. The usual combination is "reconstruct the event chronology".

D Not here. outcome measure: A measure of the result the process is intended to influence. The usual combination is "review the outcome measure".

Return to the exercise, page 43

D4. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "The immediate suggestion is a refresher session for schedulers. That may help, but we have not established that missing knowledge caused this event." Asha: [missing response] Ben: "We can thank the person who noticed the mismatch. At the same time, the review should not stop with the fact that the second check worked." Which response belongs in the gap?

B. Identify the contributing factors first. The interface, referral wording, workflow, or other conditions may need examination. We should not select a cause merely because the proposed action is easy to organize.

A Not here. This line belongs to turn 14, after Ben says: "Once the findings are supported, the action plan should show who will do what. A general instruction to improve accuracy will not be easy to follow up." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Asha's response in turn 6. It connects Ben's preceding point with Ben's follow-up. Contributing factors are supported conditions that helped the event occur; lack of training has not yet been established.

C Not here. This line belongs to turn 2, after Ben says: "The patient was booked into the wrong specialty, but the second check caught it before the appointment. Can we close the issue as a harmless administrative mistake?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Ben says: "We can thank the person who noticed the mismatch. At the same time, the review should not stop with the fact that the second check worked." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 43

---

04 / Answers and explanations / continued

E / Conversation

1. near miss

Turn 2. The near miss was intercepted before reported clinical harm, but still warrants the applicable reporting and learning process.

2. event chronology

Turn 4. The event chronology reconstructs the verified sequence rather than substituting an assumed explanation.

3. contributing factors

Turn 6. Contributing factors are supported conditions that helped the event occur; lack of training has not yet been established.

4. detection control

Turn 8. The detection control identified an error already made; it is different from preventing the initial wrong booking.

5. process map

Turn 10. The process map shows workflow steps and handoffs, helping locate where the error could enter or escape.

6. just culture

Turn 12. A just culture combines learning about system conditions with fair accountability rather than automatic blame or absence of responsibility.

7. action owner

Turn 14. The action owner is accountable for a defined task; a general aspiration does not assign responsibility.

8. implementation evidence

Turn 16. Implementation evidence shows an action occurred, while effectiveness requires evidence about its result.

9. effectiveness measure

Turn 18. The effectiveness measure tests the intended improvement using a defined, relevant comparison rather than task completion alone.

04 / Answers and explanations / continued

10. accreditation evidence

Turn 20. Accreditation evidence should accurately show the event, investigation, actions, and follow-up, without overstating completion or effectiveness.

Return to the conversation, page 45

F / Accuracy and scope

F1. Which measure most directly tests whether wrong-specialty booking accuracy improved?

A. Verified wrong-specialty bookings divided by comparable reviewed bookings over the planned period

A consistently defined booking-error rate measures the intended process result. Attendance and task completion provide implementation evidence, not evidence that the booking process became more accurate.

Return to the exercise, page 48

F2. Why include a balancing measure?

C. To detect unintended effects such as added delays or workarounds

A change can improve one aspect while harming another, so monitoring side effects supports a fuller assessment.

Return to the exercise, page 48

F / Guided speaking

1. delivered

Lead: The session has been delivered.

Turn 1. The briefing confirms completion of the session for the scheduled staff.

2. evidence

Reviewer: Attendance supplies implementation evidence.

Turn 2. Attendance demonstrates that the activity occurred, not that its intended effect has been achieved.

3. untested

Lead: Effectiveness remains untested.

Turn 3. No post-session accuracy review has occurred, so the result has not been evaluated.

04 / Answers and explanations / continued

4. review

Reviewer: Complete the planned four-week review.

Turn 4. The action plan specifies a follow-up review of booking accuracy as the next evidence-gathering step.

Return to the conversation, page 49

F / Rebuild the exchange

1. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "Once the findings are supported, the action plan should show who will do what. A general instruction to improve accuracy will not be easy to follow up." Asha: [missing response] Ben: "For the accreditation discussion, we can show that an action was completed. But completion alone will not tell us whether it reduced the error risk." Which response belongs in the gap?

D. Name an action owner and a date for each specific task. Connect each action to the relevant finding so the plan is more than a list of familiar training activities.

A Not here. This line belongs to turn 2, after Ben says: "The patient was booked into the wrong specialty, but the second check caught it before the appointment. Can we close the issue as a harmless administrative mistake?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 20, after Ben says: "I will prepare the timeline and mark the open questions. We can show what caught the error without claiming that we have already established its cause." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 6, after Ben says: "The immediate suggestion is a refresher session for schedulers. That may help, but we have not established that missing knowledge caused this event." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Asha's response in turn 14. It connects Ben's preceding point with Ben's follow-up. The action owner is accountable for a defined task; a general aspiration does not assign responsibility.

Return to the exercise, page 49

05 / Answers and explanations

A / Read the situation

A1. What is the stated purpose?

A. Count missed appointments by clinic for last month

The brief describes a limited administrative count, not treatment review or publication of identifiable records.

Return to the exercise, page 53

A2. What remains unconfirmed?

A. Appropriate data scope and export permission

The purpose and requested fields are supplied, but scope and export authority still require confirmation.

Return to the exercise, page 53

A3. Which output may meet the stated purpose?

A. A suitably reviewed clinic-level count

A clinic-level count may answer the reporting question, subject to the applicable privacy and disclosure review.

Return to the exercise, page 53

D / Response choices

D1. Which proposed output best matches the stated reporting purpose, subject to privacy review?

B. Clinic-level monthly missed-appointment counts with appropriate small-group checks

The purpose is counting missed appointments by clinic, not reviewing individuals' diagnoses. Aggregation may meet it, but output scope, permissions, and identification risk still need review.

Return to the exercise, page 57

D2. What does technical download access establish?

C. That the system permits the action technically, not that the proposed sharing is authorized

System capability does not settle purpose, permission, recipient, or sharing conditions.

Return to the exercise, page 57

05 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: A health plan, clearinghouse, or qualifying provider covered by HIPAA. Which term fits?

B. covered entity

A Not here. recipient verification: Confirmation of the intended recipient's identity and authority. The usual combination is "complete recipient verification".

B Correct. covered entity: A health plan, clearinghouse, or qualifying provider covered by HIPAA. The usual combination is "identify the covered entity".

C Not here. disclosure: Release or access to information outside the entity holding it in the relevant privacy context. The usual combination is "assess the proposed disclosure".

D Not here. data minimization: Limiting collected or shared data to what is appropriate for the task. The usual combination is "practice data minimization".

Return to the exercise, page 57

D4. Rebuild the exchange from "A full patient list for a counting task". Omar: "The colleague works in operations and can view some records in the system. That does not tell me whether they can receive a broad export." Nina: [missing response] Omar: "My reporting account can technically download the full table. I should not treat the availability of an export button as approval for this request." Which response belongs in the gap?

D. Check role-based access and the specific permissions. A legitimate role can require some information without authorizing every field or every way of transferring it.

A Not here. This line belongs to turn 4, after Omar says: "A count for each clinic may answer the question. I do not see why the report would need individual clinical notes." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 2, after Omar says: "Operations wants last month's missed appointments by clinic. The request asks for names, dates of birth, diagnoses, and full visit notes. That seems broader than the question." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 20, after Omar says: "I will hold the extract and ask whether clinic-level counts answer the question. Please review the scope and recipient before we send anything." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Nina's response in turn 6. It connects Omar's preceding point with Omar's follow-up. Role-based access ties permissions to work responsibilities; internal employment does not establish unrestricted access.

Return to the exercise, page 57

05 / Answers and explanations / continued

E / Conversation

1. permitted use

Turn 2. The permitted use must match an appropriate purpose and basis; the limited counting task does not itself justify every requested field.

2. aggregate report

Turn 4. An aggregate report combines records into clinic-level counts that may satisfy the stated purpose with less individual information.

3. role-based access

Turn 6. Role-based access ties permissions to work responsibilities; internal employment does not establish unrestricted access.

4. export permission

Turn 8. Export permission is authority to extract information under the applicable controls, distinct from technical download capability.

5. minimum necessary

Turn 10. Minimum necessary limits information where the standard applies; the provider-treatment exception does not describe this administrative task.

6. de-identification

Turn 12. De-identification requires meeting applicable criteria; deleting names while retaining identifying details is not enough.

7. re-identification risk

Turn 14. Re-identification risk includes linking small groups or combined information to individuals, even in an aggregate report.

8. secure transfer

Turn 16. Secure transfer uses the authorized protected channel, but does not replace the need for an appropriate purpose and recipient.

9. audit log

Turn 18. The audit log records access or activity and supports review of how information was handled.

05 / Answers and explanations / continued

10. recipient verification

Turn 20. Recipient verification confirms identity and authority before information is shared through the approved route.

Return to the conversation, page 59

F / Accuracy and scope

F1. Which statement about the treatment exception is accurate?

B. Provider treatment disclosures and requests have an exception to minimum necessary; this case is administrative reporting.

The exception is context-specific and does not convert this limited administrative task into a treatment request.

Return to the exercise, page 62

F2. Why review an aggregate report?

D. Small groups or combined information may still allow identification.

Aggregation can reduce exposure but does not automatically eliminate identification risk in every output.

Return to the exercise, page 62

F / Guided speaking

1. capability

Analyst: The button shows technical capability.

Turn 1. The system allows an action technically, but that alone does not establish authorization.

2. unconfirmed

Reviewer: The report's approved scope is unconfirmed.

Turn 2. The briefing says output scope has not yet been verified.

3. purpose

Analyst: Clinic-level counts may meet the stated purpose.

Turn 3. The operational request concerns monthly counts, which may not require individual diagnoses.

4. sharing

Reviewer: Complete recipient and scope checks before sharing.

Turn 4. Authority and scope remain unresolved, so the information should not yet be sent.

Return to the conversation, page 63

05 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "A full patient list for a counting task". Omar: "Even clinic counts could be revealing if a group is very small or the recipient can combine the report with other information." Nina: [missing response] Omar: "Once the output is approved, I will verify the recipient and use our designated channel. I will not attach it to an unapproved personal message." Which response belongs in the gap?

A. Review re-identification risk before release. Appropriate aggregation or other controls may be needed, depending on the purpose, context, and applicable review process.

A Correct. This is Nina's response in turn 14. It connects Omar's preceding point with Omar's follow-up. Re-identification risk includes linking small groups or combined information to individuals, even in an aggregate report.

B Not here. This line belongs to turn 6, after Omar says: "The colleague works in operations and can view some records in the system. That does not tell me whether they can receive a broad export." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Omar says: "Operations wants last month's missed appointments by clinic. The request asks for names, dates of birth, diagnoses, and full visit notes. That seems broader than the question." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Omar says: "My reporting account can technically download the full table. I should not treat the availability of an export button as approval for this request." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 63

06 / Answers and explanations

A / Read the situation

A1. What can Arun verify?

A. Both earlier messages appear in the call record.

The call record confirms the messages, but no clinical interpretation or clinician callback time is supplied.

Return to the exercise, page 64

A2. What can Arun promise?

C. His own status update by 3 p.m.

Arun controls his administrative update, not the clinical team's timing or judgment.

Return to the exercise, page 64

A3. What does Leah say about the present call?

C. It concerns the missing response, not a current urgent symptom.

The brief explicitly defines this as a communication-delay call and does not provide a current urgent symptom.

Return to the exercise, page 64

D / Response choices

D1. Which opening best addresses Leah's experience?

C. I can see both messages, and I am sorry you have had to call again without a clear update.

The opening acknowledges the documented history and frustration without unsupported clinical or timing reassurance.

Return to the exercise, page 68

D2. Which promise can Arun make without committing another team?

C. I will update you by three, even if the clinician's callback time is still unconfirmed.

Arun controls his own status call. Forwarding, receipt, and silence do not establish a clinician's agreed time, completed result review, or resolution of the complaint.

Return to the exercise, page 68

06 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Language recognizing another person's experience or difficulty. Which term fits?

A. empathy statement

A Correct. empathy statement: Language recognizing another person's experience or difficulty. The usual combination is "use an appropriate empathy statement".

B Not here. status update: Information about the current state of an unresolved matter. The usual combination is "provide a status update".

C Not here. handoff acceptance: Confirmation that the receiving person or team accepts responsibility for the specified task. The usual combination is "verify handoff acceptance".

D Not here. de-escalation: Communication intended to reduce tension and support a constructive exchange. The usual combination is "use respectful de-escalation".

Return to the exercise, page 68

D4. Rebuild the exchange from "Two calls, still no clear answer". Leah: "Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything." Arun: [missing response] Leah: "What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this." Which response belongs in the gap?

C. I cannot give that clinical interpretation. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.

A Not here. This line belongs to turn 20, after Leah says: "All right. Include the earlier messages, find out who is handling it, and contact me by three whether or not the time is confirmed." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 18, after Leah says: "And if you have not got a clinician's time by three? That is usually when the promised update disappears." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Arun's response in turn 6. It connects Leah's preceding point with Leah's follow-up. Clinical interpretation requires qualified medical judgment; silence and call records do not establish that a result is normal.

D Not here. This line belongs to turn 8, after Leah says: "What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 68

E / Conversation

1. call history

Turn 2. The call history confirms Leah's earlier contacts, so Arun can acknowledge them without asking her to prove she called.

06 / Answers and explanations / continued

2. communication delay

Turn 4. The communication delay is the unresolved response gap; acknowledging it does not require interpreting the medical result.

3. clinical interpretation

Turn 6. Clinical interpretation requires qualified medical judgment; silence and call records do not establish that a result is normal.

4. status update

Turn 8. The status update is Arun's controlled commitment to report progress, not a promise of a completed clinical discussion.

5. callback window

Turn 10. A callback window must be agreed by the responding team; Arun's update time is not an established clinician appointment.

6. handoff acceptance

Turn 12. Handoff acceptance confirms that the receiving person or team accepts responsibility for the follow-up; sending or receiving a message alone does not establish that.

7. contact preference

Turn 14. The contact preference identifies the agreed communication method and should be checked under the usual verification process.

8. grievance pathway

Turn 16. The grievance pathway provides the appropriate formal route for the concern, separate from clinical interpretation of the result.

9. follow-through

Turn 18. Follow-through means honoring the promised update even when the underlying clinical response remains unresolved.

10. named owner

Turn 20. The named owner is the identified person accountable for this administrative follow-up, not a substitute for the clinician's medical role.

Return to the conversation, page 70

06 / Answers and explanations / continued

F / Accuracy and scope

F1. Leah asks whether silence means the result is normal. What should Arun say?

A. I cannot infer that or interpret the result; I will route the question to the clinical team.

Administrative silence and call records do not establish a clinical interpretation.

Return to the exercise, page 72

F2. What closes the administrative handoff?

A. Confirm that a clinical team member accepts the follow-up and keep the promised patient update.

Explicit acceptance assigns the clinical follow-up, while Arun retains his promised patient update. Merely forwarding or receiving a message does not establish responsibility.

Return to the exercise, page 72

F / Guided speaking

1. acknowledged

Administrator: The clinical team has acknowledged the request.

Turn 1. Receipt has been confirmed, so that limited status can be reported accurately.

2. unconfirmed

Patient: Is the callback time confirmed?" Administrator: "It is still unconfirmed.

Turn 2. The briefing explicitly says no clinical callback time has been agreed.

3. update

Administrator: This call honors my promised update.

Turn 3. The administrator promised a status update at three, not a completed medical discussion.

4. interpretation

Patient: Does receipt establish a result?" Administrator: "No clinical interpretation follows from receipt alone.

Turn 4. Acknowledging a message does not establish what the test result means.

Return to the conversation, page 73

06 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Two calls, still no clear answer". Leah: "Use the contact method already on my record. I do not want details sent to a different number during another handover." Arun: [missing response] Leah: "I also want the delay reviewed. Will raising a complaint distract from getting the result explained?" Which response belongs in the gap?

D. I will confirm that contact preference through our usual checks. We will use the agreed route rather than assume a new destination is acceptable.

A Not here. This line belongs to turn 12, after Leah says: "Please do not make me explain the first two calls to another person. They are already in the record." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 6, after Leah says: "Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 4, after Leah says: "This is about the missing response, not a new urgent symptom. Last time I was told the message had been passed on." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Arun's response in turn 14. It connects Leah's preceding point with Leah's follow-up. The contact preference identifies the agreed communication method and should be checked under the usual verification process.

Return to the exercise, page 73

07 / Answers and explanations

A / Read the situation

A1. What has the transport service confirmed?

B. Receipt of the request only

Acknowledgment confirms receipt, while vehicle assignment and pickup time remain unconfirmed.

Return to the exercise, page 76

A2. What is the home-health status?

B. The referral was received; the first visit is unconfirmed.

The briefing separates receipt of the referral from confirmation of a first visit.

Return to the exercise, page 76

A3. What does the 2 p.m. commitment mean?

D. Imran will give the care team a status update.

The stated commitment is communication at a set time, not transport, a visit, or a clinical discharge decision.

Return to the exercise, page 76

D / Response choices

D1. Which handoff statement is most actionable?

C. Transport acknowledged the request; no vehicle or time is assigned; Imran owns follow-up and a 2 p.m. update.

The actionable statement contains confirmed status, unresolved steps, ownership, and timing.

Return to the exercise, page 80

D2. Which next check goes beyond a home-health referral receipt?

D. Whether the service accepts responsibility and what first-visit arrangements are confirmed

A received referral is not yet service acceptance or a confirmed first visit. The sending record and transport acknowledgment cannot establish the receiving service's commitment.

Return to the exercise, page 80

07 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Organization of practical arrangements related to leaving a care setting. Which term fits?

A. discharge coordination

A Correct. discharge coordination: Organization of practical arrangements related to leaving a care setting. The usual combination is "document discharge coordination".

B Not here. home-health referral: A request for an appropriate service delivered in the home setting. The usual combination is "track a home-health referral".

C Not here. handoff checklist: A structured aid for communicating required transition information. The usual combination is "complete the handoff checklist".

D Not here. social needs: Practical social circumstances affecting a person's ability to access or manage care. The usual combination is "assess reported social needs".

Return to the exercise, page 80

D4. Rebuild the exchange from "Transport pending, but whose next action?". Rachel: "The home-health referral has also been received, but we do not have a confirmed first visit. It belongs on the handoff as a separate open item." Imran: [missing response] Rachel: "Good. Those arrangements support discharge planning, but neither of us should turn the administrative status into a decision about clinical readiness." Which response belongs in the gap?

D. I will verify service acceptance and the first-visit plan. The fact that a referral arrived does not tell the care team when the next service will begin.

A Not here. This line belongs to turn 16, after Rachel says: "If this transport cannot be arranged, check the approved alternative with the team. Please do not book a different type before suitability and authority are confirmed." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Rachel says: "Good. Those arrangements support discharge planning, but neither of us should turn the administrative status into a decision about clinical readiness." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Rachel says: "Before I finish at noon, I need to hand over this discharge coordination case. The note says transport pending, but that wording leaves too much unclear." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Imran's response in turn 6. It connects Rachel's preceding point with Rachel's follow-up. Service acceptance means the receiving service agrees to take the task; simple receipt does not establish that commitment.

Return to the exercise, page 80

E / Conversation

1. transport barrier

Turn 2. The transport barrier is the unresolved practical arrangement, which must be specified rather than hidden under pending.

07 / Answers and explanations / continued

2. dispatch confirmation

Turn 4. Dispatch confirmation verifies actual transport arrangements; acknowledgment of a request alone does not provide it.

3. service acceptance

Turn 6. Service acceptance means the receiving service agrees to take the task; simple receipt does not establish that commitment.

4. transition of care

Turn 8. A transition of care involves movement between care settings or stages and requires the appropriate clinical and practical coordination.

5. handoff checklist

Turn 10. The handoff checklist helps preserve required status, references, open actions, and responsibilities during the transfer.

6. care-team update

Turn 12. The care-team update is a promised status communication at 2 p.m., not a guaranteed pickup or clinical discharge time.

7. pickup window

Turn 14. A pickup window must come from confirmed transport arrangements; the administrative update time does not establish it.

8. contingency plan

Turn 16. The contingency plan is the approved response to an unresolved arrangement, not an improvised substitute for assessed transport needs.

9. first-visit confirmation

Turn 18. First-visit confirmation establishes the agreed initial service arrangement, which is not yet supplied by the referral receipt.

10. closed-loop communication

Turn 20. Closed-loop communication includes acknowledgment and confirmation of understanding, making responsibility explicit rather than assumed.

Return to the conversation, page 82

07 / Answers and explanations / continued

F / Accuracy and scope

F1. Which statement preserves the clinical boundary?

A. The qualified team determines discharge readiness; administrative coordination supports that decision.

Coordination communicates arrangements but does not transfer clinical decision authority to administrative staff.

Return to the exercise, page 86

F2. What closes the ownership transfer?

B. Imran acknowledges the tasks and the agreed next actions are documented.

Explicit acceptance and documented next actions prevent responsibility from disappearing during the shift change.

Return to the exercise, page 86

F / Guided speaking

1. acknowledged

Coordinator: The request was acknowledged.

Turn 1. The provider confirmed receipt, which is the only completed transport step supplied.

2. unassigned

Colleague: A vehicle remains unassigned.

Turn 2. The provider explicitly says no vehicle has been assigned.

3. check

Coordinator: One o'clock is the next check.

Turn 3. The coordinator promised another status check at one, not transport arrival.

4. pickup

Colleague: Do not present it as a confirmed pickup.

Turn 4. The briefing excludes a pickup promise and supplies no assigned vehicle.

Return to the conversation, page 87

07 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Transport pending, but whose next action?". Rachel: "If transport is still unresolved, the team needs to know early enough to use the appropriate escalation route. We should not imply that two is a pickup time." Imran: [missing response] Rachel: "If this transport cannot be arranged, check the approved alternative with the team. Please do not book a different type before suitability and authority are confirmed." Which response belongs in the gap?

A. Correct. Any pickup window must come from the provider's actual arrangements. The update deadline is our communication commitment, not a transport guarantee.

A Correct. This is Imran's response in turn 14. It connects Rachel's preceding point with Rachel's follow-up. A pickup window must come from confirmed transport arrangements; the administrative update time does not establish it.

B Not here. This line belongs to turn 4, after Rachel says: "The provider acknowledged receipt. No vehicle or pickup time has been assigned, so we should not describe the journey as booked." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 8, after Rachel says: "Good. Those arrangements support discharge planning, but neither of us should turn the administrative status into a decision about clinical readiness." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 2, after Rachel says: "Before I finish at noon, I need to hand over this discharge coordination case. The note says transport pending, but that wording leaves too much unclear." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 87

08 / Answers and explanations

A / Read the situation

A1. What changed between the compared data sets?

D. The reporting period and visit count, with clinic mix still unchecked

One figure is monthly across 200 visits and the other weekly across 50, so comparability needs assessment.

Return to the exercise, page 88

A2. What does 40 to 30 minutes establish arithmetically?

D. A ten-minute, 25% reduction between the reported medians

The arithmetic is correct, but it does not establish individual change, causation, or sustained comparable performance.

Return to the exercise, page 88

A3. What decision is proposed?

A. Approve a four-week validation review

The supplied request is a bounded review to validate the comparison, not an implementation or causal conclusion.

Return to the exercise, page 88

D / Response choices

D1. Which headline accurately describes the observed 40-to-30-minute comparison?

A. Reported median is ten minutes lower; period, mix, and causal comparability remain under review.

Ten minutes divided by forty is a 25% relative difference between medians, not percentage points. Different periods and unchecked clinic mix prevent the headline from establishing a sustained or staffing-caused improvement.

Return to the exercise, page 92

D2. What is the relative reduction between the reported values?

B. 25%

Subtract thirty from forty and divide the ten-minute difference by the forty-minute reference value.

Return to the exercise, page 92

08 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: The time interval covered by a reported result. Which term fits?

C. reporting period

A Not here. completeness check: Assessment of whether expected records or fields are present. The usual combination is "perform a completeness check".

B Not here. mean: The total of observed values divided by their number. The usual combination is "distinguish the mean from the median".

C Correct. reporting period: The time interval covered by a reported result. The usual combination is "align reporting periods".

D Not here. relative change: A difference expressed as a proportion of the reference value. The usual combination is "calculate relative change".

Return to the exercise, page 92

D4. Rebuild the exchange from "A shorter wait, or a different reporting window?". Malik: "We should also be careful with the word average. Both values are medians, and people may hear that every patient's wait fell by ten minutes." Jo: [missing response] Malik: "One clinic may have contributed a larger share of visits this week. If clinics have different waiting patterns, that could affect the combined result." Which response belongs in the gap?

C. Retain median in the label. It is a group summary, not a statement about each person's experience. A different summary statistic could tell a different part of the story.

A Not here. This line belongs to turn 20, after Malik says: "I will remove the proven-improvement claim and ask for the four-week review. Put the comparability caveat beside the figures, where the board will see it." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 2, after Malik says: "The board chart says waiting time improved by twenty-five percent because of the staffing change. Before I use that headline, are the two figures comparable?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Jo's response in turn 6. It connects Malik's preceding point with Malik's follow-up. The median summarizes the middle of a group's ordered values; its change does not describe every individual's change.

D Not here. This line belongs to turn 12, after Malik says: "Can the team reconstruct the data from source records? I want a repeatable calculation rather than a number copied between presentations." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 92

E / Conversation

1. reporting periods

Turn 2. The reporting periods differ: one result covers a month and the other a week.

08 / Answers and explanations / continued

2. relative change

Turn 4. The relative change is ten divided by forty, or 25%, without proving comparability or causation.

3. median

Turn 6. The median summarizes the middle of a group's ordered values; its change does not describe every individual's change.

4. case mix

Turn 8. Case mix describes the composition of visits; changes in clinic contribution may affect the overall comparison.

5. metric definition

Turn 10. The metric definition specifies what interval is measured and must remain consistent across the compared results.

6. data lineage

Turn 12. Data lineage traces the reported measure back to its sources and calculation so the result can be checked.

7. Causal attribution

Turn 14. Causal attribution claims that staffing produced the change; temporal sequence and two unmatched summaries do not establish that.

8. run chart

Turn 16. A run chart displays a measure over time and can support pattern review, while not establishing causation by itself.

9. decision request

Turn 18. The decision request is approval of the bounded validation review, not a claim of proven staffing effectiveness.

10. data-quality caveat

Turn 20. The data-quality caveat makes the comparison limitations visible beside the reported finding rather than hiding them in supporting material.

Return to the conversation, page 94

08 / Answers and explanations / continued

F / Accuracy and scope

F1. Which validation task is most relevant?

C. Align time periods, definitions, and clinic composition, then review the underlying data.

The proposed checks address the actual comparability gaps instead of selecting a more favorable number.

Return to the exercise, page 97

F2. Which board request matches the brief?

A. Approve the four-week validation review and require a report-back on comparable results.

A bounded validation request addresses uncertainty without claiming proof or expanding the requested authority.

Return to the exercise, page 97

F / Guided speaking

1. ten

Analyst: The absolute reduction is ten minutes.

Turn 1. Fifty minus forty equals ten minutes, expressed in the original units.

2. 20%

Reviewer: Relative to fifty, the reduction is 20%.

Turn 2. Divide the ten-minute reduction by the fifty-minute reference to obtain 20%.

3. medians

Analyst: These are group medians.

Turn 3. The briefing specifies median waiting times, not each individual's change.

4. unestablished

Reviewer: The cause remains unestablished from these figures alone.

Turn 4. Even a comparable difference does not by itself identify the factor that caused it.

Return to the conversation, page 98

08 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "A shorter wait, or a different reporting window?". Malik: "The staffing change happened before this week's result, but other factors may have changed too. We should not equate timing with proof." Jo: [missing response] Malik: "A series over comparable periods would be more informative than selecting one month and one week. We should look at the broader pattern." Which response belongs in the gap?

D. Correct. Causal attribution needs more than these two values. We can describe the timing without presenting the staffing explanation as established.

A Not here. This line belongs to turn 8, after Malik says: "One clinic may have contributed a larger share of visits this week. If clinics have different waiting patterns, that could affect the combined result." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 4, after Malik says: "The arithmetic is still a ten-minute reduction, relative to forty. I want to preserve that fact without suggesting it answers every other question." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 10, after Malik says: "Do both extracts start the clock at the same event? Arrival and completed registration would give us different waiting intervals." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Jo's response in turn 14. It connects Malik's preceding point with Malik's follow-up. Causal attribution claims that staffing produced the change; temporal sequence and two unmatched summaries do not establish that.

Return to the exercise, page 98

Review 1 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: The clinician requesting another service or consultation. Which term fits?

D. referring clinician

A Not here. interpreter request: A request for appropriate language assistance. The usual combination is "arrange an interpreter request".

B Not here. cancellation list: A list used to offer newly available appointments. The usual combination is "check the cancellation list".

C Not here. clinical triage: Qualified assessment of clinical priority and appropriate next care steps. The usual combination is "route for clinical triage".

D Correct. referring clinician: The clinician requesting another service or consultation. The usual combination is "contact the referring clinician".

Return to the exercise, page 26

2. A colleague needs the term for this meaning: Information explaining payment and adjustments for processed claims. Which term fits?

D. remittance advice

A Not here. clean claim: A claim meeting the applicable completeness and processing requirements. The usual combination is "define the clean-claim measure".

B Not here. revenue cycle: The sequence of administrative and financial processes from patient access through payment follow-up. The usual combination is "manage revenue-cycle performance".

C Not here. denial reason: The stated basis for a payer's denial. The usual combination is "categorize denial reasons".

D Correct. remittance advice: Information explaining payment and adjustments for processed claims. The usual combination is "review the remittance advice".

Return to the exercise, page 26

Review 1 / Explanations / continued

3. Rebuild the exchange from "The referral arrived, but the booking is not ready". Elena: "No appointment is reserved. I can update the patient by three, but I cannot promise that the missing information or a suitable slot will be available then." Marcus: [missing response] Elena: "I will say we received the referral and are checking two details before booking. I can avoid blaming the other office or making the patient repeat everything." Which response belongs in the gap?

D. Make that a callback commitment. State the time and your responsibility clearly. The patient should hear a reliable next contact, not an implied promise of a consultation.

A Not here. This line belongs to turn 6, after Elena says: "Could you ask the referring office to confirm the specialty? I should not choose whichever service happens to have an opening this afternoon." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 2, after Elena says: "The referral arrived this morning, but the specialty and appointment reason are blank. The patient expects a booking confirmation today. I need to explain what happens next." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 18, after Elena says: "I will update the patient by three. Please confirm when the referring office receives your request, and tell me if you need help reaching them." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Marcus's response in turn 10. It connects Elena's preceding point with Elena's follow-up. The callback commitment promises a status update by a stated time, not a completed appointment booking.

Return to the exercise, page 26

Review 1 / Explanations / continued

4. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "Create two lists, then: twelve rejections and eight denials. I need an owner, reason, deadline, and latest status against each claim." Tessa: [missing response] Leo: "The twelve are sixty percent of exceptions, but twelve percent of all submissions. I will put the denominator next to each percentage." Which response belongs in the gap?

A. I will structure the workqueue that way and retain claim identity across resubmissions. Several messages about one claim must not become several claims.

A Correct. This is Tessa's response in turn 18. It connects Leo's preceding point with Leo's follow-up. The workqueue organizes follow-up with owners and status, rather than leaving a combined exception total without actions.

B Not here. This line belongs to turn 16, after Leo says: "What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Leo says: "Twenty exceptions out of one hundred submissions. Can I call that a twenty-percent denial rate in the management update?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 10, after Leo says: "The summary has a broad adjustment reason. Is there another code that explains the missing detail?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 26

Review 2 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: The number of bed spaces physically present. Which term fits?

B. physical bed capacity

A Not here. confirmed discharge: A departure or discharge status verified under the relevant process. The usual combination is "distinguish confirmed discharges".

B Correct. physical bed capacity: The number of bed spaces physically present. The usual combination is "distinguish physical bed capacity".

C Not here. acuity: The level of a patient's clinical complexity or care needs. The usual combination is "account for patient acuity".

D Not here. transfer request: A request to move a patient between care locations. The usual combination is "track transfer requests".

Return to the exercise, page 51

2. A colleague needs the term for this meaning: Study of how people interact with tools, tasks, and environments. Which term fits?

B. human factors

A Not here. near miss: A potentially harmful event intercepted or otherwise not resulting in harm. The usual combination is "report a near miss".

B Correct. human factors: Study of how people interact with tools, tasks, and environments. The usual combination is "assess human-factors issues".

C Not here. process map: A representation of the steps and handoffs in a workflow. The usual combination is "build a process map".

D Not here. effectiveness measure: A defined measure used to judge whether an action worked. The usual combination is "specify the effectiveness measure".

Return to the exercise, page 51

Review 2 / Explanations / continued

3. Rebuild the exchange from "One exception total hides two kinds of follow-up". Leo: "The summary has a broad adjustment reason. Is there another code that explains the missing detail?" Tessa: [missing response] Leo: "Prioritize by deadline as well as category. I do not want older items expiring while we fix the easier ones." Which response belongs in the gap?

B. Check for a remittance advice remark code alongside the adjustment reason and group code. Read them together before assigning correction, reconsideration, or appeal work.

A Not here. This line belongs to turn 16, after Leo says: "What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Tessa's response in turn 10. It connects Leo's preceding point with Leo's follow-up. A remittance advice remark code adds detail that may be needed to interpret the adjustment or processing result.

C Not here. This line belongs to turn 2, after Leo says: "Twenty exceptions out of one hundred submissions. Can I call that a twenty-percent denial rate in the management update?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 18, after Leo says: "Create two lists, then: twelve rejections and eight denials. I need an owner, reason, deadline, and latest status against each claim." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 51

Review 2 / Explanations / continued

4. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "We could review a consistently defined sample of bookings afterward. The measure and review period should be agreed before we decide whether the change worked." Asha: [missing response] Ben: "I will prepare the timeline and mark the open questions. We can show what caught the error without claiming that we have already established its cause." Which response belongs in the gap?

B. That provides an effectiveness measure. Also monitor unintended consequences, such as delays or new workarounds, so an apparent improvement in one measure does not hide another problem.

A Not here. This line belongs to turn 16, after Ben says: "For the accreditation discussion, we can show that an action was completed. But completion alone will not tell us whether it reduced the error risk." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Asha's response in turn 18. It connects Ben's preceding point with Ben's follow-up. The effectiveness measure tests the intended improvement using a defined, relevant comparison rather than task completion alone.

C Not here. This line belongs to turn 10, after Ben says: "A simple diagram of the referral-to-booking route may help the teams see where information changed or responsibility became unclear." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 12, after Ben says: "Staff are worried this meeting will start by naming someone to blame. How do we get an honest account of the workarounds without losing accountability?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 51

Review 3 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Protected health information maintained or transmitted electronically. Which term fits?

A. electronic PHI

A Correct. electronic PHI: Protected health information maintained or transmitted electronically. The usual combination is "protect electronic PHI".

B Not here. secure transfer: Sharing through an approved protected method. The usual combination is "use an approved secure transfer".

C Not here. purpose limitation: Restricting information handling to a specified appropriate purpose. The usual combination is "document the purpose limitation".

D Not here. permitted use: A use allowed under the applicable legal and organizational basis. The usual combination is "verify the permitted use".

Return to the exercise, page 74

2. A colleague needs the term for this meaning: An agreed period during which a return call is expected. Which term fits?

B. callback window

A Not here. complaint: An expression of dissatisfaction requiring the applicable response. The usual combination is "document a complaint".

B Correct. callback window: An agreed period during which a return call is expected. The usual combination is "confirm the callback window".

C Not here. patient experience: The interactions a patient has with the health-care system. The usual combination is "improve patient experience".

D Not here. warm transfer: A handoff connecting the person with the next team while conveying relevant context. The usual combination is "arrange a warm transfer".

Return to the exercise, page 74

Review 3 / Explanations / continued

3. Rebuild the exchange from "Caught in time, but not yet explained". Ben: "A simple diagram of the referral-to-booking route may help the teams see where information changed or responsibility became unclear." Asha: [missing response] Ben: "Staff are worried this meeting will start by naming someone to blame. How do we get an honest account of the workarounds without losing accountability?" Which response belongs in the gap?

B. Use a process map grounded in actual practice. Include the handoffs and workarounds people really use, not just the ideal sequence written in the procedure.

A Not here. This line belongs to turn 12, after Ben says: "Staff are worried this meeting will start by naming someone to blame. How do we get an honest account of the workarounds without losing accountability?" It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Asha's response in turn 10. It connects Ben's preceding point with Ben's follow-up. The process map shows workflow steps and handoffs, helping locate where the error could enter or escape.

C Not here. This line belongs to turn 16, after Ben says: "For the accreditation discussion, we can show that an action was completed. But completion alone will not tell us whether it reduced the error risk." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Ben says: "The referral record and booking history are available. We can identify the sequence without relying only on what people remember after the correction." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 74

Review 3 / Explanations / continued

4. Rebuild the exchange from "Two calls, still no clear answer". Leah: "And if you have not got a clinician's time by three? That is usually when the promised update disappears." Arun: [missing response] Leah: "All right. Include the earlier messages, find out who is handling it, and contact me by three whether or not the time is confirmed." Which response belongs in the gap?

C. I will still call. That follow-through is my responsibility. You will hear the actual status and next action, even if the clinical timing remains unresolved.

A Not here. This line belongs to turn 20, after Leah says: "All right. Include the earlier messages, find out who is handling it, and contact me by three whether or not the time is confirmed." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 6, after Leah says: "Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Arun's response in turn 18. It connects Leah's preceding point with Leah's follow-up. Follow-through means honoring the promised update even when the underlying clinical response remains unresolved.

D Not here. This line belongs to turn 8, after Leah says: "What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 74

Review 4 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Movement between care settings, providers, or stages. Which term fits?

A. transition of care

A Correct. transition of care: Movement between care settings, providers, or stages. The usual combination is "support a transition of care".

B Not here. receiving provider: The professional or organization expected to take the next care role. The usual combination is "contact the receiving provider".

C Not here. community resource: A local service supporting practical or health-related needs. The usual combination is "verify community-resource availability".

D Not here. risk stratification: Grouping by assessed risk to support appropriate planning. The usual combination is "review risk-stratification criteria".

Return to the exercise, page 99

2. A colleague needs the term for this meaning: The middle ordered observation, or mean of the two middle observations for an even count. Which term fits?

A. median

A Correct. median: The middle ordered observation, or mean of the two middle observations for an even count. The usual combination is "report median waiting time".

B Not here. data lineage: The traceable path from source data to a reported result. The usual combination is "verify data lineage".

C Not here. executive dashboard: A concise display of measures supporting leadership oversight and decisions. The usual combination is "review the executive dashboard".

D Not here. special-cause variation: Variation associated with an identifiable change outside usual process behavior in context. The usual combination is "investigate potential special-cause variation".

Return to the exercise, page 99

Review 4 / Explanations / continued

3. Rebuild the exchange from "Two calls, still no clear answer". Leah: "So should I keep three o'clock free for the clinician? I do not want to miss the explanation if that is the time." Arun: [missing response] Leah: "Please do not make me explain the first two calls to another person. They are already in the record." Which response belongs in the gap?

D. We do not have a clinician's callback window yet. Three is my update time; I will not describe it as a medical appointment that has not been agreed.

A Not here. This line belongs to turn 8, after Leah says: "What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 6, after Leah says: "Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Leah says: "This is my third call about the result. I have answered the identity questions again, but I still do not know who will call me." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Arun's response in turn 10. It connects Leah's preceding point with Leah's follow-up. A callback window must be agreed by the responding team; Arun's update time is not an established clinician appointment.

Return to the exercise, page 99

Review 4 / Explanations / continued

4. Rebuild the exchange from "A shorter wait, or a different reporting window?". Malik: "For the board, the immediate request is a four-week validation review. It is not a proposal for unlimited staffing changes or a declaration that the intervention succeeded." Jo: [missing response] Malik: "I will remove the proven-improvement claim and ask for the four-week review. Put the comparability caveat beside the figures, where the board will see it." Which response belongs in the gap?

B. Put that decision request up front, with an owner and report-back. The directors should know what they are being asked to authorize before interpreting the chart.

A Not here. This line belongs to turn 10, after Malik says: "Do both extracts start the clock at the same event? Arrival and completed registration would give us different waiting intervals." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Jo's response in turn 18. It connects Malik's preceding point with Malik's follow-up. The decision request is approval of the bounded validation review, not a claim of proven staffing effectiveness.

C Not here. This line belongs to turn 4, after Malik says: "The arithmetic is still a ten-minute reduction, relative to forty. I want to preserve that fact without suggesting it answers every other question." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Malik says: "One clinic may have contributed a larger share of visits this week. If clinics have different waiting patterns, that could affect the combined result." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 99

Additional case 1 / Explanations

Extended conversation

1. preferred language

Turn 2. Preferred language identifies the language the patient wants to use for communication; basic English conversation does not establish comfort with medical discussions.

2. qualified interpreter

Turn 4. A qualified interpreter provides accurate, impartial spoken communication with appropriate language and specialist skills.

3. remote interpreting

Turn 8. Remote interpreting connects an interpreter by an approved telephone or video service rather than requiring physical attendance.

4. confidentiality

Turn 10. Confidentiality concerns protecting the patient's information; interpretation should not require discussing private matters in an exposed setting.

5. communication need

Turn 14. Communication need records the support required for effective understanding, rather than treating a nod as proof of comprehension.

6. confirmation

Turn 18. Confirmation means the requested support has actually been arranged; a request alone is not that completed status.

Return to the conversation, page 102

Transfer exchange

1. pending

Registrar: The appointment is booked, but interpreting remains pending.

Turn 1. The request has no acceptance, so the interpreting arrangement is not yet confirmed.

Additional case 1 / Explanations / continued

2. status

Colleague: Keep that separate from the visit booking status.

Turn 2. The confirmed visit and unconfirmed support have different administrative states.

3. service

Registrar: I will follow up with the interpreting service.

Turn 3. The service needs to respond before the requested support can be treated as arranged.

4. update

Colleague: Tell the patient the actual position in the next update.

Turn 4. An accurate update distinguishes the completed booking from support still being coordinated.

Return to the conversation, page 104

Additional case 2 / Explanations

Extended conversation

1. patient identifiers

Turn 2. Patient identifiers are person-specific details used together to match the individual to the correct record, not merely a similar name.

2. duplicate record

Turn 4. A duplicate record is more than one record for the same person; these records have not been shown to belong to one person.

3. referral provenance

Turn 8. Referral provenance identifies where the referral came from and how it entered the record, supporting investigation of the attachment error.

4. impact assessment

Turn 12. Impact assessment examines what actions or disclosures may already have been affected by the mismatch, beyond correcting one attachment.

5. privacy

Turn 14. Privacy requires avoiding unnecessary disclosure of another patient's information while explaining the delay.

6. record merge

Turn 18. A record merge combines records and must follow authorized identity review; it is not a routine fix for similar names.

Return to the conversation, page 106

Transfer exchange

1. person

Registrar: Similar names do not prove these records belong to the same person.

Turn 1. The differing identifiers require review and do not establish one shared identity.

Additional case 2 / Explanations / continued

2. merge

Reviewer: Do not perform a record merge on that basis.

Turn 2. Merging combines records and is not justified by name similarity alone.

3. resolution

Registrar: I will preserve the discrepancy and route it for authorized resolution.

Turn 3. The designated process must establish and correct the identity issue with traceability.

4. delay

Reviewer: Do not disclose the other record while explaining the delay.

Turn 4. The patient can receive a useful explanation without learning another person's information.

Return to the conversation, page 108

Additional case 3 / Explanations

Extended conversation

1. service-continuity plan

Turn 2. The service-continuity plan sets out how essential services and communication are maintained during a disruption.

2. contact worklist

Turn 4. The contact worklist tracks each affected patient's communication status and next action, not just the number of calls attempted.

3. alternative capacity

Turn 6. Alternative capacity means suitable usable service capacity elsewhere; a possible room alone is not confirmation.

4. successful notification

Turn 10. Successful notification means the message has been communicated through the defined process; an unanswered call alone does not establish it.

5. status update

Turn 14. A status update is a communication commitment, distinct from guaranteeing a replacement appointment.

6. handover owner

Turn 18. A handover owner is the named person responsible for unfinished actions after responsibility transfers.

Return to the conversation, page 110

Transfer exchange

1. rebooked

Scheduler: The call was attempted; the appointment is not yet rebooked.

Turn 1. No replacement slot has been offered or accepted, so rebooking is incomplete.

Additional case 3 / Explanations / continued

2. action

Lead: Record the voicemail and the required next contact action.

Turn 2. The record should identify what must happen next rather than imply the case is finished.

3. team

Scheduler: Any question about delaying care goes to the clinical team.

Turn 3. Clinical staff, not the scheduler's guess, must assess time-sensitive care questions.

4. handover

Lead: Carry the unfinished item into the shift handover.

Turn 4. The unresolved task needs a clear transfer of responsibility at shift change.

Return to the conversation, page 112

Find a phrase by purpose

Patient Access, Scheduling, and Referrals

Acknowledge receipt: We received your referral and are checking the information needed to book.

Name the missing detail: The requested specialty and appointment reason are not shown.

Request clarification: Please confirm the intended receiving service with the referring team.

Full phrase bank, page 7

Revenue Cycle, Coding, and Denials

Separate the stages: Twelve submissions were rejected before adjudication; eight claims were denied after review.

Avoid an inflated conclusion: The other eighty claims have not been analyzed here.

Request the source: Show the acknowledgment or remittance record behind this category.

Full phrase bank, page 19

Patient Flow, Capacity, and Staffing

Time-stamp the count: At 10 a.m., all twenty-four staffed operational beds are occupied.

Correct the availability: The two discharges are planned, not completed.

State the denominator: Occupancy is 100% of staffed operational capacity.

Full phrase bank, page 31

Quality, Safety, and Accreditation

State the event: The patient was booked into the wrong specialty clinic.

Report the interception: A second check caught the error before the appointment.

Limit the impact claim: No clinical harm has been reported in the available information.

Full phrase bank, page 42

Find a phrase by purpose / continued

HIPAA, Privacy, and Information Governance

Clarify the purpose: Which decision requires this information, and what output is needed?

Match the scope: A clinic-level count may answer the stated question.

Challenge excess fields: Why would this counting task require full clinical notes?

Full phrase bank, page 56

Patient Experience and Service Recovery

Acknowledge the history: I can see both earlier messages in the record.

Recognize the impact: I understand why having to call again is frustrating.

Apologize specifically: I am sorry you have not received a clear update.

Full phrase bank, page 67

Population Health and Care Coordination

Clarify pending: Is transport awaiting acceptance, vehicle assignment, or a pickup time?

State the confirmed step: The provider acknowledged the request but has not assigned a vehicle.

Name the next owner: I will take responsibility for the transport follow-up after noon.

Full phrase bank, page 79

Executive Dashboards and Board Updates

Describe the figures: The reported medians are forty minutes last month and thirty this week.

State the arithmetic: That is ten minutes lower, or 25% relative to forty.

Limit the conclusion: The comparison does not yet establish sustained improvement.

Full phrase bank, page 91

Vocabulary index

absolute change
Lesson 08, p. 90

accessibility need
Lesson 01, p. 6

accreditation evidence
Lesson 04, p. 41

action owner
Lesson 04, p. 41

adjudication
Lesson 02, p. 17

aggregate report
Lesson 05, p. 55

alternative capacity
Case 3, p. 109

appeal
Lesson 02, p. 18

appointment slot
Lesson 01, p. 5

authorization
Lesson 05, p. 54

balancing measure
Lesson 04, p. 41

bed census
Lesson 03, p. 29

boarding
Lesson 03, p. 30

business associate
Lesson 05, p. 54

call history
Lesson 06, p. 65

callback window
Lesson 06, p. 65

capacity forecast
Lesson 03, p. 30

care gap
Lesson 07, p. 77

care-team update
Lesson 07, p. 78

causal attribution
Lesson 08, p. 90

claim denial
Lesson 02, p. 17

clean claim
Lesson 02, p. 18

access entitlement
Lesson 05, p. 54

accounts receivable
Lesson 02, p. 18

acknowledgment
Lesson 02, p. 17, Lesson 06, p. 65

acuity
Lesson 03, p. 30

admission demand
Lesson 03, p. 30

aging bucket
Lesson 02, p. 18

anticipated discharge
Lesson 03, p. 29

appeal deadline
Lesson 02, p. 18

audit log
Lesson 05, p. 55

authorization requirement
Lesson 01, p. 6

bed allocation
Lesson 03, p. 30

bed turnaround
Lesson 03, p. 29

booking confirmation
Lesson 01, p. 5

business associate agreement
Lesson 05, p. 54

callback commitment
Lesson 01, p. 6

cancellation list
Lesson 01, p. 6

care coordination
Lesson 07, p. 77

care plan
Lesson 07, p. 77

case mix
Lesson 08, p. 89

claim adjustment reason code
Lesson 02, p. 17

claim submission
Lesson 02, p. 17

clearinghouse
Lesson 02, p. 17

Vocabulary index / continued

clinical interpretation
Lesson 06, p. 65

clinical triage
Lesson 01, p. 5

closed-loop communication
Lesson 07, p. 78

closed-loop referral
Lesson 01, p. 6

closure confirmation
Lesson 06, p. 66

coding integrity
Lesson 02, p. 18

common-cause variation
Lesson 08, p. 90

communication delay
Lesson 06, p. 65

communication need
Case 1, p. 101

community resource
Lesson 07, p. 78

complaint
Lesson 06, p. 66

completeness check
Lesson 08, p. 90

confidentiality
Case 1, p. 101

confirmation
Case 1, p. 101

confirmed discharge
Lesson 03, p. 29

contact attempt
Lesson 06, p. 65

contact preference
Lesson 06, p. 66

contact worklist
Case 3, p. 109

contingency plan
Lesson 07, p. 78

contingency staffing
Lesson 03, p. 30

contributing factor
Lesson 04, p. 40

corrected claim
Lesson 02, p. 18

corrective action plan
Lesson 04, p. 41

covered entity
Lesson 05, p. 54

data lineage
Lesson 08, p. 90

data minimization
Lesson 05, p. 55

data-quality caveat
Lesson 08, p. 90

de-escalation
Lesson 06, p. 66

de-identification
Lesson 05, p. 55

decision request
Lesson 08, p. 90

demographic verification
Lesson 01, p. 6

denial reason
Lesson 02, p. 18

denominator
Lesson 08, p. 89

detection control
Lesson 04, p. 40

discharge barrier
Lesson 03, p. 29

discharge coordination
Lesson 07, p. 77

disclosure
Lesson 05, p. 54

dispatch confirmation
Lesson 07, p. 77

duplicate record
Case 2, p. 105

effectiveness measure
Lesson 04, p. 41

electronic PHI
Lesson 05, p. 54

eligibility verification
Lesson 01, p. 6

empathy statement
Lesson 06, p. 65

environmental services
Lesson 03, p. 29

Vocabulary index / continued

escalation pathway
Lesson 01, p. 6

event chronology
Lesson 04, p. 40

executive dashboard
Lesson 08, p. 89

expectation setting
Lesson 06, p. 66

export permission
Lesson 05, p. 55

failure mode
Lesson 04, p. 41

first-visit confirmation
Lesson 07, p. 78

flow huddle
Lesson 03, p. 30

follow-through
Lesson 06, p. 66

follow-up appointment
Lesson 07, p. 78

front-end rejection
Lesson 02, p. 17

grievance pathway
Lesson 06, p. 66

handoff acceptance
Lesson 06, p. 66

handoff checklist
Lesson 07, p. 78

handover owner
Case 3, p. 109

HIPAA
Lesson 05, p. 54

home-health referral
Lesson 07, p. 78

human factors
Lesson 04, p. 40

identity verification
Lesson 06, p. 66

impact assessment
Case 2, p. 105

implementation evidence
Lesson 04, p. 41

incident report
Lesson 04, p. 40

intake queue
Lesson 01, p. 6

interpreter request
Lesson 01, p. 6

just culture
Lesson 04, p. 40

key performance indicator
Lesson 08, p. 89

like-for-like comparison
Lesson 08, p. 90

mean
Lesson 08, p. 89

measurement window
Lesson 08, p. 89

median
Lesson 08, p. 89

medication reconciliation
Lesson 07, p. 78

member identifier
Lesson 02, p. 17

metric definition
Lesson 08, p. 89

minimum necessary
Lesson 05, p. 54

named owner
Lesson 06, p. 66

near miss
Lesson 04, p. 40

network status
Lesson 01, p. 6

non-emergency medical transport
Lesson 07, p. 77

numerator
Lesson 08, p. 89

occupancy rate
Lesson 03, p. 29

operational bed
Lesson 03, p. 29

outcome measure
Lesson 04, p. 41

patient access
Lesson 01, p. 5

patient experience
Lesson 06, p. 65

Vocabulary index / continued

patient flow
Lesson 03, p. 29

patient identifiers
Case 2, p. 105

patient panel
Lesson 07, p. 77

patient responsibility
Lesson 02, p. 18

payer identifier
Lesson 02, p. 17

percentile
Lesson 08, p. 89

permitted use
Lesson 05, p. 54

physical bed capacity
Lesson 03, p. 29

pickup window
Lesson 07, p. 78

plain language
Lesson 06, p. 66

population health
Lesson 07, p. 77

preferred language
Case 1, p. 101

preventive control
Lesson 04, p. 40

privacy
Case 2, p. 105

privacy incident
Lesson 05, p. 55

process map
Lesson 04, p. 41

process measure
Lesson 04, p. 41

protected health information
Lesson 05, p. 54

provisional hold
Lesson 01, p. 5

psychological safety
Lesson 04, p. 40

purpose limitation
Lesson 05, p. 55

qualified interpreter
Case 1, p. 101

re-identification risk
Lesson 05, p. 55

reason for referral
Lesson 01, p. 5

receiving provider
Lesson 07, p. 78

receiving service
Lesson 01, p. 5

recipient verification
Lesson 05, p. 55

record merge
Case 2, p. 105

referral
Lesson 01, p. 5

referral completeness
Lesson 01, p. 5

referral provenance
Case 2, p. 105

referring clinician
Lesson 01, p. 5

refresh date
Lesson 08, p. 90

relative change
Lesson 08, p. 90

remittance advice
Lesson 02, p. 17

remittance advice remark code
Lesson 02, p. 17

remote interpreting
Case 1, p. 101

reporting period
Lesson 08, p. 89

resolution commitment
Lesson 06, p. 65

resubmission
Lesson 02, p. 18

retention schedule
Lesson 05, p. 55

revenue cycle
Lesson 02, p. 17

risk register
Lesson 04, p. 41

risk stratification
Lesson 07, p. 77

Vocabulary index / continued

role-based access
Lesson 05, p. 54

root cause analysis
Lesson 04, p. 40

safety event
Lesson 04, p. 40

scope of role
Lesson 06, p. 65

service acceptance
Lesson 07, p. 78

service-continuity plan
Case 3, p. 109

small-cell disclosure
Lesson 05, p. 55

special-cause variation
Lesson 08, p. 90

staffed capacity
Lesson 03, p. 29

status update
Lesson 06, p. 65, Case 3, p. 109

successful notification
Case 3, p. 109

systems approach
Lesson 04, p. 40

timely-filing limit
Lesson 02, p. 18

transfer request
Lesson 03, p. 30

transport barrier
Lesson 07, p. 77

warm transfer
Lesson 06, p. 66

room readiness
Lesson 03, p. 29

run chart
Lesson 08, p. 90

scheduling template
Lesson 01, p. 5

secure transfer
Lesson 05, p. 55

service recovery
Lesson 06, p. 65

skill mix
Lesson 03, p. 30

social needs
Lesson 07, p. 77

specialty
Lesson 01, p. 5

staffing coverage
Lesson 03, p. 30

stratification
Lesson 08, p. 89

surge capacity
Lesson 03, p. 30

throughput
Lesson 03, p. 30

tracer review
Lesson 04, p. 41

transition of care
Lesson 07, p. 77

waitlist
Lesson 01, p. 6

workqueue
Lesson 02, p. 18

References and publication notes

All cases, organizations, figures, and conversations are fictional teaching material, not transcripts. Communication practice does not replace qualified advice or current workplace procedures.

[1] US Department of Health and Human Services. Minimum Necessary Requirement.

Background for the US privacy example, including the standard's exceptions. The exercise is not a universal rule for every disclosure. Source checked 10 October 2026.

https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/minimum-necessary-requirement/index.html

[2] Centers for Medicare & Medicaid Services. Health Care Payment and Remittance Advice.

Background on remittance information and adjustment explanations. Fictional claim counts do not prescribe payer-specific correction or appeal steps. Source checked 10 October 2026.

https://www.cms.gov/medicare/coding-billing/electronic-billing/health-care-payment-remittance-advice

[3] Agency for Healthcare Research and Quality. What Is Patient Experience?

Background on patient interactions, timely information, and communication. The scheduling and service-recovery dialogues are original. Source checked 10 October 2026.

https://www.ahrq.gov/cahps/about-cahps/patient-experience/index.html

[4] AHRQ Patient Safety Network. Communication During Transitions of Care.

Background on clear handoffs and coordination. The administrative cases do not replace clinical transition protocols. Source checked 10 October 2026.

https://psnet.ahrq.gov/perspective/communication-during-transitions-care

> **Companion course**
>
> The website offers the eight-lesson course and related practice. This book can be used independently. No audio access or external account is needed for its exercises.
>
> https://englishladder.com/efsp-healthcare-administration.html

Clarify the status. Coordinate the next step. Keep the patient informed.

Practice eight administrative conversations in which precise status language, ownership, and follow-through matter.

Patient Access, Scheduling, and Referrals

Received means the referral arrived; complete means the required information is present; booked means an appointment is arranged. None of these statuses automatically establishes clinical urgency, insurance coverage, or authorization. State each confirmed fact separately.

Revenue Cycle, Coding, and Denials

For this case, rejected means not accepted into adjudication; denied means reviewed and not allowed as requested. Real systems may use labels differently. Verify the acknowledgment and remittance details rather than relying on one word in a dashboard.

Patient Flow, Capacity, and Staffing

Occupancy is 100% on the staffed operational basis: 24 divided by 24. Dividing by 28 gives about 86% physical-bed occupancy, a different measure. Neither calculation makes the four unstaffed beds or two anticipated discharges available now.

Quality, Safety, and Accreditation

No clinical harm reported is a statement about known impact, not proof that the process is reliable. A successful second check is evidence of detection in this event. The review still needs to understand the earlier failure and whether the control works consistently.

Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

English Ladder / English for Work / Revised 2026 edition

This publication includes AI-generated text and cover illustration, revised and checked during production. Brand artwork remains the English Ladder mark. This edition makes no claim of independent professional certification.

Typeface: Bitstream Vera, embedded under its accompanying license. Illustration created with OpenAI's native image generator for this edition. The copyright notice is not a claim to ownership of third-party frameworks or references.